World Health Organization

THE 5th ANNUAL SEAPHEIN MEETING

Reducing health inequities through the strengthening of health systems and ICT applications in public health education and practice in PHC

18-19 October 2009



Faculty of Public Health, Mahidol University, Bangkok, Thailand

Supported by : ***WHO - SEARO***
Hosted by : ***Faculty of Public Health, Mahidol University***

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Background

Background:

Thirty years ago, Alma Ata Declaration of "Health for All by the Year 2000" resulted in development of Primary Care. Many perceive that the Declaration has failed to achieve its goal of Health for All. Certainly, all people are not healthy and freedom from diseases has not been achieved. Given current inequities in technology and socio-economic conditions it is unlikely that these goals will be achieved. However, the concept of Primary Health Care is alive and continues to be an important approach to achieving at least essential health services, with an emphasis on public health for all. We may not be able to achieve comprehensive health services for all in the next 30 years or even by the MDG target of 2015. However, efforts to attain equity in at least essential health services should and will continue.

Our world is developing even faster than predicted. Although there have been many advances that have made our lives easier we have also seen some negative impacts resulting from these changes. In public health, we have achieved many improvements such as increasing life expectancy, and decreasing infant, maternal, and child mortality. The concept of primary health care has evolved and is practiced with various modifications, in accordance with local demands and conditions. Lessons learned from practicing primary health care in South-East Asia Region (SEAR) countries will result in further improvements through sharing experiences and collaborating our efforts.

Looking towards the future, it is essential that schools of public health and other health related institutions adopt novel approaches to meet new challenges. The concept of Primary Health Care may need to evolve into one of Primary Health Services in order to account for not only treatment but also prevention and promotion interventions. Therefore, WHO SEARO has initiated a link to revitalize PHC and public health education thereby strengthening capacity and ensuring that PHC will continue to gain effectiveness in the future.

WHO-SEARO partnered with South East Asia Public Health Educational Institutes Network (SEAPHEIN) to develop a conference in conjunction with SEAPHEIN's Annual Meeting. SEAPHEIN is the leading organization body for public health education in the region. Its purpose is to facilitate collaboration among its members and coordinate efforts to develop capacity strengthening throughout the region. SEAPHEIN's annual meeting provides a forum that will enable us to reach the innovators in public health education in SEAR. This is vital to the success of our endeavor as these institutions are shaping the future of public health as they train the next generation of public health professionals and leaders. Specifically the Network undertakes activities to develop training partnerships between member institutions, promote novel approaches education/training program provision, standardize curricula and assist in the development of country plans and strategies. By joining with SEAPHEIN, WHO-SEARO was ensured access to public health educators and planners who actively participate in the development of the field in our region. Combining our administrative expertise and network of contacts we were certain to achieve the goals set for this conference.

Objectives:

1. To refresh understanding of SEAPHEIN's members to the ideals of primary health care/services: principles and methods
2. To share experiences among SEAPHEIN's members of the implementation of primary health care/ services in each of their countries
3. To sharpen the role and function of SEAPHEIN's members in the implementation of primary health care/services in their own countries
4. To develop curriculum and modules on PHC to be used by all SEAPHEIN members
5. To discuss the future focus and direction of SEAPHEIN

SEAPHEIN Charter

SEAPHEIN Charter

1.0 Name

The name of this organization is The South-East Asia Public Health Education Institutes Network and is referred to in this Charter as SEAPHEIN.

2.0 Purpose

The following definitions were officially adopted 7 April 2004 by SEAPHEIN members at the international forum in Bangkok, Thailand

2.1 Vision

To be a collaborative network of Public Health Education Institutes in the SE Asian region for strengthening public health capacity

2.2 Mission

To collaborate with SE Asian member countries in partnership to improve and sustain the quality and relevance of public health education to address the increasing challenges of health improvement

2.3 Objectives

SEAPHEIN seeks to share its resources in order to achieve the following eight objectives:

- 2.3.1 To make public health education programs relevant to meet the health challenges of individual countries in the region
- 2.3.2 To facilitate the development of health information systems in all countries
- 2.3.3 To establish collaborative programs in education and training
- 2.3.4 To provide evidence-based and new knowledge through research
- 2.3.5 To strengthen the capacity of members through faculty, student and information exchange, learning materials and methods
- 2.3.6 To facilitate the implementation of accreditation programs in public health education
- 2.3.7 To provide consultation, advocacy and technical advice to improve national public health programs of member countries
- 2.3.8 To promote leadership development in public health

2.4 Strategies

The strategies of SEAPHEIN include:

- 2.4.1 Develop curriculum and continuing education programs, reorient learning methods and conduct research to refine curriculum
- 2.4.2 Create information systems in line with education needs
- 2.4.3 Form partnerships and assign focal points among public health institutes, strengthen existing programs including searching and learning materials and establish regional MPH programs
- 2.4.4 Maintain an epidemiological database, prioritize and conduct cross-country research and build comprehensive capacity among stakeholders in all stages of research
- 2.4.5 Institutionalize programs for capacity building, inventory available resources and learning material resources and identify needs for staff and student exchange
- 2.4.6 Develop institutional accreditation mechanisms by establishing a regional accreditation council and devise strengthening strategies
- 2.4.7 Organize consultation meetings on priority areas of public health programs and health care
- 2.4.8 Promote leadership and management skills at national, state and district levels and generate learning organizations

3.0 Executive Committee

The Executive Committee is composed of a President, President-elect, Treasurer, Secretary General and one representative for each country. The term of office shall be two years and is not renewable. President elected should be put in place. The Term of Office shall start and end at the closing of the Annual Meeting.

3.1 The President is responsible for calling and presiding at SEAPHEIN meetings and workshops, appointing committee members, presenting awards and all other SEAPHEIN duties. Any vacancy of an elected officer shall be filled by President appointment.

3.2 The Treasurer, working with the Secretary shall collect and administer all SEAPHEIN funds and report on these matters.

3.3 The Secretary General shall be appointed by the President.

3.4 The Secretary General shall head the SEAPHEIN Secretariat Office and is responsible for maintaining files, handling correspondence, preparing and circulating official agendas and minutes of the Annual Meeting and reports, assisting the Treasurer in preparing the financial report and receiving and processing membership applications. Written notices of the Annual Meeting shall be sent two months prior to the date, specifying the time, place and agenda items to be considered. In addition, the Secretariat Office shall develop strategies for SEAPHEIN funding, fund raising, and organizing activities.

3.4.1 Secretariat Office

The Office of the Secretariat of the SEAPHEIN Network is currently located at: Faculty of Public Health, Mahidol University 420/1 Rajvithi Road, Ratchathewi, Bangkok 10400 THAILAND Telephone/Fax: 662 354 8227 E-mail seaphein@diamond.mahidol.ac.th

4.0 General Assembly

The General Assembly shall be the primary policy making body of SEAPHEIN and approve the actions of the Executive Committee. Only Full Members may vote, each Full Member having one vote only.

4.1 The General Assembly shall elect the SEAPHEIN Executive Committee Officers, approve new members, determine membership fees and have the power to amend SEAPHEIN Charter provisions. One quarter of the full membership shall constitute a quorum provided that at least two thirds of the votes cast are in favor in order to pass a resolution. All votes shall be given personally or in the case of Charter amendments, personally or by proxy. Charter amendments must be recommended by the Executive Committee and be approved by at least half of the Full Membership. Voters must be valid, Full Members.

4.2 The SEAPHEIN General Meeting shall be held at least one in two years as designated by the General Assembly, prior to the end of the preceding General Meeting.

5.0 SEAPHEIN Committees

Standing and ad hoc committees and their Chairs shall be appointed by the President and announced at the General Meeting.

5.1 Each standing and ad hoc committee shall prepare an annual report for submission to the General Assembly at the General Meeting.

5.2 Each country should have its own national network of Public Health Education Institutes.

6.0 Membership

SEAPHEIN Membership is open to public health education institutes that subscribe to the SEAPHEIN vision, mission and objectives, have completed membership application procedures and have been duly nominated and approved by the General Assembly. Membership is open to countries outside the WHOSEA Region and is not limited by geographic boundary.

6.1 Members

6.1.1 A member institute is represented by its CEO or by a designee of the institute duly appointed by the CEO in writing.

6.1.2 Only full institutional members of SEAPHEIN shall constitute the SEAPHEIN General Assembly.

6.1.3 SEAPHEIN and its General Assembly shall facilitate the primary academic functions of its member institutes.

6.2 Membership Types

There shall be five types of membership: full, associate, personal, honorary and network.

6.2.1 Full Members are either original charter members or institutions whose application and nomination are unanimously approved by the General Assembly in due process and pay the Full Member Annual Fee.

6.2.2 Associate Members are academic health institutions or health related organizations, whose missions and objectives are consistent with the SEAPHEIN Mission and Objectives, but does not otherwise qualify for full membership, have the unanimous approval of the General Assembly and pay the Associate Member Annual Fee.

6.2.3 Individual Members are persons who work in the public health field, sympathetic to the SEAPHEIN Mission and Objectives, approved unanimously by the General Assembly and pay the Personal Member Annual Fee.

6.2.4 Honorary Members are individuals unanimously approved by the General Assembly who have contributed significant service to SEAPHEIN, wish to maintain their relationship with SEAPHEIN, agree to participate in SEAPHEIN activities at no cost to SEAPHEIN and may serve in an elected position with the unanimous approval of the General Assembly. Honorary Members are not required to pay the annual membership fee.

6.2.5 Network Members are public health networks whose missions and objectives are consistent with the SEAPHEIN Mission and Objectives approved unanimously by the General Assembly and pay the Network Member Annual Fee.

6.3 Member Rights

Only Full Members are eligible to submit nominations, be voting members of the General Assembly, hold elective office or be appointed to committees unless otherwise determined by the General Assembly.

6.4 Member Applications and Approval Procedures

Application for membership shall be submitted to the Secretariat Office at least one month prior to the SEAPHEIN General Meeting and must be sponsored by an existing Full Member. Applications are presented at the SEAPHEIN Executive Committee for approval and ratified by the next General Assembly.

6.5 Termination of Membership

Members in arrears will be placed on provisional status for one year at which point their membership will lapse. Members on provisional status may not vote or hold elective office, and must pay all arrears owing before Full Membership status is resumed.

6.6 Membership fees

Full, Associate, and Individual Members contribute the amount of US $100 annually. The Institution which hosts the secretariat office is exempt from membership fee. Members are expected to ensure that their organizations have paid their annual fees.

Executive Summary

The 5th Annual South East Asia Public Health Education Institutes Network (SEAPHEIN) meeting was held on 18 and 19 October 2009 in conjunction with the WHO-SEARO meeting on Health Care Reform (20-22 October 2009). The theme of the meeting was "Reducing health inequities through the strengthening of health systems and ICT applications in public health education and practice in PHC". The venue for these events was the Dusit Thani Hotel in Bangkok, Thailand. There were 64 participants representing 11 countries.

The meeting was opened with welcome addresses by Dr. Phitaya Charupoonphol, SEAPHEIN President, and Dr. Napatawn Banchuin, Vice-President of Mahidol University. There were also opening remarks by Dr. Hasbullah Thabrany, SEAPHEIN President-elect and Dr. Samlee Plianbangchang, Regional Director WHO-SEARO. Group photos were taken of all the participants.

The SEAPHEIN president reported on the work done over the past year and representatives from Indonesia, Nepal, Japan, Myanmar and India then presented their respective country's reports. Participants then listened to addresses by the three keynote speakers on the three key conference issues of networking and collaboration, strategies to reduce inequities in health, and ICT in public health education and training. The keynote speaker on networking and collaboration was Dr. Orapin Singhadej, Secretary-General of the Network of WHO Collaborating Centres and Centres of Expertise Thailand (NEW-CCET). Dr. Viroj Tangcharoensathien, the Director of the International Health Policy Program (IHPP) Thailand, delivered the keynote speech on approaches and strategies to reduce inequities in health, and Ms. Cecil Benavidez representing INVENT Health Asia, also presented a keynote speech on the subject of ICT in public health education and training. Each participant then took part in one of the three workshop sessions, which focused on these three conference themes.

An executive board meeting and general assembly were held to discuss organizational issues. Charter amendments were adopted, new member applications were approved, and officers were elected. Dr. Hasbullah Thabrany, SEAPHEIN President-elect, was installed as the President of SEAPHEIN.

Conclusions and recommendation of the three workshops

1) Workshop on the scope for networking and collaboration among member institutes:
    - To share knowledge and practices on collaborative activities through country or institution reports, websites, and SEAPHEIN international conferences
    - To formulate groups researching specific areas, such as management, epidemiology, and educational research in public health and technology
    - To identify training courses such as health economics, Public Health policy, health care financing, and Public Health ethics.
    - Public Health curricula and competency levels should be evaluated.
    - The forming of a committee responsible for training and conferences, collaborative research, country/institution profiles on websites, capacity building for junior staff, and the establishment of a SEAPHEIN Council.

2) Workshop on approaches and strategies in reducing health inequities:
   - The role of academic institutions is teaching, sharing and researching health inequities
   - Strategies to strengthen health system reforms through curriculum review, the transforming of research into policy, field research study in communities including community diagnoses, treatment, development, empowerment and participation.
   - Building political commitments for health care reform
   - To strengthen and distribute manpower at different levels of health services focused on Primary Health Care.
   - The setting up of a task force to share the best practices in health equity from conferences, the SEAPHEIN journal, and ICT
3) ICT application in public health education and training
   - Utilizing ICT, such as teleconference facilities, e-learning, telemedicine systems and satellite programs in public health education and training, and to improve access to health services in rural areas
   - The utilizing of ICT in health services for the diagnoses and treatment of diseases and the management of outbreaks through multi-sector collaborative campaigning in critical areas
   - The establishment of a SEAPHEIN E-library and SEAPHEIN website using IT staff development.

Charter amendments

The Executive Board reviewed and amended the charter as follows:

1. Change the title of Chair to President throughout the document (starting in item 3.0).
2. Item 3.0 should be amended to read as follows:
   "The Executive Committee is composed of a President, a President-elect, a Treasurer, a Secretary-General, an immediate past president and five focal points who will be elected as representatives. The President-elect should be installed. Terms of office shall be two years and are not renewable. Terms of office shall commence at the closing of an annual meeting and end at the closing of the annual meeting two years henceforth."
3. Focal points are defined as representatives of each country selected by that country's network, or, if only one institute exists within a selected country, that institute shall be the sole representative of that country.
4. Add the office of President-elect.
5. An item will be added stating that a sub-secretariat office will be located at the institute where the SEAPHEIN President is located.
6. An item will be added noting that the Secretary shall be appointed by the President.
7. An item will be added stating that fundraising activities will be pursued in order to maintain sustainability and a decreased reliance on WHO-SEARO support.
8. Change the title of Secretary to Secretary-General throughout the document.

After extensive discussion in general assembly, the following items have been approved by the General Assembly.

1. Change the title of Chair to President throughout the document starting at item 3.0
2. Item 3.0 should be amended to read as follows:
   "The Executive Committee is composed of a President, President-elect, Treasurer, Secretary General and one representaive from each country, The terms of office shall be two years and are not renewable. The President elected should be put in place. The Term of office shall start and end at the closing of an Annual Meeting."
3. Add office of President-elect.
4. An item will be added stating that a sub-secretarial office will be located at the institute where the SEAPHEIN President is located.
5. Add an item stating that the Secretary General shall be appointed by the President.
6. Change the title of Secretary to Secretary-General throughout the document.
7. Add an item stating that fundraising activities will be pursued in order to maintain sustainability and a decreased reliance on WHO-SEARO for support.

New SEAPHEIN members

The following institutes were unanimously approved as new SEAPHEIN members by the general assembly:

- Faculty of Public Health, Pathumthani University, Thailand
- University of Community Health, Magway, Myanmar
- School of Public Health, National Institute of Public Health, Cambodia
- Maldives College of Higher Education
- School of Medical Sciences, Kathmandu University, Nepal
- Royal Institute of Health Sciences (RIHS), Royal University of Bhutan

This brings the total number of SEAPHEIN member institutes to 56 from 13 countries.

Election of officers

The following were elected to two-year terms of office:

Dr. Phitaya Charupoonphol as President-elect and Dr. Ardini Raksanagara as Treasurer. Dr. Phitaya will resume as President of SEAPHEIN at the closing of the 2011 annual meeting.

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18-19 October 2009
Dusit Thani Hotel, Bangkok, Thailand

Date and Time	Program	
Sunday 18 October 2009		
08.00 – 09.00	Registration – Outside Vimarnsuriya room	
09.00 – 10.00	Opening Ceremony – Vimarnsuriya room Welcome by Dr. Phitaya Charupoonphol, SEAPHEIN President Welcome by Dr. Napatawn Banchuin, Vice-President Mahidol University Remarks by Dr. Hasbullah Thabrany, SEAPHEIN President-Elect Remarks by Dr. Samlee Plianbangchang, Regional Director, WHO-SEARO	
10.00 – 10.30	Group photo - Vimarnsuriya room	
10.30 – 11.00	Break – Vimarnsuriya foyer	
11.00 – 12.00	Dusit Hal lPresident's report and country reports	
12.00 – 13.00	Lunch – Il Cielo restaurant	
13.00 – 14.30	Dusit Hall Keynote speaker: Dr. Orapin Singhadej, NEW-CCET Scope for Networking and Collaboration among Member Institutes Discussants: Dr. Prawit Taytiwat, Naresuan University Dr. Tin Min, University of Public Health, Myanmar	
14.30 – 15.00	Break – Dusit Hall	
15.00 – 17.00	Dusit Hall Group work: Scope for Networking and Collaboration	Lumpini room SEAPHEIN Executive Board Meeting
18.00 – 21.00	Welcome dinner and cultural show – Benjarong Restaurant	

Workshop schedule

<table>
<tr><th>Date and Time</th><th colspan="2">Program</th></tr>
<tr><td>Monday19 October 2009</td><td colspan="2"></td></tr>
<tr><td>08.00 – 09.30</td><td colspan="2">Dusit Hall Keynote speaker: Dr. Viroj Tangcharoensathien, IHPP Thailand Inequities in Health Approaches and Strategies Discussant: Dr. Hasbullah Thabrany, University of Indonesia Dr. Myint Htwe, Programme Director, WHO-SEARO</td></tr>
<tr><td>09.30 – 10.00</td><td colspan="2">Break - Dusit Hall</td></tr>
<tr><td>10.00 – 12.00</td><td colspan="2">Dusit Hall Keynote speaker: Ms. Cecil Benavidez, In Went Health Asia ICT in Public Health Education and Training Discussant: Dr. Dechavudh Nityasuddhi, Head, Department of Bioststistics</td></tr>
<tr><td>12.00 – 13.00</td><td colspan="2">Lunch – Il Cielo restaurant</td></tr>
<tr><td>13.00 – 15.00</td><td>Dusit Hall Group work: ICT in Public Health Education and Training</td><td>Lumpini room Group work: Inequities in Health : Approaches and Strategies</td></tr>
<tr><td>15.00 – 15.30</td><td colspan="2">Break - Dusit Hall</td></tr>
<tr><td>15.30 – 17.00</td><td colspan="2">Dusit Hall
General Assembly
* Presentations of group work
* Installment of Dr. Hasbullah Thabrany as SEAPHEIN President
* Voting – new members, President-elect, Treasurer, Secretary General
* Future Plans of SEAPHEIN
* Adjourn meeting</td></tr>
</table>

Opening ceremony
Welcome message from SEAPHEIN President
Assoc. Prof. Dr. Phitaya Charupoonphol

Good morning Dr. Samlee Pliangbangchang, Regional Director of WHO-SEARO Dr. Napatawn Banchuin, Vice-President, Mahidol University Dr. Maureen Birmingham, WHO Representative to Thailand, Members of SEAPHEIN, Distinguished guests, Ladies and gentlemen,

It gives me great pleasure to welcome you all to Thailand and the 5th Annual SEAPHEIN Meeting.

We are honored to host this important event and provide a forum for discussing the challenges and opportunities to reduce inequities in health.

The theme of this year's meeting is particularly important as the global financial crisis has resulted in not only increasing the size of the gap in health inequities but also increasing the number of those underserved. The issue of health inequities lies at the heart of achieving the Millennium Development Goals and as educators we should focus on guiding the next generation of public health professionals to develop solutions for these ongoing challenges.

As you can see from the schedule we have an outstanding array of keynote and guest speakers who have agreed to share their views and experiences with you relating to reducing inequities in health. Their presentations will cover the most up to date thinking on the topics and provide you with fresh ideas and inspiration to take home and apply in your work. In the next two days our objectives include identifying the role of SEAPHEIN in reducing inequalities through health system strengthening and ICT applications, and identifying areas of collaboration among SEAPHEIN member institutes.

The conference aims to foster international exchange and friendship in the pursuit of improving Public Health education. Asking difficult questions, using information intelligently and involving key stakeholders and institutions are central to the practices that underlie successful health system strengthening. It will take our combined efforts as current and future public health professionals to make advances in this area. Our activities at this conference will serve to strengthen our institutions capacities and plant the seeds of alliances that we must actively nurture in order to reap the benefits of full partnerships and collaborations.

As representatives of the leading public health educational institutions in the region we bear the responsibility of shaping the next generation of public health leaders to continue the work we have started and lead our countries to a healthy future. Together, we have the influence, knowledge, and responsibility to make our region a better home for all people.

I would like to thank the members of the Faculty of Public Health, Mahidol University, particularly Assoc. Prof. Pornpimol Kongtip and her team, for their tireless efforts to organize this meeting. In addition this event would not be possible without the continued guidance and financial support of WHO-SEARO.

Thank you all for participating in this conference. In addition to the opportunities presented by the conference's working sessions I hope you enjoy your experience of Thai culture and hospitality.

Thank you.

Opening ceremony

Welcome message from Prof. Napatawn Banchuin, Vice president of Mahidol University at the 2009 SEAPHEIN annual meeting

Dr. Samlee Plianbangchang, Regional Director of WHO-SEARO, Dr. Maureen Birmingham, W.HO. representative to Thailand, distinguished participants, ladies and gentlemen.

It is my pleasure to welcome you to Bangkok and the 2009 SEAPHEIN annual meeting. Mahidol University is honored to host this important international forum where academic leaders in the field of public health can meet and share their perspectives on current and future issues and opportunities. The theme of this year's meeting "Reducing health inequities through the strengthening of health systems and ICT applications in public health education and practice in PHC", is particularly timely given the global financial crisis.

In the past decade, we have seen unprecedented development throughout our region. However, not all of our people have reaped the rewards of this economic and social transformation. Progress has brought with it new challenges and in some cases has exacerbated existing issues. Globalization has brought with it the rapid spread of communicable diseases. Economic development has brought an increased burden of non-communicable diseases, and as the current global economic crisis has shown, the gap between rich and poor continues to grow.

As leaders from the most prestigious public health educational institutions in our region, it is our responsibility to produce the next generation of public health professionals and to equip them with the knowledge, skills, and abilities to face both the known and unknown challenges that are sure to come. It is our responsibility to set high standards, provide inspiration, and push our students to learn the best local, national, and international practices. We must also teach our students how to work outside academia and apply their knowledge in practice so they are able to make positive contributions to their communities. By continuing to develop our institutes' capacities through collaboration and knowledge sharing, we can provide the best education possible to our students.

I strongly encourage you to take advantage of the opportunities provided by this forum to create new pathways to further develop your organizations' capacity. Through sharing our experiences and being open to new ideas, we may discover ways to elevate the condition of public health not only in our own back yards but also around the world. Collaborative efforts in research, organization and development will contribute to capacity building and the strengthening of public health throughout the international community.

I would like to thank the local organizing committee, particularly Dr. Phitaya Charupoonphol and Assoc. Prof. Pornpimol Kongtip and their staff for their endless hours and efforts to bring us all together. Also, this meeting and workshop would not have been possible without the generous support of WHO-SEARO.

Thank you all for participating in this important event. I wish you success in your endeavors.

Thank you.

Address by Dr. Hasbullah Thabrany, SEAPHEIN President-Elect.

Good morning Dr. Samlee Plianbangchang W.H.O. Regional Director South-East Asia and his team, Dr. Minway, Dr. Jigme, Dr.Maureen Birmingham W.H.O. representative to Thailand, Dr. Napatawn Banchuin, Vice President, Mahidol University, and Dr. Phitaya Charupoonphol, SEAPHEIN President and his team from Mahidol University:

It was five years ago when we met Dr. Chalermchai Chikittiporn here in Bangkok to establish SEAPHEIN, then, an initiative of SEARO. Time goes very fast and the challenges of public health in this region have increased which also means that we must improve our capacity in order to overcome all of the public health challenges in this region. I am glad that this morning we can convene here to further discuss SEAPHEIN's future role and also to review what we discussed a few years earlier. We have to remember the Public Health Initiatives declared in Calcutta in 1999.

I think we have to look at how far we have come in the last 10 years to achieving the initiatives declared in Calcutta. We need to speed up our progress because the world is going very fast and therefore, I would like to thank all of you who have come here. You have sacrificed your own time or perhaps quality time with your children to attend this meeting, and some of us will also attend the meeting on W.H.O. health care reform. Hopefully, we will reform our institutions and ourselves so that we can reform healthcare in our respective countries. This is necessary because W.H.O. initiatives in the region are being reformed. If we don't reform ourselves, we will not be able to meet the goal of public health, which is to make the people of the region healthy.

I would like to give special thanks to our guest speakers, Dr. Orapin, Dr. Virot and also our guest from the German Institution INVENT, Ms. Cecil Benavidez. She is here with us to share her knowledge on what we need to do to reduce health inequities.

Lastly, I would very much like to thank the organizing committee, Dr. Phitaya's team, and the secretary, Nancy, for their hard work to ensure the success of this meeting.

I wish all of us a successful meeting and a stronger commitment to improve and reform ourselves in order to meet health care reform in this region.

Thank you very much.

Opening ceremony

Address by Dr. Samlee Plianbangchang W.H.O. Regional Director, South-East Asia.

Dr. Napatawn Banchuin, Vice President, Mahidol University, Dr. Phitaya Charupoonphol, SEAPHEIN President, Dr. Hasbullah Thabrany, SEAPHEIN President-Elect, distinguished SEAPHEIN members, honorable guests, ladies and gentlemen:

I am pleased to have the opportunity to speak at the opening of SEAPHEIN's annual meeting. This is the network's 5th meeting and is also another step forward in SEAPHEIN's work. I thank the organizers for their invitation. This meeting will deliberate in three important areas: inequities in health, ICT in Public Health education, and networking and collaboration.

Concerning inequities in health, this subject will also be discussed at the Regional Meeting on Health Care Reform for the 21st Century. The regional meeting will take place from the 20th to the 22nd of October in this hotel. It will greatly facilitate the improvement of the quality of public health education and practice. Late or reluctant applications of ICT will slow down the progress of public health development. Networking and collaborations can bring us a long way forward in improving our national and international co-operative projects. This co-operation can play an important role in improving the quality of public health education and training in participating institutes. In this connection, we have to look at SEAR in a different perspective and in a broader context than merely a region of W.H.O. Let us think in these terms, and try to get more like-minded people to work as part of SEAPHEIN. This will form a critical mass of like-minded people who will improve the quality of public health education and practice. WHO/SEARO will continue to fully support the work of SEAPHEIN. I hope that SEAPHEIN will become stronger and stronger so its work can bring more benefits to its member institutes everywhere.

Ladies and gentlemen, the aim of public health is to keep individuals and groups of people ever healthy and with the least amount of disabilities, dependencies and deaths. In this context, we need public health programmes which prevent and solve people's health problems and which pay special attention to health risks and physical and socio-economic health determinants. We need public health programmes that have been collaboratively developed and managed through multidisciplinary and multi-sectoral efforts and programmes that are implemented through community action in close collaboration with local governments. Such fruitful collaborations need the full participation and involvement of other partners including various social organizations. The primary function of public health is the prevention of physical, mental and social diseases. Public health ensures social and economic productivity and prevents community dependence. A healthy public also ensures socio-economic productivity, which contributes significantly to a country's prosperity.

Ladies and gentlemen, despite these important roles, Public Health is yet to find its appropriate place in the national developmental agenda. Public Health is still yet to receive a reasonable share of national resources for its development. Nor has Public Health been credited with the social recognition it deserves. A lot of work needs to be done to raise the status of Public Health issues to their proper place in the country's developmental agenda. That is why we have the SEAR Public Health Initiative and SEAPHEIN. We are all well aware that these are small organizations, but we can contribute to the public health of the South East

Asian Region. Both of these movements started in 2004. Our mission, among other things, is to further uplift public health to its proper status in the national health development plan. With contemporary global crises, such as emerging diseases and climate change, public health has inadvertently become more of a recognized necessity. However, its importance is still under recognized. A lot more work needs to be done in further advocating improvements in public health and while working to improve the quality of public health education and practice, we also need to energetically convene.

In particular, we have to convince the politicians who make decisions on national health policy. No less important is the need to convince our own peers in health community, so that they appreciate the value of preventive interventions in health care. Our aim in this advocacy is to at least achieve the right balance between prevention and cure. This balance equally benefits the healthy and the ill. Ultimately however, we should look forward to the day when our countries have entirely socially and economically productive healthy populations, which have the least number of sick, disabled or socially and economically dependent people.

Ladies and gentlemen, with these words, I wish you all every possible success in this meeting and also an enjoyable stay in Bangkok.

Thank you.

SEAPHEIN President's annual report for 2008 – 2009

Assoc. Prof. Dr. Phitaya Charupoonphol, Dean, Faculty of Public Health, Mahidol University.

Dr. Phitaya was the acting President of SEAPHEIN and began by stating his vision of SEAPHEIN as being a collaborative network of Public Health Education Institutes of the South-East Asian region, which was established to strengthen regional public health capacity. He also welcomed members from outside the South East Asia region such as the PDR of Lao, Cambodia, Japan and others.

The mission of SEAPHEIN is to collaborate with South-East Asian member countries in order to improve and sustain the quality and relevance of public health education necessary to address the increasing challenges of health improvement. We could move forward by providing a directory of experts, and attending conferences and meetings supported by the World Health Organization.

Our responsibilities are:

- Serving local, national and international communities
- Promoting the health and welfare of the populations of our countries
- Sharing and collaborating in order to develop public health curricula in our region
- Augment the capacity building of our member institutes

Dr. Phitaya also took a retrospective look at the 4th annual SEAPHEIN meeting held in Jakarta, Indonesia, from 8-10 of August 2008 and linked discussions from the 4th annual meeting to the present 5th annual meeting. The theme of the 4th annual meeting was "Developing curriculum and modules to strengthen Primary Health Care to support the attainment of millennium development goals". Discussions of the 4th annual meeting were on the following topics.

- Charter review- the executive board discussed revising the SEAPHEIN charter.
- Research in Primary Health Care by a member country or collaborative research by member countries may provide results useful to other countries.
- Accreditation: Accreditation of School of Public Health following the Calcutta declaration in 1999.
- Curriculum and modules to strengthen PHC: To support SEAPHEIN member countries by helping them to develop their curricula in Primary Health Care and encouraging them to include Primary Health Care components in their MPH courses.

Workshop recommendations:

- Delegates to consult with relevant Primary Health Care stakeholders in their respective countries
- The defining of competency levels for both Bachelor and Master degree programs. For example, Bachelor degree programs to focus on PHC at operational level competencies including specific activities occurring at the sub-district health center level. Master degree programs to focus on management strategies and the revitalizing of PHC at the district, provincial, and national levels.
- Inter-country working groups should be formed in order to further develop a regional core curriculum standard framework in PHC.
- Inter-country working groups should consult with their respective W.H.O. (SEARO or WPRO) regional office in regards to their international PHC curricula
- To establish an MPH program with a PHC focus

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Meeting outcomes

- A Memorandum of Understanding (MOU) was reached between the Faculty of Public Health, University of Indonesia, and the National Institute of Preventive and Social Medicine (NIPSOM), Bangladesh, BPKIHS, and the School of Public Health & Community Medicine, Dharan, Nepal.

In 2008, the Faculty of Public Health, Mahidol University, arranged for a climate change conference on the theme of Protecting our climate; improving our health: international conference on the Public Health implications of climate change", to be held in August of 2008 in Bangkok.

Tokai University held a training course for future health leaders:

- 14 fellows attended the course and the international symposium titled "Attainment of healthy ageing: how to develop a healthy and active life."
- Participants represented Bangladesh, Cambodia, India, Indonesia, Japan, the PDR of Laos, Palestine, the Philippines, South Africa, Tanzania, Thailand, and Vietnam
- The training focused on future planning using the health forecasting method utilized by JICA.

Kathmandu University is collaborating with the Faculty of Public Health, Mahidol University (FPHMU)

- The FPHMU has granted a scholarship to a qualified candidate of Kathmandu University (KU) to attend the FPHMU, MPH International Program
- The FPHMU will assist KU in developing their curriculum and also help them to establish a Public Health institution.

The College of Public Health Sciences (CPHS), Chulalongkorn University, was designated as a W.H.O. collaborative centre for research and training in Public Health development. Dean and Professor, Dr. Surasak Taneepanichskul is the Director of the center.

SEAPHEIN activities in 2009

- To assist in the development of Bachelor of Public Health degree program at the Royal Institute of Health Sciences (RIHS), Bhutan
- Chulalongkorn University, Thailand & the University of Indonesia hosted study visits for members of the Ministry of Health and RIHS Bhutan
- An MOU has been established between the Faculty of Public Health, Mahidol University and the Royal Institute of Health Sciences (RIHS), Bhutan
- The Faculty of Public Health, University of Indonesia, conducted training for the Ministry of Health and Family Welfare, Bangladesh

New member applications

SEAPHEIN received new member applications from 6 institutes:

1. Pathumthani University, Thailand
2. University of Community Health, Magway, Myanmar
3. School of Public Health, National Institute of Public Health, Cambodia
4. Maldives College of Higher Education
5. School of Medical Sciences, Kathmandu University, Nepal
6. Royal Institute of Health Sciences, Royal University of Bhutan

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The objectives of the 5th annual SEAPHEIN meeting, 2009, are as follows:

- To share country issues and experiences related to inequities in health.
- To introduce measures to improve the quality of public health education and practice in S.E.A.R. countries.
- To determine the role of SEAPHEIN in the reduction of health inequities through the strengthening of health systems and ICT applications in public health education and practice in PHC.
- To identify areas of networking and collaboration between SEAPHEIN member institutes.
- To discuss the future focus and direction of SEAPHEIN

Dr. Phitaya encouraged SEAPHEIN members to develop an international textbook in Public Health including the listing of a directory of experts. He emphasized that we cannot stand-alone and that your whole-hearted support was necessary.

Country Report - Bhutan

I. Background

Bhutan is a small land locked country in the eastern Himalayas surrounded by two Asian giants, China to the north and India to the south, east and west. It has a total area of 38,394 square kilometers and a population of 671,083 thousand. of which nearly 70% are rural engaged mainly in subsistence farming and rearing livestock.

The country is divided into 20 administrative units called dzongkhags or districts which are further sub divided into blocks or geogs. Geographically, there are 4 regions-eastern, western, central and southern.

The rugged mountainous terrain and scattered population pose a daunting challenge to all developmental activities notwithstanding the health care delivery system.

II. Country Health Scenario

Although the first modern hospital was opened only in 1961, tremendous progress has been made particularly in the health sector. 90% of the populations now have access to basic health services.

The health care delivery system based on the Primary Health Care approach is delivered through a three tier system in an integrated manner. Both modern and traditional medicine services are provided under one roof.

At the community level are the Basic Health Units manned by health workers and provide basic health care including preventive and promotive services. Health services are further taken down to the village level through outreach clinics and supported by a network of village health workers.

At the district level are the District Hospital which provide higher level of services through trained health and medical professionals. They also serve as the referral point for the basic health units.

Larger and strategically placed hospitals function as regional referral hospitals where specialist care is available.

At the apex is the National Referral hospital which has most of the specialist and super specialist departments. Patients who cannot be treated here are referred outside the country.

There is no private medical system. Health care is provided free of cost at all levels.

Health Infrastructure

1	Hospitals	31
2	Basic Health Unit I	15
3	Basic Health Unit II	163
4	Indigenous Units	36
5	Outreach Clinics	519

Health Human Resource

1	Doctors	171
2	Nurses	567
3	Nurse Assistant	99
4	Health workers	506
5	Drungtshos (traditional medicine physicians)	36
6	Menpas (traditional medicine pharmacists)	54
7	Pharmacists	14
8	Laboratory Technologists	10
9	Technicians	528
10	Administrative and Support staff	1429

Some selected health indicators

1	Infant Mortality Rate	40.1/1000 live births
2	Under 5 Mortality Rate	61.5/1000 live births
3	Maternal Mortality Ratio	255/100,000 live births
4	Deliveries attended by health personnel	66%
5	Immunization Coverage	94.5%
6	Access to safe drinking water	83%
7	Access to safe excreta disposal	91%
8	Crude Birth Rate	20
9	Crude Death Rate	7
10	Growth Rate	1.3
11	Life Expectancy at birth	66.1

III. Recent Trends

The most significant development in Bhutan has been the introduction of parliamentary democracy and adoption of the Constitution in 2008 marking 100 years of rule under Monarchy and a smooth transition to a Democratic Constitutional Monarchy.

The Constitution of Bhutan enshrines that “the State shall provide free access to basic public health services in both modern and traditional medicines” (Article 9-Principles of State policy, Para 21)

The country’s development philosophy of Gross National Happiness (GNH) remains the guiding principle of all national development plans. Health is an important dimension in achieving GNH and therefore receives one of the highest priorities in resource allocation.

Other significant achievements have been

1. Universal Child Immunization
2. Elimination of Iodine Deficiency Disorders
3. Essential Drugs Programme
4. Leprosy elimination status
5. Polio elimination phase
6. Introduction of pentavalent vaccine
7. Establishment of the Bhutan Health Trust Fund

Recents trends in the health sector has been to:

1. Ensure access, equity, quality and sustainability of health services
2. Assure and protect the health of the people through promotive, preventive and rehabilitative services.
3. Advocate access to safe drinking water and sanitation for health
4. Provide safe, quality and efficacious drugs, equipment and other medical supplies and reduce wastage and promote effective and efficient utilization of healthcare resources.
5. Introduce evidence-based medicines, treatment interventions and technologies that provide cost-effective solutions to health problems through regular reviews and studies.
6. Preserve & promote traditional medicine that is based on rich culture and tradition and integrate with modern medical services.
7. Ensure mechanisms for motivated and dynamic health workforce
8. Facilitate and support the technical back stopping for decentralized activities
9. Provide efficient infrastructure management services
10. Harness IT to enhance the quality and accessibility of health services
11. Promote, regulate and harness Private Public Partnership for effective /efficient health care system

IV. Key Current Issues

- **Sustaining free basic health care services**

While it is a Constitutional obligation but in the light of improving quality, increasing cost, increasing demand and changing technology in health care how far can we go on providing free basic health care is challenge and issue that we are trying to address. Cost sharing or user fee introduction is one solution but with 23% of the population living below the poverty line and mostly in rural areas that form 70% of the population, the ability to pay is questionable. The Bhutan Health Trust Fund is another means to sustain the free health care system. Established in 1997, the BHTF is to ensure a permanent funding source to finance essential drugs and vaccines for the country's primary health care system. With a targeted capital of US$ 30 million, the proceeding of the fund would cover annual recurrent cost for vaccine procurement, essential drugs and medical supplies. The government is committed to match donor contribution on one-to-one basis. The operationalization of the fund has begun from 2003-2004 financial year and the BHTF has funded around US$ 100,000 for various immunization activities such as introduction of Rubella and pentavalent vaccines.

- **Improving Accessibility**

  While it is said that 90% of the population have access to basic health care services, 10% of the population still don't have easy access. This is due to the rugged terrain of the country and the scattered nature of the population which makes them hard to reach. Efforts are being made to increase accessibility through other means such as connecting remote health centres with telephones, improving referral systems by increasing ambulances.

- **Human Resource Shortages**

  Bhutan still has severe shortage of doctors, specialist, nurses, health workers and technicians. The limited capacity for production in the country as well as difficulty in getting placement for higher studies in other countries has been the cause of this perpetual problem.

- **Improving quality of services**

  The shortage of health workers and lack of proper monitoring, supervision and evaluation of health care services are major drawback towards improving the quality of services

V. Emerging Health Problems

- **Changing epidemiology and double-burden of diseases:**

  While still struggling with communicable diseases is now faced with an increasingly daunting and more expensive challenge of life-style related non-communicable diseases like hypertension, diabetes, heart and kidney diseases, cancer, depression, injuries and accidents. The enormous resources demanded for this double burden of diseases might jeopardize the current level of resource allocation accorded to the basic health servicesthe primary health care.

- **HIV/AIDS**

  Though Bhutan is still a low HIV prevalence country, the increasing trend of HIV infection is a big concern. From a report case of 2 in 1993, it has increased to 160 as of 2009. This rising trend in infection remains a cause of serious concern in the context of several risk factors that the country faces-close proximity to countries with high prevalence, high degree of mobility across the borders, commercial sex becoming more common, high occurrence of sexually transmitted infections, rising levels of substance abuse, low levels of condom usage and the youthful demographic profile of the population. Currently 4 children and 6 pregnant mothers are on ARV treatment. The rising trend of HIV infection is thus seen as a risk factor in not only maternal and child mortality but also drain a significant portion of resources thereby affecting progress on MDGs.

- **Rapid Urbanization**

  The proportion of urban population has increased from an estimated 21% in early 2004 to 31% in 2005. This rapid urbanization concentrating in the west and bordering towns in the south has posed undue pressure on the health facilities in the urban areas. Rapid urbanization has also contributed to increasing numbers of motor vehicles, road congestion, increasing road traffic accidents, unemployment and crimes. Increasing trend of alcohol, substance abuse and mental health disorders are other areas of concern.

- **Climate Change and Natural Disasters**

  Global warming and climate change has also made Bhutan vulnerable to threats from natural disasters. Bhutan is situated in one of the most seismically active zones of the world. A recent earthquake in the east of the country has resulted in loss of several lives and untold damages to property including schools and basic health units. Bhutan has also many glacial lakes, some of which are at risk for outburst like the Glacial Lake Outburst Flood in 1994 that left a trail of destruction to life and property in its wake. At the same time, global warming has caused the glaciers to recede rapidly threatening the country's water security and hydropower generation potentials which is Bhutan's foremost income generating source. Landslides and floods are quite common during the rainy seasons disrupting transportation and accessibility to health services.

VI. Inequity in Health

Inequities in health in Bhutan mainly arise out of access to health services as health care is provided free for all the people. Access maybe defined along 4 dimensions:

1. Geographical accessibility

Although physical access to health services is fairly high, 10% of the population still live within more than 3 hours of walking distance from a health facility.

2. Availability of health services

Little information is available on the quality of services provided. Quality assurance mechanisms and service standards are weak and there is no proper supervision and monitoring system. Shortages of female health workers negatively affect the demand for women oriented services and institutional deliveries.

3. Affordability of services

The government continues to provide free access to basic health services and also endeavours to provide security in the event of sickness and disability as enshrined in the Constitution. While affordability is not a major constraint, opportunity costs such as time due to long distance to travel, long waiting times, lack of transportation and ability to pay for it are some deterrents.

4. Acceptance of services

The willingness of people to avail health services both traditional and modern systems are generally high but certain gender sensitive issues and institutional deliveries are lagging due to non availability of female health care providers or non women friendly health facilities.

VII. Responses and strategies in public Health education and training to address the issues related to inequities in health (including use of ICT)

The Royal Institute of Health Sciences (RIHS) was established in 1974 with the objective to train mid-level Health Workers (Health Assistants, Auxiliary Nurse Midwives, Basic Health Workers) to provide primary health care services to the people. Over the years with the unprecedented pace of development particularly in the health sector, training of other categories of health workers such as nurses and technicians were introduced. At the same time, as the pool of health workers increased, the need to update and up-grade them periodically became imperative and thus up-gradation course such as the Assistant District Health Officer (ADHO), Assistant Clinical Officer (ACO) and BHW and AN up gradation courses were initiated. In 2001, a 2 year part time Bachelor of Nursing conversion course for senior GNMs was started in collaboration with La Trobe University of Australia and successfully produced in 3 batches a cohort of 52 nurses with leadership and managerial skills making a difference in the provision of quality nursing care services to the people.

The Institute is the only health education and training institute in the country. It offers the following courses:

A. Pre-service

S.No	Course	Duration (years)	Award	Intake/yr
1	General Nursing and Midwifery (GNM)	3	Diploma	50 (30 female, 20 male)
2	Health Assistants (HA)	2	Certificate	25(15 female, 10 male)
3	Technicians*	2	Certificate	60

* 10 different categories of technicians: Dental Hygienist, Dental Technician, ENT, EYE, Pharmacy, Physiotherapy, Laboratory, Orthopedic, Operation Theatre and X-Ray. The intake number depends on the requirement of the respective departments and their capacity.

B. In-service

S.No	Course	Duration	Award	Eligibility	Intake
1	Bachelor of Nursing	2 (Part-time)	Bachelor Degree	Senior GNM	15-25
2	District Health Management	1	Diploma	Senior HA	10-15
3	Assistant Clinical Officer	1	Diploma	Senior HA	10-15
4	Up-gradation	1	Certificate	Senior BHW	15-20
5	Up-gradation	1	Certificate	Senior AN	20
6	Short courses	1-2 weeks	Continuing Education	All catego ries of health workers	

So far more than 2000 health workers have graduated from the Institute. They form the backbone of the health care delivery system in the country.

More than 300 trainees are enrolled in various training programmes. There are 25 full time faculty members and 20 support staff. Part time faculty are engaged from the hospitals and the Ministry of Health. The National Referral Hospital, district hospitals and basic health units are used are clinical and field practice areas for the students.

Responses and strategies to address the inequities

1. **Increasing Intake**

   Despites its built capacity of 200, the institute has increased the number of trainees to 300 to address the acute shortage of health human resource. Certain numbers of seats are reserved for female candidates to ensure gender sensitive staffing. Plans are also underway to increase the infrastructure capacity.

2. **Improving the quality of training**

   To produce competent committed and compassionate health workers is the mission of the institute. To achieve this end, major efforts have been in ensuring that the curriculum for the different courses are reviewed and revised periodically to ensure currency and relevance to the needs of the country. 80% of the faculty are with Masters degree and the faculty are continuously updated in their skills and knowledge with short trainings and workshops. Teaching and training methods and facilities are updated to incorporate best practices and student centered learning strategies.

3. **Introduction of new programmes**

   To have better qualified health workers, the institute will soon be launching Bachelors degree courses in Public Health and Nursing.

4. **Use of ICT**

   The use of ICT is still in the development stages both in teaching and service areas.

Country report - Cambodia
FOR
2009 SEAPHEIN ANNUAL MEETING

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Country Profile

Cambodia, with the territory of 181, 035 square kilometers, is located in South East Asia, bordered with Vietnam, Laos, Thailand and the Gulf of Thailand. The estimated population in 2004 was 13.09 millions, among which 48.3% were male (National Institute of Statistic 2005). The 2004 census also showed that the distribution of rural/urban population was very different with 85% was living in rural areas. This finding is consistent with the fact that most of Cambodian population is working in agriculture. An average size of a Cambodian household is 5.1 people. Regarding the literacy rate, male has higher rate than that of female (73.6% Vs 64.1%). However, female have higher life expectancy at birth then their counterpart (64yr Vs 58 yr). It is estimated that approximately 35% are currently living under the poverty line of US$ 0.45 per day (UNDP 2007/8).

Current Health Status

The current health indicators in Cambodia are still among the lowest in the regions. However, some health status improvements in some areas have been observed recently.

The country is successfully controlling the spread of HIV and expanding the provision of care and treatment to PLHIV in need of care and treatment. HIV prevalence among the general population has declined from 1.2% in 2003 to about 0.9% in 2006 with approximation of 67,000 people are currently living with HIV/AIDS (NCHADS 2007).

The 2005 Cambodia Demographic and Health Survey (CDHS) detected the decrease of infant mortality rate from 95 per 1,000 live births in 2000 to 66 per 1000 live births in 2005, and the under five mortality rate from 124 to 83 within the same study period (National Instatitute of Statistic 2006). However, the maternal mortality rate still remained unchanged with 472 deaths per 100,000 live births, which is comparable to the figure in 2000 (437 per 100,000 live births).

Poliomyelitis has been eliminated from the country through successful vaccination programs, as result Cambodia was certified as polio free in 2000 after 3 years of no cases of poliomyelitis reported (Department of Planning and Health information 2007).

Despite Poliomyelitis eradication, other communicable diseases such as diarrhea, acute respiratory infections (ARI), dengue hemorrhagic fever, malaria, tuberculosis, malnutrition, and other vaccine-preventable diseases still be the major causes of morbidity and mortality among children (Department of Planning and Health information 2007).

Not only among children, infectious diseases are also the main cause of hospitalization and death among adult population. Malaria is a good example for such diseases. Malaria affects both children and adults. It is estimated that malaria incidence rate is 6/1,000 population, whereas the case fatality rate of severe cases is 10.5% in 2005 (Department of Planning and Health Information 2006).

Tuberculosis is another health burden for Cambodia health care system. The incidence of all forms of TB is estimated to be 508/100,000 inhabitants and the incidence of smear-positive pulmonary form is 225/100,000 population. These figures are among the highest in the Western Pacific Region (WPR). The death rate is 95 per 100,000 populations (Department of Planning and Health information 2007).

Cambodia is also facing with growing concerns of non-communicable diseases. It has been found that 10% of adult population living in urban areas and 5% of those who live in poor rural communities were suffering from diabetes (WHO 2007)

There is no exception for Cambodia in case of the outbreak of communicable disease. The first poultry outbreak of H5N1 was in 2004 (WHO 2007) and then, Swine flue outbreak in 2009.

Table 1: Key health indicators for Cambodia

	CDHS 2000	CDHS 2005
Life expectancy (years)		
Male	58	64
Female	54	58
Mortality rates		
Neonatal mortality	37	28
Post-neonatal mortality	58	37
Infant mortality	95	66
Child mortality	33	19
Under 5 mortality	124	83
Maternal Mortality		
(per 100,000 live birth)	437	472

Source: NIPH, NIS. 2006. Cambodia Demographic& Health Survey 2006

Emerging Health Problems

Non-communicable diseases and injury appear to be increasing and may become the main cause of morbidity and mortality among Cambodian adult population in the future (WHO 2007). One disturbing example of these emerging health problems is road traffic accident.

In Cambodia, the increase of road traffic accident is believed to be due to the increase of the number of moto-vehicles, rapid socio-economic development and population growth. Alarmingly, road traffic accidents casualties and fatalities continue to increase even faster than that of the volume of road traffic and the population. Over the last 5 years, the number of accidents has increased by 50% and the number of fatalities has more than double (HOUR 2007).

Road traffic accident could be considered as a new humanitarian tragedy for the country as it kills on average 3 people per day in 2004, which was two times higher than the ASEAN average. With this fatality rate Cambodia has been ranked as the second highest in the region (Cambodia Road Traffic Accident and Victim Information System 2004). In response, the Cambodian Royal Government has set up the Cambodian Road Traffic Accident and Victim Information System (RTAVIS), which connect all different involved ministries and the Handicap International to collect strategic information, which will be ultimately used for the development of programs for road safety.

Inequities in Health

Cambodia has maintained economy stability since 1993. The country has enjoyed an average annual economic growth rate of more than 8% every year. This growth, on one hand, reduces the poverty by 10% to 15% and on the other hands, increases inequality on the socio-economic status among the population (WHO 2007). Despite these optimistic annual growth rates, Cambodia is still one of the poorest countries in the world. In 2006, per capita GDP was US$ 419 and in some rural areas, the percentage of population living under the poverty line is as high as 79% (WHO 2007).

The government of Cambodia has spent about a little more than 1% of GDP on health. The level of spending has reached US$6 per capita in 2007 (WHO 2007). However, it has been estimated that the total health expenditure per capita in 2005 was US$ 37, which corresponds to about 8% of GDP. Thus there is a huge gap between government health spending and actual health spending per capita. To minimize this gap, international donors have allocated about US$7 per capita in 2007 (WHO 2007).

Despite the funding from both government and donors, the largest portion of health care expenditure comes from out-of-pocket sources. What is worse, the out-of-pocket expenditure often goes to poor quality or private health service providers. Consequently, the burden of the out of pocket payments is becoming a substantial barrier to access to quality health services, especially among the poor.

Besides, within a household that has barely affordable wealth for health, the chance that women and children using those budget for taking care of their health is very small since their household may also have a long list of priority and health of dependants is not among the top. CDHS 2005 showed that only 75% of women of child bearing age reported getting money needed for treatment of their sickness and that the difficulty in accessing health services was greater for those living in the lowest wealth quintile (National Instatitute of Statistic 2006).

Therefore, it is accurate to state that the wealth of households is one of the key factors influencing the access to health care services among household members and that health inequity is inevitable given the fact that about 35% of the Cambodian population is currently living under the poverty line.

Responses and Strategies in Public Health Education to Inequities

Acknowledging of the existence of heath inequity, the policy makers, especially at the Ministry of Health (MOH) starts looking for strategies to bridge the gap. Those strategies are; improving the quality of public health services, reducing out of pocket spending and increasing the utilization of public health facilities and increasing access to health services among the poor.

With support from international assistance, the strengthening of the quality health services, including the rebuilt of health infrastructures, has been undertaken. MOH also allows public health centers and hospitals to collect user fee, which will become additional revenue for the staffs working at public institutions and consequently it will improve the quality of services.

Paradoxically, the introduction of user fee at the public health centers and hospitals may became an additional barrier to the poor whose access to health care have already been severely affected by the lack of out of pocket budget. This contradiction led MOH to develop its strategic framework for Equity Fund in 2003 (Ministry of Health 2003). The aim of the fund is to provide financial support to the poor for using public health services. In addition, Community-Based health insurance was also piloted among those who live just above the poverty line. It has been state that the health equity funds have effectively reached the poor and work best with strong management in improving the quality of health services. Consequently, health equity funds and community based health insurance schemes are being developed and scale up in Cambodia (Peter Leslie Anner 2008)

As far as the public health education is concern, not many things have been done specifically in response to the health inequity. However, a school of public health was established within the National Institute of Public Health (NIPH) and started providing MPH programs in 2007. The establishment of the School of Public Health, which is the first government institution to provide such programs and one among two of public health schools in the country, is very much in line with one of the government strategies focusing on human resource development.

Furthermore, one of the MPH track offered at the NIPH School of Public Health is health system development, which focuses more on health service management. This program may enables students, who come from provincial health departments, national programs, non-governmental organizations and hospitals, to improve the quality of services they routinely provide to the population.

The use of ICT in Public Health education and Training

The use of ICT (Information Communication Technology) in heath sector in general and in Public health education and training, in particular, is very limited. According to Department of planning at MOH, the use of computer software for health information is available only at the central and provincial levels (Department of Planning and Health information 2007). Due to this limitation, the International Classification of Disease has not yet been introduced to National Health Information Systems (Department of Planning and Health information 2007). There is no computer available at the health center levels since there is either no or not reliable electricity at the sites. Consequently, data collected from health centers had been manually compiled from all paper-based registries; outpatient consultation, patient records…etc by health center staff. This has been one of the main causes of the delay and poor quality of health information reported.

Currently, at the NIPH School of Public Health, some type of ICT has been used to facilitate the training. The website for the school, for instance, has been established and planned to be used as a forum for students, faculty members and the interaction between the two groups regarding their academic, as well as social matters.

Although the utilization of ICT has been seen as an advantage in public health education and training, its expansion is not so simple especially in Cambodia, where the Internet cost is very expensive and its coverage is also limited. In addition, the shortage of human resource in ICT sector is also another obstacle for the implementation of ICT in public health training in the near future.

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Country report - Indonesia

The development of recent public health issues, policies, programs both at national and international level influence missions, initiatives and activities conducted by some Public Health Institutes in Indonesia. In Indonesia, basically there are two types of public health institutes having graduate and post-graduate program in public health. First type is School of Public Health as an independent entity in running education for both graduate and post-graduate programs such as School of Public Health Universitas Indonesia, School of Public Health Airlangga University etc. Second type is under Faculty of Medicine, such as Public Health Study Program, Faculty of Medicine, Universitas Gadjah Mada, Public Health Study Program, Faculty of Medicine, Padjajaran University etc. The following are some Public Health institutes in Indonesia currently active as member of INDOPHEIN (Indonesian Public Health Education Institutes Networks).

Universitas Gadjah Mada – Yogyakarta

Public Health Study Program is under the Faculty of Medicine. Faculty of Medicine Universitas Gadjah Mada has committed to increase their professionalism in medical and public health education to became the world class faculty. In 2009, the Faculty of Medicine Universitas Gadjah Mada was ranked 106th in the world according to report by Times Higher Education Survey (THES) for life sciences and Bio-Medicine category. The Medical Faculty has been putting a lot of efforts into improving its quality and competitiveness in the area of medical sciences, public health and education to produce professional and competent health staff.


Prof. Ali Ghufron Mukti, is delivering a speech

At the end of the 2008, Prof. dr Ali Ghufron Mukti, MSc, PhD, the Head of Department of Public Health, was officially inaugurated as the Dean of Faculty of Medicine, Universitas Gadjah Mada. On March 2009, Faculty of Medicine conducted various academic activities such as seminars and workshops related to its 63rd anniversary

Public health study program in the Faculty of Medicine is designed to become a center of excellent for education, research and quality community service by applying processes of discovery, dissemination and implementation of theories through management of health resources efficiently and effectively in an effort to prevent illnesses and increase level of the health of the community.

During in the last year some activities have been conducted by public health study program these include: One of the majoring of public health study program, namely Hospital Management was CERTIFIED for ISO 9001: 2008 after two days of audit by PT SAI Global. We would like to thank our partners, colleagues, alumni, and students for giving continuous support in this endeavor. With this certification, our commitment to improve quality of hospital management and public health study program as a whole certainly is life-long for the benefit of our customers.

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Training on Human Resource Assessment was conducted in *Jakarta, 11-12 August 2009.*

Human resource is the most valuable asset in any organizations, including hospitals. The complexity and specificity of human resources working in hospitals require an appropriate assessment method applicable for hospital organization.

At the end of training, participants should be able to conduct human resources needs assessment.

Hospital Strategic Marketing training.

Hospital competition has becoming a growing concern since the last decade. An average, public and private hospitals increase 1.9-2% annually. In addition to hospital growth, the flow of patients in Indonesia to continuously seek care abroad indicates a high demand to receive high quality care. Therefore, it is a must for hospitals to understand and apply hospital strategic marketing. Topics on this training covers: implementation of hospital service marketing, service quality for hospital, customer relationship strategy, marketing tactics, integrated marketing communication strategy, marketing budgeting and audit, marketing intelligence and hospital marketing planning

Patient Safety And Clinical Governance held in *Yogyakarta, June 29 – July 11, 2009*

A paper-based case was used to describe the overall content of this module, followed by introduction of the topics through interactive lectures jointly delivered by experts and hospital practitioners (averaged 4-5 hours per topics). To comprehend the understanding and to enrich the learning experiences, group discussions and a 3-day hospital visit was applied to recognize patient safety problems and discuss its implementation, as well as to apply quality improvement principles to assess the implementation of clinical governance. Findings from the visit and feedback sessions were arranged on campus, inviting hospital representatives. For communicating patient safety to patient/community and organizations, students were given opportunities to practice and improve their interpersonal skills practice at the leadership laboratory.

Management of Training for the education and training managers in hospital

Management of Training for the education and training managers in hospital, Jakarta, May 4-6, 2009, in collaboration with the Centre for Health Manpower Education and Training, Ministry of Health.

Human resources are the most important asset in any organization, including hospitals. Consequently, continuous professional development of the hospital human resources should be assured, such as through quality training. Comprehensive mastering of Management of Training (MOT) is key to this success. At the end of the training, participants are able to conduct training needs assessment, develop instructional design, plan training, implement training, evaluate training, and undertake quality assurance.

Short Course Surveillance In Reproductive Health as an elective subject accredited by TropEd (European Union).

Some subjects offered in Public Health Study Program, Faculty of Medicine, Gadjah Mada University have been accredited by TropEd (European Union). It is the only study program recognized by Trop-Ed in Indonesia. Currently it seems only one Public Health Study program in Vietnam and Gadjah Mada University offer these elective subjects accredited by Trop-Ed in the region.

Type: Advanced Module In Reproductive Epidemiology
Duration And Dates: 2 Weeks 18 - 29 August, 2008
Course Venue (Optional):
Credit Points And SIT: 3 ECTS 90 Study Hours (36 Contact Hours, 54 Self-Study Hours): 24 Hour Lecture, 12 Hour Tutorials And Workshops In Computer Labs, 20 Hour Home-Work Assignments (Data Analysis), 20 Hour Report Writing, 10 Hour Private Reading, And 4 Hour Exam And Assessment.

Dr. Siswanto Agus Wilopo, MSc, ScD is delivering a lecture in a Short Course Surveillance In Reproductive Health

Indonesian University-Jakarta

UI was founded in 1849 and represents the most experiencing educational institution in Asia during that period. Additionally, Christian Eijkman (The Director of Javaneese Medical Doctor School-the earlier form of Universitas Indonesia) was awarded the Noble Prize in Physiology or Medicine in 1929. He was the first to point out a substance in the rice skin and later was to be known as vitamin B1. He was also awarded the Prize for his new of investigating and his method to control diseases caused by vitamin deficiency.

Recently, UI produces more than 400.000 alumni and continues its important role both nationally and internationally. Furthermore, it is our commitment to produce high quality of education system, global standard research activities and maintaining high standard of international academic research publications. UI has been collaboration with 35 countries and 165 institutions.

The Faculty of Public Health University of Indonesia (FKM UI) was established to provide experts in public health to handle problems related to public health in Indonesia. The idea was first mentioned by Dr. Mochtar who at that time was the FKUI's Head of The Public Health and Preventive Medicine. It was followed through by Dr. Sajono Sumodidjojo in the year 1964 by proposing the project to UI's Rector, FKUI's Dean and The WHO representative in Indonesia.

On 26 February 1965, with the Department of Education, Teaching and Culture's decree number 26, The Faculty of Public Health was formed under the auspice of The University of Indonesia. On March 13, 1965, The Preparation Committee for the establishment of The FKMUI was formed. The members were representatives from FKUI, The Department of Health, The Department of Education, Teaching and Culture, The Department of the Work-Force. Later on the Ministry of Education, Teaching and Culture issued a decree number 153/1965 as an amendment to the previous decree and declared the establishment of FKMUI on July 1, 1965.

As part of a Research University, FPHUI set its vision to be the Center of Excellent for public health education in Indonesia and to be one of the best public health educations in Asia through enhancing research, education, and consulting services.

Diponegoro University-Semarang-Center Java

The Faculty of Public Health of Diponegoro University never feel tired but always enthusiastic in contributing to the development and improvements of the Faculty of Public Health as an academic, research and community center institution. We intend to share our knowledge and dedication to the people of Indonesia as well as the whole world in a holly effort to improve health status of the people. Public Health Field has many challenges and treats as emerging diseases arise, new paradigm is popularized and new public health efforts are promoted. Therefore, we would like to the be the front leader in educating community, educating prospective public health officers and public health professionals who have high capability in skill, intelligent and initiative toward the current situation in public health issues and problem.

Padjajaran University-Bandung-West Java

Universitas Padjajaran (Unpad) was established based on the initiative of prominent society members of the West Java who eager to have an higher institution at which young generation of West Java can be prepared for serving as future leaders.

After a throughout processes, on the 11th of September 1957, Unpad was established through the Government Regulation No. 37 dated on 24 September 1957 and officially opened by the President Soekarno on the 24th of September 1957.

At the time of its establishment, Unpad had only four faculties, but now it has developed to become one of the leading higher educational institutions in Indonesia with 16 faculties and 1 Graduate Programs. Academic programs offered are 9 Doctoral programs, 18 Master Studies Programs, 42 Undergraduate Studies Programs, 4 Profession Studies Programs, 26 Specialist Studies Programs, 1 Diploma (D-4) Programs, and 27 Studies Programs Diploma (D-3) Programs.

Department of Public Health Padjajaran University – Bandung – West Java has conducted many researchs in public health, examples of the research in 2009:

1. Nita Arisanti, dr.,MSc-FM *The Effectiveness of Face to Face Education Using Catharsis Education Action (CEA) Method in Improving the Adherence of Private General Practitioners to Recommended National Guideline on Management of Tuberculosis in Bandung, Indonesia, WONCA, Hongkong, 2009.*
2. Dr. Henni Djuhaeni, dr., MARS *Model of Health Insurence Demand Based on Economic Status, Attitude Tower Health Insurance and Satisfaction of Provision Quality*, The 7th World Congress on Health Economics, Beijing, China, 2009.

3. Irvan Afriandi, dr., MPH. *Clients' Perceived Quality on Methadone Maintenance Treatment Service in Indonesia*, The 7th World Congress on Health Economics Beijing, China, 2009.

School of Public Health Hasanuddin University

School of Public Health Hasanuddin University is a public institution which has prominent education and research especially in eastern part of Indonesia. It has 65 Lecturers, 20 administration staff, 7 departments. The vision of this School is to be the leader of public health institutions in Indonesia in 2015. Currently this school has approximately 700 students both undergraduate and post graduate program. The school now is trying to develop new paradigm of public health in teaching, research and community services.

Faculty of Public Health Airlangga University

Faculty of Public Health, University of Airlangga offers the following study programs:

1. Diploma in Occupational Health.
2. Undergraduate program in Public Health
3. Master program on Health Policy and Administration
4. Master program on Public Health
5. Master program on Occupational Health
6. Master program on Environmental Health

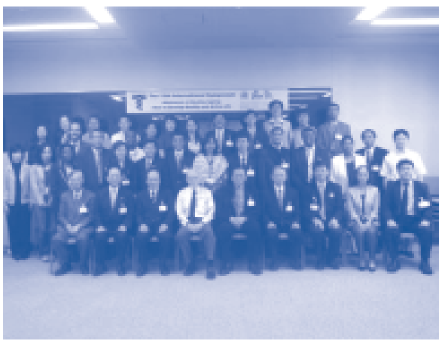
To enhance the quality of teaching-learning process and outcome of Faculty of Public Health, University of Airlangga is always putting some various efforts. One of them is that currently, the Faculty of Public Health, undergoing accreditation process under National Accreditation Body, Directorate of Higher Education, Ministry of Education, Republic of Indonesia.

Country Report – Japan
Tokai University

We, including RCIHD (Research Center for International Health Development), Department of Community Health, School of Health Sciences, in collaboration with HIAT (Head Office of International Affairs of Tokai) & TIGER (Tokai Institute of Global Education and Research) has conducted the following activities from August, 2008 to September, 2009.

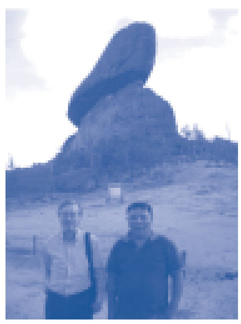
1) The 13th Training Course for Future Health Leaders (JICA Group Training "Future Health Planning based on Health Forecasting Method")

The training course was held from August 18, to September 22, 2008. For the training we invited 13 participants from 12 countries.(Viet Nam, Philippines, Cambodia, Indonesia, Thailand, Laos, Bangladesh, India, Palestine, South Africa, Lesotho, Kenya.) As a theme for the international symposium, we selected "Attainment of Healthy Ageing – How to Develop Healthy and Active Life?-, and had a wonderful discussion, with our foreign resource lecturers such as Dr. Hapsara, Ex-Director, Division of Epidemiological Surveillance and Health Situation and Trend Assessment (HST), WHO Headquarter, one of the initiators of this training course in 1990's, and Dr. Khwanchai Visithanon, 2007 ex-participant, Deputy Director, Bureau of Policy and Strategy, Ministry of Public Health, Thailand, and Ms. Anjana Bhushan from WHO-WPRO, and Dr. W. Aldis from SEARO.



2) Collaborated with ex-participant of the training course in 1999, to seek the possibility of improving of Primary Health Care System for NCD, Female Cancer Prevention, and Road Traffic Injury Prevention, in Mongolia.

Our team was dispatched to Mongolia and discussed with ex-participant of the training course in 1999, Dr. Otgon Amartsengel, Vice President of Ach Medical University to seek possibility to collaborate in primary and secondary prevention of lifestyle related disease and female cancer, and prevention of Road Traffic Injuries. Tokai University



3) Accepted Thai Mission on Long Term Care (mainly composed of Ministry of Public Health Officers) to visit TAMA NEW TOWN.

Thai delegation on long term care (LTC) paid visit to Japan to study the background and present status of TAMA NEW TOWN, Tokyo (Healthy City, planned in 1985), planned and built by use of BFT.

4) Tokai Delegation for Field Visit to Chang Mai, and Bangkok, Thailand, and discussion on Human Resources Development for LTC.

Tokai Delegation headed by Professor Ikuko Nakano, Dean of School of Health Sciences visited Thailand to see the reality and needs assessment of LTC, and discussed with staff of Nurse colleges on future collaboration in education.

5) Courtesy Call to Newly Elected Regional Director of WPRO, Dr. Shin Young-soo.

We visited WHO-WPRO to pay courtesy call to RD Dr. Shin Young Soo, and explained our activities to him as well as Dr. Henk Beckedam, Director of Health Sector Development. They agreed to continue collaboration

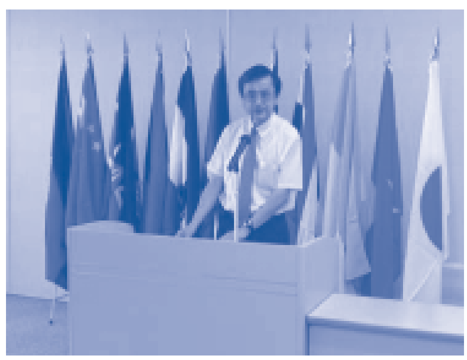
Country Report – Japan

6) Accepted Thai Mission on Long Term Care headed by Deputy Permanent Secretary Dr. Siriporn Kanchana.

Based on Japan-Thailand Economic Partnership Agreement, MOPH dispatched a delegation to study feasibility of long term care facilities and care provider education. We bridged MOPH desire and our school of Health Sciences and helped them to visit LTC facilities, Local Governmental Office, and to watch the demonstration of care giver education. They also visited our President, Professor Tatsuro Matsumae and Acting Chancellor, Professor Jiro Takano.

7) The 14th Training Course for Future Health Leaders (JICA Group Training "Future Health Planning based on Health Forecasting Method")

The training course was held from July 13, to August 17, 2009. For the training we invited 11 participants from 10 countries.(China, Viet Nam, Philippines, Cambodia, Thailand, Laos, Palestine, Lesotho, Kenya.) As a theme for the international symposium, we selected "Road Traffic Injury-Prevention and Countermeasures-". This year, Dr. Samlee carefully reviewed our programme and kindly selected and sent us Professor Myo Myint from Myanmar, Former Rector of Medical University 1, who has long been involved in RTI prevention activities. And from WPRO, we welcomed Dr. Krishnan Rajam. As other foreign resource lecturers, we welcomed Mr. Pascoe Kase form PNG (Head of Health Reforms, Department of Health) and Dr. Khwanchai Visithanon from Thailand In closing ceremony, Professor Dr. Uchida, Member of Board of Trustees of Tokai University Educational System and Executive Director of TIGER, provided his congratulatory message to the participants.



Country report – Nepal

Reducing inequalities in health through health system strengthening

Prof. N Jha,
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B P Koirala Institute of Health Sciences, Dharan, Nepal .Email:- niljha@yahoo.com

Inequalities in health exist in every nation on earth. Some variations, including biologically determined differences between men and women, are inevitable. But many inequalities are considered avoidable. Health inequalities exist largely because people have unequal access to society's resources, including education, health care, job security and clean air and water - all factors that society can influence.

Neither overall increase in economic growth nor gains in aggregate health indicators are reliable proxies for improvements in equity related development goals (Diederichsen 2001). Nepal is no exception; during the last two and a half decades, the country has experienced an average economic growth rate of four per cent. However, despite the fact that poverty alleviation programmes has been included as objectives in general macro-economic planning since the Seventh Five-year Plan , the incidence of poverty has not decreased (UNDP 2002). The number of people below the poverty line has doubled from 4.7 million in 1976 to a current 9 million. New pro-poor and development action therefore necessitates assiduous reflection and evaluation of former policies in order to guide the elaboration of future policies (Panday 2000).

The dimensions of inequality that matter most. The most traditional approach has been to think of differences in health status according to an individual's income or economic standing. However, the economic dimension is by no means the only one that matters, and some would consider other dimensions even more important. Gender inequalities in health status have received a great deal of attention in recent years

Federal Democratic Republic of Nepal is a landlocked country in South Asia and the world's youngest republic. It is bordered to the north by the People's Republic of China, and to the south, east, and west by the Republic of India. With an area of 147,181 square kilometers (56,827 sq mi) and a population of approximately 30 million, Nepal is the world's 93rd largest country by land mass and the 41st most populous country. Katmandu is the nation's capital and the country's largest metropolitan city.

Demographic Parameters

Population	29,519 Million (July 2008 est.)
Population density	200.6 persons per sq.km (2008 est.)
Population growth rate	2.095% (2008 est.)
Urban Population	3.9 Mio. (2006); approx. 16 % of total population
Growth Rate of Urban Population	approx. 7 % per year
Birth rate	29.92 births/1,000 population (2008 est.)
Death rate	8.97 deaths/1,000 population (2008 est.)
Infant mortality rate	62 deaths/1,000 live births (2008 est.)
Life expectancy at birth	total population: 60.94 years
male:	61.12 years
female:	60.75 years (2008 est.)

Total fertility rate	3.91 children born/woman (2008 est.)
People living with HIV/AIDS	61,000 (2001 est.)
Ethnic groups	Chhettri 15.5%, Brahman-Hill 12.5%, Magar 7%,
Tharu	6.6%, Tamang 5.5%, Newar 5.4%, Muslim 4.2%, Kami 3.9%, Yada 3.9%, other 32.7%, unspecified 2.8% (2001 census)
Religions	Hindu 80.6%, Buddhist 10.7%, Muslim 4.2%, Kirant 3.6%, other 0.9% (2001 census)
Languages	Nepali 47.8%, Maithali 12.1%, Bhojpuri 7.4%, Tharu
(Dagaura/Rana)	5.8%, Tamang 5.1%, Newar 3.6%, Magar 3.3%, Awadhi 2.4%, other 10%, unspecified 2.5% (2001 census)
Literacy (age 15 and over can read and write)	
total population: 48.6%	male: 62.7%
	female: 34.9% (2001 census)

Economy

GDP growth rate	3.2% (2007 est.)
Per capita GDP	$269 (2003)
Labor force by occupation	agriculture: 76%
industry:	6%
services:	18% (2004 est.)
GDP composition by sector	agriculture: 38%
industry:	20%
services:	42% (FY05/06 est.)
Industries	tourism, carpets, textiles; small rice, jute, sugar, and oilseed mills; cigarettes, cement and brick production
Population below poverty line	30.9% (2004)
Inflation rate (consumer prices)	6.4% (2007 est.)
Exchange rates	Nepalese Rupees per US dollar – 72.45 (2006)

TRENDS IN HEALTH STATUS

Life expectancy

Life expectancy at birth has been increasing for both males and females in Nepal. It has increased from 42 years for males and 40 years for females in 1971 to 60 years for males and 61 years for females in 2003. It is projected to increase to 62.9 years for males and 63.7 years for females by 2006 (World Health Report, 2005 and population projection for Nepal 2001-2021).

In Nepal, Health Adjusted Life Expectancy was 51.8 years with 53.5 years for male and 51.1 years for female in 2002 (WHO, Core Indicators 2005).

Mortality

Infant Mortality Rate

The Infant Mortality Rate has declined in Nepal from 140 per thousand live births in 1976, 64 per thousand live births in 2001 to 48 per thousand live births (Nepal Demographic and Health Survey, 2006). It is proposed to reduce IMR to 34.4 per thousand live births by 2017 (SLTHP 1997-2017).

Under-5 Mortality

The Under-five 5 mortality came down from 118 in 1997, 91 in 2001 to 61 per thousand live births (Nepal Demographic and Health Survey, 2006).

Maternal Mortality Ratio

Maternal Mortality has come down from 475 per 100,000 live births in 1997 to 281 per 100,000 live births in 2006 and is proposed to be reduced to 250 per 100,000 live births by 2017 (Health Information Bulletin 2001 and NDHS 2006).

Disability

The over all disability prevalence rate in Nepal is 1.63. It is more among the males. Disability is the highest (64.3 percent) among the working age group (15-59 years of age). A survey done in 2001 has shown that 57.6 percent of the head of households with disabled members had no education. Among the disabled, 22.2 percent of the economically active people were working in the agriculture sector. Among the disabled the following types are predominant:

1.	Mobility	19.5 %
2.	Speech	19.4 %
3.	Hearing	19.1 %
4.	Manipulation	14.8 %
5.	Epilepsy	11.1 %
6.	Mental Retardation	5.9 %
7.	Sight	5.6 %
8.	Psychopath	6.4 %

Disease is the main cause (30.3 percent) of all types of disabilities, particularly in case of disabilities in sight, mobility, hearing and metal retardation. Among the sight disabled, 10.4 percent are born with sight defects, 62.5 percent are due to disease and 10.4 percent due to accidents. Among the disabled people, 35.9 percent have hearing disabilities. Among the multiple disabled, 25.5 percent have hearing problems. The main causes of hearing disability are: congenital (57.4 percent), disease (26.4 percent) and accident (7.7 percent). Locomotor disability constitutes about 34.4 percent of which 19.5 percent is moving disability and 14.8 percent manipulative disability. The causes include congenital (27.3 percent), disease (36.0 percent) and accidents contributing to 25.5 percent of cases. In general 15.4 percent of all disabilities are due to accidents. Among the disables, about 70 percent went to some sort of healers (41 percent of them to doctors).

According to the Nepal Micronutrient Status Survey 1998, the overall prevalence of current night blindness in women of reproductive age and pregnant women was 4.7 percent and 6 percent, respectively, while 16.7 percent of women showed having night blindness during their last pregnancy. Furthermore, the rates were higher in rural areas. The highest rate of night blindness cases were seen occurring in the Eastern and central Terai regions (Ministry of Health, Annual Report 2002/2003).

For the 1st time in Nepal, bird flu virus has been detected in Kakarbhitta in the eastern district of Jhapa, bordering India. After the detection of bird flu virus, the government on Thursday [15 Jan 2008] decided to cull birds within the range of 3 km [1.8 miles] from the site. About 10,000 chickens, birds, ducks etc has been cluttered.

Katmandu, June 29 Monday, reported its first case of swine flu with three members of the same family testing positive. The health ministry called a press conference to indicate that a 48-year-old man, a 38-year-old woman and an eight-year-old boy had been found to have contracted the Influenza A(H1N1) virus. July 2, three days after Nepal reported its first cases of swine flu, health officials Thursday said the Influenza A (H1N1) virus has also been detected in two more. A non-resident Nepali couple returning from Washington to Katmandu via Doha June 21 and their eight-year-old son were found to have high fever at the Tribhuvan International Airport. Meanwhile, the health desks set up in various check points along Nepal-India border to scrutinize the incoming persons of swine flu infection have been shut due to lack of manpower. Nepal and India have a long open border and thousands of Nepal's and Indians pass through the border without much scrutiny every day. Till now total 20 cases of swine flu has been identified, among them 19 Nepali and 1 Australian.

Diarrhea disease has been big health problems in Nepal. Recently, there has been outbreak in Jajarkot; a neglected district in western Nepal about 325 km west of Katmandu, has assumed epidemic proportions. It has spread to three other adjacent districts: Salyan, Dailekh and Sukhet. It said 88 people died in Jajarkot alone, the epicenter of the disease, while 10 were killed in the other three districts. Each and every household among 21 out of 30 VDCs have diarrhea patients. In Punma and Thimi VDCs of Jajarkot, nine people have lost their life in just a week period. Five persons died in Punma VDC and one each in Paiyun, Portang and Nayakwada VDCs. Contaminated water, lack of sanitation and medicines, and ignorance of hygiene fuels the disease every monsoon, creating havoc in remote areas. In 2007, there were 250 reported deaths due to diarrhea while nearly 35,000 had been affected.

Though the figures for 2008 are yet to be made public, it is feared that they are substantially higher. Nepal's Epidemiology and Disease Control Division puts 26 out of 75 districts in the high-risk category and 34 as medium-risk areas. More than 200 people have died and thousands people have fallen sick due to diarrhea epidemic in Jajarkot, one of the worst diarrhea hit district of far-western region. The outbreak of a diarrhoeal epidemic in some districts of the Mid-Western and Far Western Development Regions has yet to come under control. At least five to six persons are still dying of the disease in the districts every day, and the total death toll has reached 142, with 122 deaths recorded in Jajarkot district alone, according to official media reports. The causative agent of cholera, Vibrio cholera, has been detected in stool samples collected from Jajarkot district where more than 150 persons died of diarrhea in recent weeks and there is little hope of the epidemic coming under control within this week, which means the toll will only rise further. Lack of health awareness was a main reason why it remained uncontrolled, government authorities stated. The diarrhoeal epidemic also throws light on the performance of the mushrooming number of NGOs and INGOs that spend millions of dollars in the health and education sectors.

The saddest aspect revealed by the diarrhea epidemic is that the Nepalese are still vulnerable to waterborne preventable diseases like diarrhea. No doubt, lack of clean drinking water and health education as well as poor hygienic practices among the masses are the main causes of diarrhea and other communicable diseases in Nepal. This shows that the health as well as the education programmes launched by the government over the years has been ineffective in yielding the desired results.

Remedies for health inequities must come not only from the health sector but also from broad social policies that address potential health gaps related to equity e.g. by distribution of income and land. Conditions that contribute positively to, or work negatively against, the individual's health status can be viewed as layers of the individual's health environment, from general environmental conditions to personal life style factors. Health policies aimed at reducing society's disease burden and promoting equity in health can therefore potentially take different entry points with various degrees of effect depending on the root causes at work.

Establishing a consensus on societal values for policy may seem a daunting task, but, through international agreements, Nepal has already committed itself to health and health care policies with common equity objectives.

Nepal has adopted five Millennium Development Goals (MDGs) in the health reform plan. All adopted MDGs are characterized by their being avoidable and mainly amenable to primary prevention intervention schemes addressing social root causes of ill-health.

Application of a multi-sectoral framework for analysis of all relevant bottlenecks that must be overcome to meet the MDGs is therefore highly relevant when identifying appropriate and targeted policies. The multi-sectoral interventions should be undertaken alongside health system interventions. People do not live their lives according to sectors; health programmes must therefore be developed in a comprehensive way to achieve maximum effect.

In formulating health interventions aimed at achieving aggregate health goals in Nepal, such as the MDG targets on U5MR and IMR, it will be key for planners to take into account the overall social structure and related mechanisms affecting health.

It is important to discuss the range of different concepts pertaining to equity held by different population sub-groups (social, ethnical and religious), and to clarify points of consensus and disagreement where possible. This understanding of points of disagreement as well as consensus is needed to permit the meaningful communication required for effective social action.

Multicultural, multilingual and multiethnic representation and participation are essential components in design of the future health system in Nepal. Forums for dialogues should be established as part of reform planning aimed at identifying ways to reduce health inequalities and inequities in access to health care services.

People do not live their lives according to sectors; health programmes must therefore be developed in a comprehensive way to achieve maximum effect. Application of a multi-sectoral framework for analysis of all relevant bottlenecks that have to be overcome to meet the MDGs are therefore highly relevant when identifying appropriate and targeted policies. Supportive multi-sectoral activities should be undertaken alongside health system interventions.

Since health needs are diverse and differ by locality due to gender, socio-cultural, ethnic and ecological diversity, it is important to develop variants of information material and approaches in the Behavior Change and Communication package (BCC) which are sensitive to specific populations. Further, experience shows that locally produced health information materials have the greatest impact. Opportunities at the local level should therefore be encouraged and finances made available to provide or create these local intervention approaches.

Current availability of essential drugs is a key factor in creating a demand-driven development of local health services and thereby raising utilization of health facilities, especially in rural areas. Health care strategies to reach poor rural women are therefore not only required to provide services to convince women to demand the services. To generate the demand for health services necessarily implies the introduction of a multifaceted strategic approach, focusing on: the availability at all times of essential drugs to fit the needs of the catchments areas; training of health personnel to staff posts compliant with the health needs and demands of the women; the provision of infrastructure to physically ensure ability to reach the health facility and to reduce other costs such as transport; the creation of an enabling environment for women according to traditions, culture and local circumstances, for example, by training staff and developing manuals/guidelines for NGO/MOH employees working in IP locations. Governments need to know the population's health status and the burden of major disease in order to frame appropriate policies and determine interventions

Public health programmes that reduce health inequalities can also be cost effective. The case can be made to give priority to such programmes (for example, improving access to cervical cancer screening in low income women) on efficiency grounds. On the other hand, few programmes designed to reduce health inequalities have been formally evaluated using cost effectiveness analysis.

The health-research system is an important component of a country's health system. Collating data and gathering the best available evidence to inform future policies for health sector reform are just two of many activities that may be entrusted to it. WHO is collaborating with Member States, particularly developing countries, to strengthen their health-research capacity and systems. It is building partnerships and closely cooperating with other international organizations involved in health research, and funding agencies, scientific organizations, regional research forums, national research councils and civil society.

Within health systems, disease-control activities play a pivotal role: they contribute substantially to strengthening the system as a whole, while depending on it for their efficient performance. Access to essential medicines is one of the most cost-effective elements of modern health care. Physical infrastructure provides both the support structure for the provision of health services and a mechanism for the delivery of medicines and vaccines to populations in need.

There is persuasive evidence for some outcomes that reducing inequalities will diminish "spill over" effects on the health of society at large. In principle, you would expect that differences in health status that are not biologically determined are avoidable. However, the mechanisms giving rise to inequalities are still imperfectly understood, and evidence remains to be gathered on the effectiveness of interventions to reduce such inequalities.

Country report – Nepal

ICT application in Public Health Education & practice in PHC
Prof. N Jha,
Chief, School of Public Health & Community Medicine.
B P Koirala Institute of Health Sciences, Dharan, Nepal. Email:- niljha@yahoo.com

Health and Wellbeing

Health and health care though used interchangeably, need to be distinguished from each other for no better reason than that the former is often incorrectly seen as a direct function of the latter. Contrary to the popular conception heath is clearly not the mere absence of disease. Optimal health often confers on a person or groups, freedom from illness and the ability to realize one's potential to the fullest possible extent. Health is therefore best understood as the indispensable basis for defining a person's sense of well being.

Alma Ata Declaration: "Health for all" was introduced to global health planners and practitioners by the World Health Organization (WHO) and the United Nations Children's Fund (UNICEF) at the International Conference on Primary Health Care in Alma Ata, Kazakhstan, in 1978. Primary health care as envisioned at Alma Ata had strong sociopolitical implications.

Information and Communication Technology for Primary Health Care- including Telemedicine/ Tele health

The requirements of the rural health care infrastructure at the Primary level are both unique and complex, with its inherent set of strengths and weakness. Presently there is a vast nationwide public healthcare infrastructure already established in Nepal. This infrastructure can be fortified by augmenting them with ICT technologies and services to address primary healthcare more effectively. Apart from the public sector we have a large and diverse Private/NGO sector addressing healthcare delivery considerably. It is pertinent and important to take this into consideration in identifying the ICT requirements for strengthening primary healthcare such that the same may be suitably adapted in different settings with minimal changes.

Presently the focus on healthcare is mainly on curative medicine. It is important to take a holistic view and address promotional, preventive and curative healthcare. This needs to be further emphasized, especially in the private sector to ensure that the private/NGO sector participates more effectively in preventive and promotive healthcare for the healthcare programme to be effective. The healthcare delivery models could be different in different regions of the country based on the local factors. It is necessary to take cognizance of this factor in designing solutions.

The major elements of opportunities for ICT and other advanced technologies in the primary health care are:

- Telemedicine and Tele-health services
- Healthcare data management
- Information systems
- Appropriate data collection devices and analysis tools

- Appropriate and affordable biomedical equipment for grass root deployment
- Video/multi-modal conferencing and e-connectivity
- Appropriate legal and administrative framework

In this context ICT provides an excellent tool for enhanced public – private partnership in healthcare delivery and optimal utilization of the skills and infrastructure strengthening of Healthcare Infrastructure.

- It is possible to establish health care information systems linking all levels of health care centers for quick data collection, compilation and access for decision support.
- Telemedicine is establishing itself strongly as an augmented or supplementary method of healthcare delivery. Given the fact that Nepal has large healthcare infrastructure but expert doctors are not available at rural locations, telemedicine techniques needs to be used effectively to provide better healthcare in rural locations.
- Other countries except Nepal progressing fast in telecommunications including fiber, wireless and satellite sectors which can be fruitfully leveraged for healthcare information systems and delivery through telemedicine.
- Inventory management including the stock position, operational status of the equipments are important parameters for facilities management.
- Availability of this information would facilitate optimal utilization of resources.
- The availability of networks, connectivity and telemedicine framework would foster public-private partnership in very effective manner for health care delivery through telemedicine and improved access to the resources in the network. Recommendations on the requirements at different levels of Public Healthcare system

The requirements may vary in the process of technology adaptation with the latest innovations making the existing ones redundant. The recommendations are essential and desirable equipment and systems at different levels of public healthcare system looking at the present scenario and what is required for ICT enabled healthcare system.

1. Sub Centre Level:

   The common types of health problems dealt at Sub centre level are as follows:

   - Women's Health: Antenatal and Postnatal cases follow up of female family planning acceptors., Reproductive tract Infections (RTIs).
   - Children's health or minor illnesses in adults: Acute Respiratory tract infections, Diarrhea, injuries, skin infections, fever etc.
   - Adult Health – major diseases as a part of the National Health Programmes: Tuberculosis, malaria, Hypertension and Diabetes – an emerging problem.

2. The essential and desirable ICT systems for a Sub Centre

1.	Large amount of time is spent in collection, compilation and analysis of data manually resulting in inordinate delay. Health workers at sub centre levels are ill equipped for providing awareness and education programmes on	• Handheld computer devices for data collection can replace the present existing manual registers/books. • The Hand held computer devices can enhance data collection and compilation • The work plans for the health workers can be generated and provided on the handheld devices. • The hand held computer can update the database at the Primary health center PC periodically. • Where ever connectivity is available the hand held computer can update the PHC PC, real time after data collection by the health care worker, thereby reducing the time for data collection and data compilation at the PHC.

- It is desirable to have a PC at Sub centre for maintenance of local database, patient's history especially high risk groups e.g. pregnant women so that it can be accessed by health workers.
- The patients have to go long distances due to location and accessibility issues
- Also there is difficulties for the health workers with little training, poor diagnostic skills and inadequate knowledge of therapy
- It is necessary to provide the health workers at least with minimum facilities handheld devices, communication and connectivity facilities to contact the higher level centers through telephone and email etc.
- It is important to look at holistic solutions in adopting technologies and solutions for rural and field usage and specific technology/product development activities are required.
- Recommendations as regards to essential and desirable requirements at Subcentre level. The essential items indicate the minimum ICT requirements and the desirable items indicate the optimum requirements at various levels for effective utilization of ICT for healthcare in the public health system.

3. Essential and desirable ICT infrastructure at Sub-center

Essential:	Desirable:
• Handheld device for field data collection, compilation with algorithms for provision of basic care and data transfer capability with PC • A Telephone link, land line /mobile to PHC for consultation • Portable Digital weighing machine for weight measurement of babies and mothers	• A PC with Printer and Web enabled digital Camera • Digital Stethoscope • Digital Glucometer • Higher telecommunication Connectivity such as email facility to PHC • Handheld devices for healthcare education and training with multimedia capability

4. PHC level
   - Primary Health Centre is the focal point of the public health care system responsible for primary patient care, administration of public health programmes and MIS reporting.
   - It is the ideal place to acquire patient demographics.
   - ICT can be used for effective administration of public health programmes, inventory management at PHC.
   - Hence the PHCs need to be integrated into Healthcare Management Information Systems along with medical consultation and treatment through Telemedicine especially in the Government Health Care Systems.

5. Essential and desirable ICT infrastructure at Primary Health Center (PHC):

Essential:	Desirable:
• PCs with printer & Web camera • Digital Weighting machine • Digital Stethoscope • Digital ECG machine • Digital Glucometer • Diagnostic test kits (blood grouping/Rh/HCG) • Pulse Oximeter • Mercury Sphygmomanometer for recording and manual entry of Blood Pressure data or Digital Sphygmomanometer for digital transmission of Blood Pressure data • Video monitors for healthcare education and training	• Digital Microscope • Digital X-ray • Digital Ultrasound (Strongly desirable. May be a low resolution one)

Essential: • Video conferencing through web cameras or professional systems. • Connectivity to Community Health Center (CHCs), District Hospitals and State/tertiary hospitals. • Computerized Inventory Healthcare management information system	

6. Telemedicine for Primary Health Care
   - Telemedicine would be a supplemental and/or augmented alternate mechanism for primary healthcare delivery. It is recommended that telemedicine facilities be available up to PHC level.
   - Video conferencing is considered as an essential facility the basic facility for telemedicine. Tele-consultation through video conferencing would form an important component of telemedicine. It is known that 90% of the cardiac problems would not require surgery and could be addressed through tele-consultation and suitable management.
   - The Core Group recommends graded biomedical facilities for normal healthcare and telemedicine at different levels of public healthcare system.
   - Low Cost Biomedical equipment need to be sourced/ developed suitable for telemedicine use in rural conditions considering weather, power supply conditions, ease of operation and maintenance and affordability.
   - Telemedicine provides an opportunity for integration of public – private/ NGO medical infrastructure for quality healthcare delivery. The telemedicine systems may preferably be web enabled. In the long run, there could be facilities to choose from a number of available alternatives based on cost of service, quality, choice of hospitals and doctors.

7. Mobile and portable models for Telemedicine
   - Mobile hospital / mobile Telemedicine is a very effective way of providing Health Care to the rural and remote population including the emergency medical services during disaster management and relief.
   - Mobile medicine and mobile telemedicine systems in certain scenarios would be viable for healthcare delivery where the same system can be used at different locations.
   - Mobile telemedicine and Telehealth is very effective for eye care especially with regard to mass eye screening for related to commonmorbidities like visual sight problems, cataracts and blindness due to conditions such as Glaucoma and Diabetic Retinopathy.
   - Mobile systems can provide sophisticated facilities to rural locations on time shared basis.
   - When connectivity is available through satellites or wireless, it is also possible to provide telemedicine services linking to the tertiary hospitals. This needs looking into issues related to equipment for mobile systems, connectivity for mobile systems etc.

- Integrated/modular systems combining individual biomedical tools into integrated systems offers cost effective and compact solutions which would suitable for mobile systems and also for PHCs and Health Kiosks.

8. Biomedical Equipment Development

These equipments have to be appropriate for rural use at the centers with low power onsumption, ease of operation and maintenance locally, low cost and suitable to local weather conditions. Most of these equipments should be able to generate digital output so that these can be integrated for telemedicine. The Core Group also felt that it is required to develop these equipment at a fraction of the cost these are available now. Some of the individual equipment can be combined into integrated system with much cost reduction due to shared computing resources. Some of the equipment identified for development is:

- Digital X-ray
- Digital microscope
- Digital ultrasound
- Digital weighting machine
- Digital Stethoscope
- Digital ECG machine
- Digital Glucometer/strips/Insulin pump

The Core Group also felt that it is also needed to develop affordable patient care and monitoring systems for secondary and tertiary care hospitals. The above equipment only lists the basic equipment required for primary healthcare. It is also important to understand the requirements dynamically and take up development activities. The advanced technologies like MEMS, nano-technology and biotechnologies need to be exploited to develop techniques, devices and systems for medical diagnostics and cure, patient care and disease surveillance.

Country Report – Sri Lanka

Reducing inequalities in health through health system strengthening and ICT application in public health education and practice in Primary Health Care

1. Introduction- Health and socio economic development

Sri Lanka has achieved world recognition despite being a low middle income country in diverse fields, of international cricket, as the repository of Therevada Buddhism, for defeating one of the worst terrorist organizations in the world and for its outstanding progress in health and social development among the other countries in the region and indeed among some of the high income countries. Some basic indicators of health (Table 1) and social development (Table 2) show the gradual sustained improvement experienced in Sri Lanka1.

Table 1: Trends in selected health indicators

Year	Crude death rate	Crude death rate	Maternal deaths	Infant deaths	Neonatal mortality	Child mortality < 5 Years
	Per1000 mid year population		Per 10,000 live births	Per 1000 live births		
1960	8.6	36.6	30.2	57	34.2	-
1965	8.2	31.6	23.9	53.2	33.2.	-
1970	7.5	29.4	14.5	47.5	29.7	-
1975	8.5	27.8	10.2	45.1	27	-
1980	6.2	28.4	6.4	34.4	22.7	-
1985	6.2	24.6	5.1	24.2	16.2	23
1990	5.7	20.8	-	19.5	-	22
1995	5.8	19.9	2.4	16.5	12.5	20.9
2000	5.7	18.2	3.5	11.2 (2002)	-	18.8
2005	6.6	17.3		3.1	-	14

Ministry of Health, Sri Lanka 2006[1]

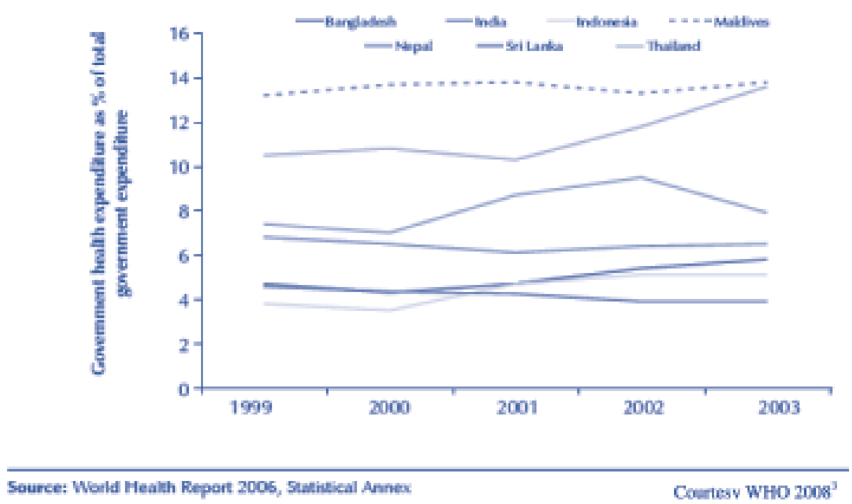
Table 2: Trends in life expectancy and literacy in Sri Lanka 1946-2006

Indicator	1946	1953	1963	1967	1971	1981	2001	2001/06
Life expectancy at birth in yrs								
Male	43.9	58.8	61.9	64.8	64.2	67.8	70.7	71.7
Female	41.6	57.5	61.4	66.9	67.1	71.7	75.4	76.4
Literacy rate								
Male	70.1	75.9	85.8	-	85.6	91.1		
Female	43.8	53.6	67.5	-	70.9	83.2	87.9	
Total	57.8	65.4	75.9	-	78.5	87.2	90.1	
Male-female difference	26.3	22.3	18.3	-	14.7	7.9	4.6	

Source: Census and Statistics 2008[2]

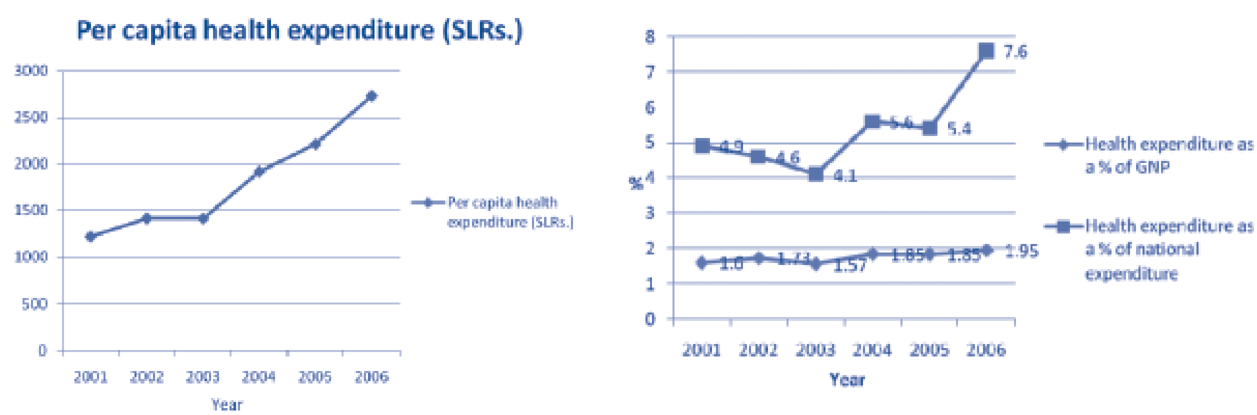
These are remarkable achievements in view of the health expenditure compared to other countries in the region (Figure 1)3, and given the expenditure on health over the years by Sri Lanka (Figure 2)[4].

Figure 1: Trends in expenditure on health in the region 1999-2003



Courtesv WHO 2008[3]

Figure 2: Trends in expenditure on health



Ministry of Health, Sri Lanka 2008[4]

The Millennium Development Goals (MDGs) which set the agenda for sustainable social development in the 21st century has incorporated health indicators. Sri Lanka is a signatory to the Millennium Declaration of 2000 and the MDGs have been included into the country's development plan and thus, MDGS have been accorded high priority. The health related MDG indicators and the bench mark values for indicators for 1990 with the target expected to be achieved in 2015 are shown in the data given in Figure 34. The performance of Sri Lanka is also exemplified in the high physical quality of life index (PQLI). Sri Lanka has not preformed as well in Human Development Index (HDI- 0.755), ranking 93rd among 177 countries in 2004, although an increase is shown from its values of 0.613 in 1975.

Figure 3: Health related MDGs, bench marks and targets -Sri Lanka

Indicator	Bench Mark (1990)	Target for 2015
Prevalence of underweight children <5 years	29.40%	20%
% population with minimal level of dietary energy consumption	36%	See[7]
Under 5 mortality rate	4.4/1000 under 5 population	3/1000 under 5 population
Infant mortality rate	12.2/1000 LB	9/100 000 LB
% of 1 year old immunized against meals	90.10%	99%
Maternal mortality ratio	47/100 000 LB	24/100 000
% births attended by skilled health personal	97%	99%
HIV prevalence among 15-24 years old pregnant woman	See Below[1]	-
Condom use rate of the contraceptive prevalence rate	3.70%	4.50%
Proportion of children orphaned by HIV/AIDS	See Below[2]	-
Prevalence and death rate associated with Malaria	See Below[3]	* to have halted by 2015, and begun to reverse the spread of Malaria
% Population in Malaria risk areas using effective Malaria preventive measures	See Below[4]	-
Prevalence and death rates associated with TB	See Below[5]	* have halted by 2015 & begun to revers the spread of TB
% of TB cases detected & cured under directly observed treatment short course (DOT)	73%	-
% Population using solid fuels	See Below[6]	-
% Population with sustainable access to an improved water source	93.90%	100%
% Population with access to improved sanitation	75.40%	100%
% Population with access to affordable essential drugs on a sustainable basis (% of hospital with access to 75% of the essential drugs on a sustainable basis)	75%	100%

Source: Ministry of Health, Sri Lanka 2008[4]

[1] Sri Lanka is HIV low prevalence country., and prevalence among Antenatal population is not screened for HIV

[2] Sri Lanka is a low prevalence country this indicator is not very relevant to Sri Lanka.

[3] The data on Malaria prevalence is not available in Sri Lanka. The total number of Malaria cases reported in Sri Lanka in 2001 was 27,791 and total number of deaths reported was 27.

[4] New indicator for Sri Lanka. Data on this indicators is not yet available.

[5] The data on the prevalence of TB in Sri Lanka is not available.

[6] The data on solid fuel was not available in Sri Lanka. The reduction of use of solid fuel is targeted through increasing the fuel efficacy.

[7] Global target is to halve between 1990 and 2015, the proportion of people who suffer from hunger. A value for 1990 is not available. The target for Sri Lanka by the year 2015 has to be decided.

* Global targets.

Sri Lanka has made significant progress in reducing mortality, and morbidity to some extent from communicable diseases (leprosy, typhoid, malaria and the childhood vaccine preventable diseases of polio, diphtheria, whooping cough, tetanus, measles etc.) aided by high literacy and public health programmes. The freely available outreach preventive and primary care facilities of the public health system and trained primary health care staff, both within a few km of any household have also contributed to the outstanding health indicators. Sri Lanka's progress in the achievement of the MDGs (Figure 3[4]), the indicators and the targets at the last mid term evaluation. It is clear that Sri Lanka has performed well and is on track in all targets and is likely to achieve the levels set for 2015 in time or ahead of time[5].

Sri Lanka also has shown a steady decline in poverty. In 1990, 26.1 percent was below the poverty line which dropped to 22.7 percent in 2002. Urban poverty declined faster from 16.3 percent in 1990/91 to 7.9 percent in 2002 than rural poverty, where the decline is from 29.4% in 1990/91 to 24.7 percent in 2002. The percentage of people below poverty line has fallen by 40 percent from 1990/91 during a 17-year interval to 15.2 percent in 2006/07 (5). According to the World Bank classification Sri Lanka entered the lower middle income category of countries whose per capita income falls in the range of USD 795- 3,125 in late nineties. However, as highlighted by the World Bank country director for Sri Lanka, 'despite an encouraging drop in poverty regional disparities remain between the wealthier Western province and the other region'.

Education is an important indicator of social development of the people. The government of Sri Lanka introduced free education to every citizen of the country from grade 1 to university education in 1946. This has resulted in commendable progress in the literacy rate of the population with a gradual narrowing of the gender inequality in education, which itself, is a significant achievement (Table 2). The national literacy rate increased steadily within a short period from 57.8 percent in 1946 to 87.2 percent in 1981. The female literacy rate increased at a greater pace reducing the gap between the rates of entry for males and females into the labour force at all levels. The national literacy rate in 1994 was 90.1 percent while rates for males and females were 92.5 percent and 87.9 percent respectively. By the year 2001, adult literacy rate (age of 15-24 years) has reached 95.6 percent, reaching 95 percent for male and 96 percent for female[6].

The intention of this paper is also to present inequalities in health and the role of health systems strengthening and ICT applications where relevant to address these disparities. As such no attempt will be made to judge the fairness or the rights of the existing gaps, differences and disparities, and the word inequality rather than inequity is used interchangeably[7].

2. Inequalities in health

In order to study health disparities it is necessary to analyze the indicators by groups of people who are identified to have different levels of underlying social and economic attributes(7). A closer look at the data disaggregated by socio demographic variables such as region, sector, gender, income, education, ethnicity etc. would highlight the determinants of the inequalities. In this section the data for some MDGs are summarized by different levels of such socio demographic attributes. In view of the constraints of space, I have selected MDG Goals 4 and 7 or this section using secondary data.

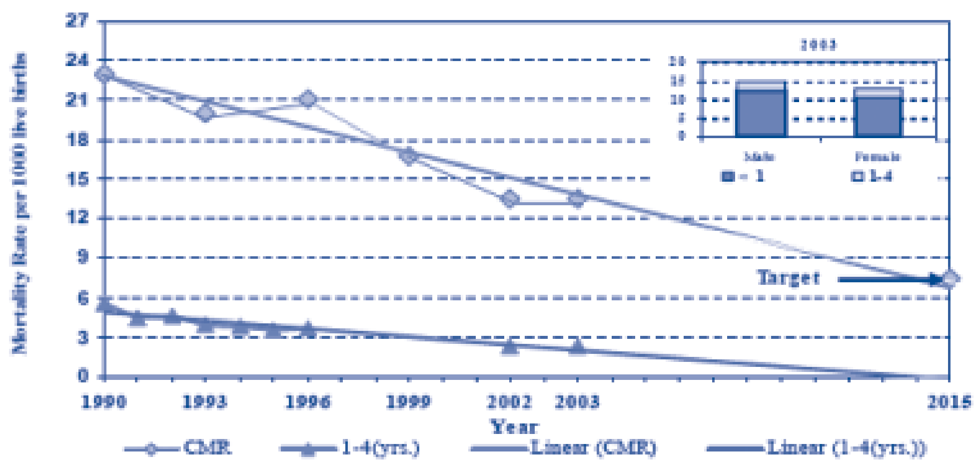
2.1 Underlying social inequalities in health

MDG Goal 4- Reduce child mortality

The target set for 2015 under indicator 13 is to reduce child mortality by two thirds between 1990 and 2015. Figure 4a shows the time trend with projected value for 2015, well on track[(5)].

An overall drop of child mortality by 40% since early 90s has been recorded. The regional disparities, sectoral differences are shown in Figures 4b and 4c, with children of North Central, North Western, Central and Sabaragamuwa provinces recording higher mortality.

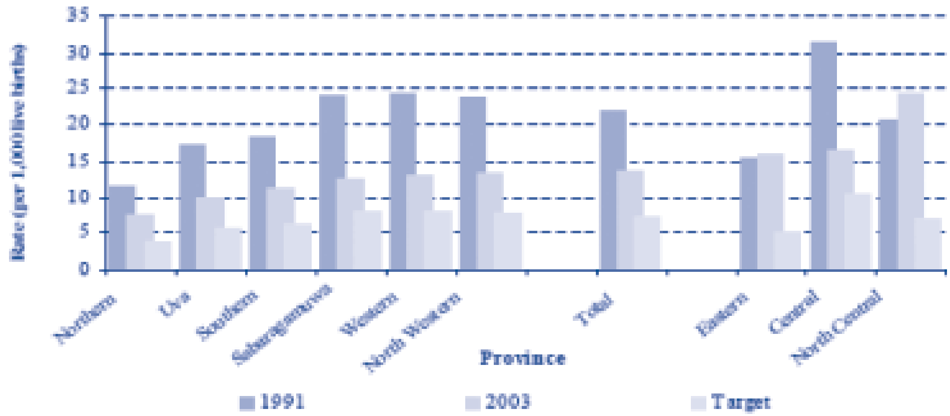
Figure 4a: Time trend for reduction of child mortality



Source: Department of Census and Statistics 2008[5]

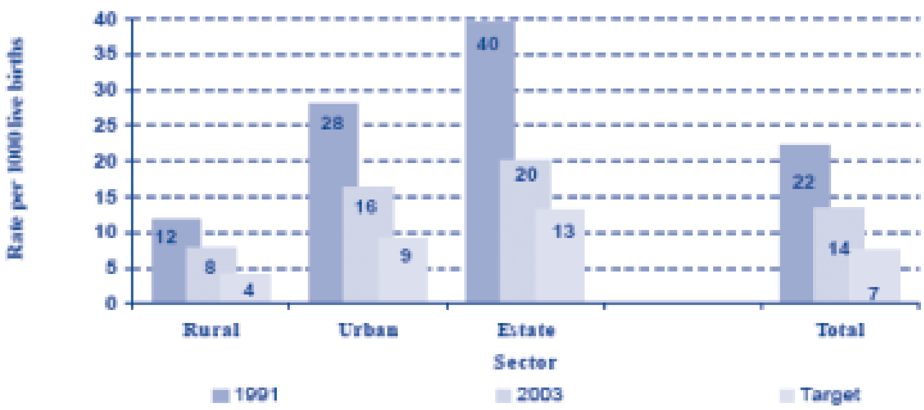
The child morality in estate sector remains the highest followed by the urban and rural sectors respectively. The estate sector has recorded the highest decline since early 90s.

Figure 4b: Time trend for reduction of child mortality by province



Source: Department of Census and Statistics 2008[5]

Figure 4c: Time trend for reduction of child mortality by sector

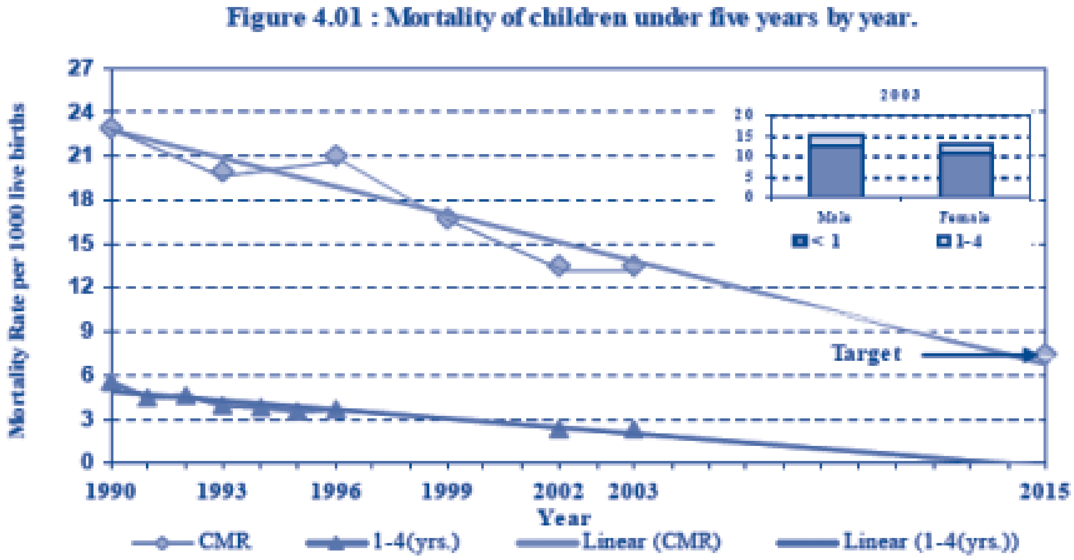


Source: Department of Census and Statistics 2008[5]

MDG Goal 4 -Reduce Infant mortality

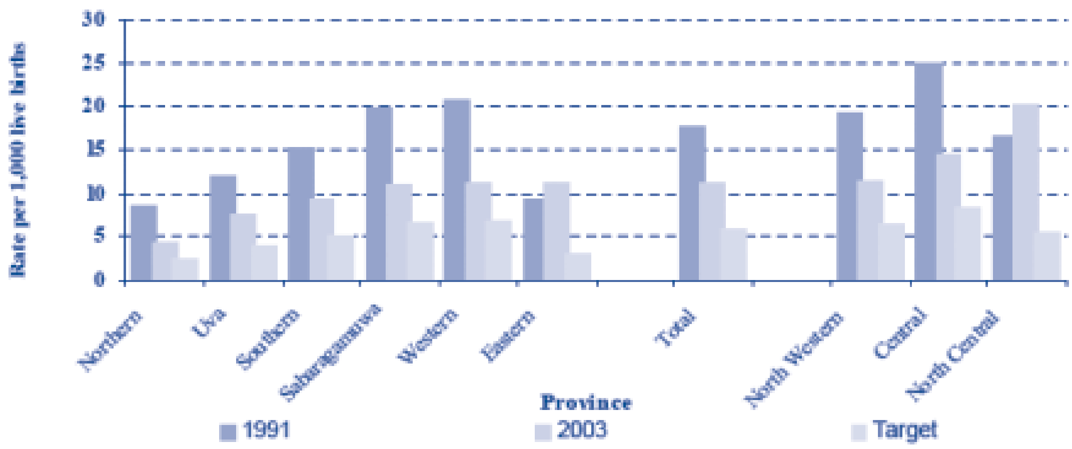
Infant mortality rate is 11.3 per 1,000 live births in 2003, and shows a one third drop since 1990 (Figure 5a)(5). Survival opportunities of infants have improved in virtually all the provinces with the only exceptions of North Central and Eastern provinces (Figure 5b)5. Mortality among infants is highest in Polonnaruwa district (27.6) of the North Central followed by Batticaloa district (21.1) of the Eastern provinces. In the twelve years from 1991 to 2003, a spectacular drop in infant mortality rate is reported in the estate sector reaching a slightly lower rate than that of the urban sector in 2003 (Figure 5c). The rural sector infant has the lowest risk of dying, although registration of infants from rural areas who are admitted to and die in urban sector hospitals and are registered as an infant death in urban sector has to be borne in mind. Infant mortality is seen to increase with the lower maternal education with sharp rise when the mother has not had more than primary education (Figure 5d).

Figure 5a: Infant mortality trends



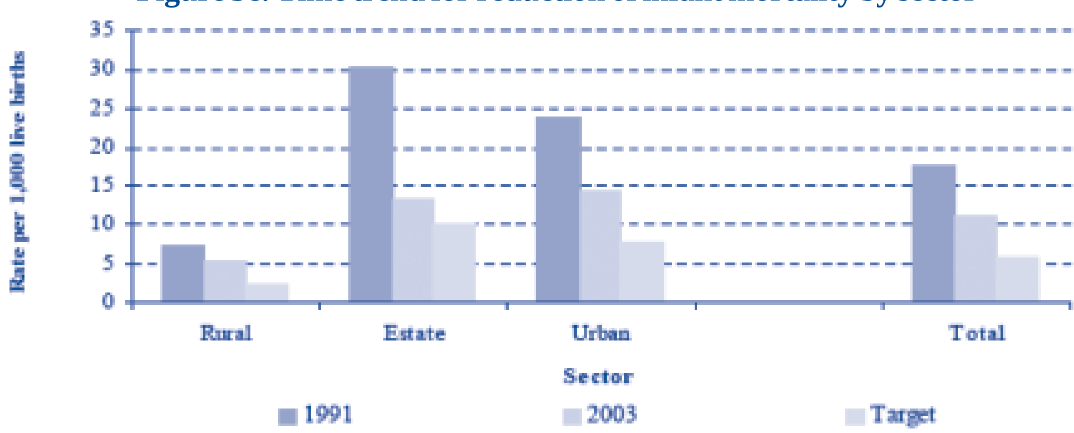
Source: Department of Census and Statistics 2008[5]

Figure 5b: Time trend for reduction of infant mortality by province

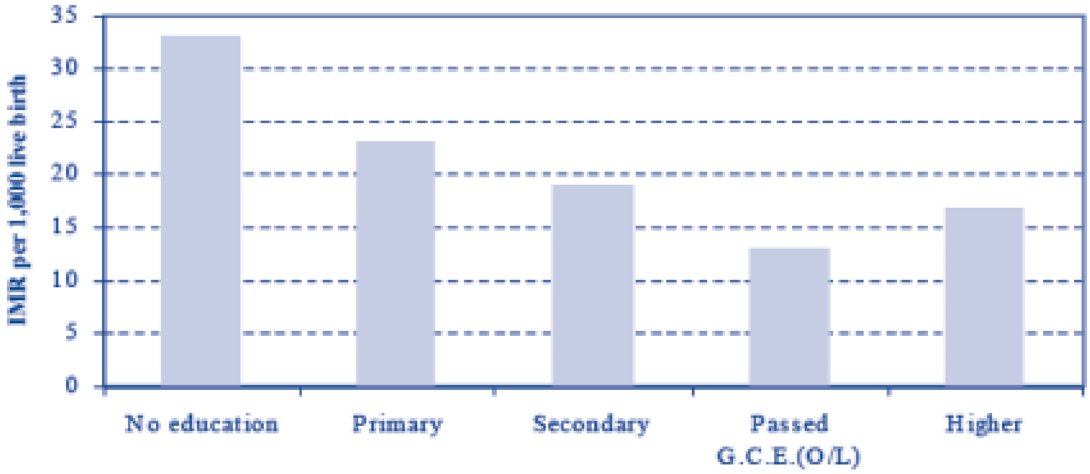


Source: Department of Census and Statistics 2008[5]

Figure 5c: Time trend for reduction of infant mortality by sector



Source: Department of Census and Statistics 2008[5]



2.2 Underlying social inequalities in health care

i. Pregnancy and delivery care

Self reported access to antenatal care has shown a slightly higher coverage for richest income quintile compared to the coverage of those in the poorest group, except in measurement of blood pressure (Table 3a)8. This may be due to the access to Public Health Midwife who carries out blood pressure measurements in the field. Importantly, trained assistance at delivery care is also available to a larger percent in all income quintile groups. A high proportion in all income quintiles have used public health (state sector) health services for delivery is the underlying reason.

Table 3a: Access to antennal care by income quintiles

Indicators	**Income quintiles**				
	Q1 (poorest)	**Q2**	**Q3**	**Q4**	**Q5 (richest)**
% Coverage of antenatal care (reporting a live birth in last 5 years)					
i by a health care professional (examined => 3 times)	57.7	53.4	56.6	55.2	65.3
ii blood pressure (examined => 3 times)	84.8	88.3	83.2	77.9	84.4
Iii blood sample taken	69.9	66.1	69.6	60.6	78.1
iv told of signs of pregnancy complications	67	78.4	72.7	74.6	82
Delivery care (received assistance at	91.8	96.6	92.7	85.3	88.4
delivery from a doctor nurse or midwife					
Place of delivery					
i. pubic health facility	81.8	93.3	91.3	85	72.7
ii. private health facility	1.2	1	.2	.8	18.6

World Health Organization[8]

However, more in the poorest income quintile utilized state sector health facilities while those in the highest income quintile have also used private sector facilities. The recent Demographic and Health Survey has also shown that 98.5 percent of births have been attended by a skilled health person i.e. a doctor, a nurse or a midwife[9].

Although rural sector has consistently fared less well in accessing antenatal care compared with the urban sector (Table 3b), trained assistance of a doctor, a nurse or a midwife has been available to them and have also used public health sector for delivery care. The state sector appears to play a significant role for the poorer income groups and the rural sector to access antenatal care. These contribute to the low maternal mortality indicators experiences by Sri Lanka. The scope for improvement in all services is also apparent from this data.

Table 3b: Access to antennal care by sector

Antenatal care- coverage (reporting a live birth in last 5 years	**Urban**	**Rural**
i by a health care professional (examined => 3 times)	63.6	57.4
ii blood pressure (examined => 3 times)	87.5	82.6
iii blood sample taken	75.9	68.4
iv told of signs of pregnancy complications	85.1	75.4
Delivery care (received assistance with delivery of birth	79.7	92.1
from a health care professional -doctor nurse or midwife)		
Place of delivery		
i. pubic health facility	65.5	87.2
ii. private health facility	22.4	2.9

World Health Organization [8]

ii Self reported coverage of non communicable diseases (NCD)

Unlike antenatal care the self reported coverage for NCD has not been satisfactory. Those in the highest income quintiles in general has had better coverage (Table 4a). The middle quintiles too appear to have relatively a poorer coverage.

Table 4a: NCD coverage by income quintiles

Self reported NCD coverage	**Income quintile**				
	Q1 (poorest)	Q2	Q3	Q4	Q5 (richest)
Angina	71.5	59.2	54.4	68.1	79.7
Osteoarthritis	47.2	51.3	45.7	54.1	62.9
Asthma	68.8	53.5	61.9	75.8	49.6
Diabetes	80.4	92.7	77.8	91.3	95.7

World Health Organization [8]

Rural sector has been benefited for NCD coverage more than the urban sector. Gender disparities with males showing a higher coverage is seen. Overall diabetes care has had much better access for all income quintiles, both sectors and sexes.

Table 4b: NCD coverage by sector and gender

Self reported NCD coverage	Sector		Gender	
	Urban	Rural	Male	Female
Angina	64.6	75.3	73.3	65.4
Osteoarthritis	6.4	51.3	50.1	54.6
Asthma	42.7	65.2	67.5	53.2
Diabetes	93.8	91.2	90.9	91.6

World Health Organization[8]

The striking difference in maternal care and NCD coverage seen above may be related to the fact that delivery if ANC has been successfully achieved through PHC approach whereas NCH has not been under the purview of PHC although the health care system is still free health –provided by the state sector.

2.3 Underlying inequalities in access to improved environmental sanitation

The three environment sanitation related indictors are shown in Table 5a[8] by income quintiles. The disadvantage faced by those in the poorest income quintile is seen for all thee indicators[8].

A similar disadvantage is observed among the rural sector too (Table 5b)[8].

Table 5a: Access to improved environmental by income quintiles

Access to improved sanitation	Income quintiles				
	Q1 (poorest)	**Q2**	**Q3**	**Q4**	**Q5 (richest)**
Access to improved drinking water					
Piped into household	4.7	6.8	17.8	39.2	74.4
No improved sources	50.3	47.3	37.5	21	6.8
Flush toilet to sewage system					
Other improved toilet facilities	0	1.9	1.0	5.9	24.4
Total improved sanitation	39.4	60.1	78.1	83.1	70
No improved sanitation	58.9	37.4	19.9	8.8	3.8
Use of fuel					
Electricity/gas	0	0.3	4.9	10.4	71
Solid fuel on an improved stove	0.3	2	7	10.1	4.6
Solid fuel on a traditional stove	91.8	88.6	83.1	64.2	18.3

World Health Survey 2006[8]

Table 5b: Access to improved environmental sanitation by sector

Access to improved drinking water	Urban	Rural
Piped into household	62.7	30.7
No improved sources	10.2	32.2
Access to improved sanitation		
Flush toilet to sewage system	20.8	0.7
Other improved toilet facilities	62.1	71.4
Total improved sanitation	83	78.1
No improved sanitation	14.7	20
Use of fuel		
Electricity/gas	59.9	18.9
Solid fuel on an improved stove	1.4	0.3
Solid fuel on a traditional stove	25.8	68.8

Thus, even though Sri Lanka has performed progressed well in health related indicators, disparities can be seen in disaggregated data in areas that have been discussed. The approaches to address these are discussed in the next section.

3. Addressing inequalities through PHC reforms and application of IT education in public health

3.1 Improving health systems responsiveness

There is evidence for the contribution made by free health services, the primary health care and other policies of social benefit to the high achievements in health and health care, and improving coverage and access to the less advantages groups. It is also clear that there is still much to be done to improve the health situation and the utilization by all sectors reducing the inequalities seen in the preceding examples. The World Health report 2008, 10 highlights the need for PHC reforms in four groups in keeping with the PHC movement and to meet the expectation of the modernizing societies. These recommendations are; universal coverage, reforms improve health equity, service delivery reforms to make health systems people centred, public policy reforms to promote and protect health of the communities, and leadership reforms to make health authorities more reliable.

Enhancing the health systems responsiveness to improve utilization through a patient centred approach is thus in line with this. Table 6 shows the people's view of the proportion of people who perceive the health system being responsive in the eight domains it is assessed.

Table 6: Patient assessment of inpatient care as responsive

Responsiveness do?mains	Income quintile				
(%) rating hospital inpatient service as 'poor'	Q1	Q2	Q3	Q4	Q5
1. *Prompt action -wait time*	41.1	46.5	48.8	31.7	24.5
2. *Dignity-talked respectfully*	22.4	23.6	28.9	21.1	12.7
3. *Communication*					
- clear explanation	25.4	24.5	31.4	29	22.4
- time for questions	41.3	44	40.4	35.5	30.6
4. *Autonomy*					
- treatment information	39	43.3	46.1	34	31.6
- involvement	35.6	44.3	43.8	35.5	41.3
5. *Confidentiality*					
- talk privately	25.4	39.9	36.8	40.9	39.5
- of records	32.9	31.4	41.5	36.1	31.3
6. *Choice of health care*	35.5	45.7	42.6	43.3	41.2
7. *Basic amenities-cleanliness*	36	34.7	37.4	45.7	36.4
8. *Social support –family visit*	27.2	29.2	38.9	39	41

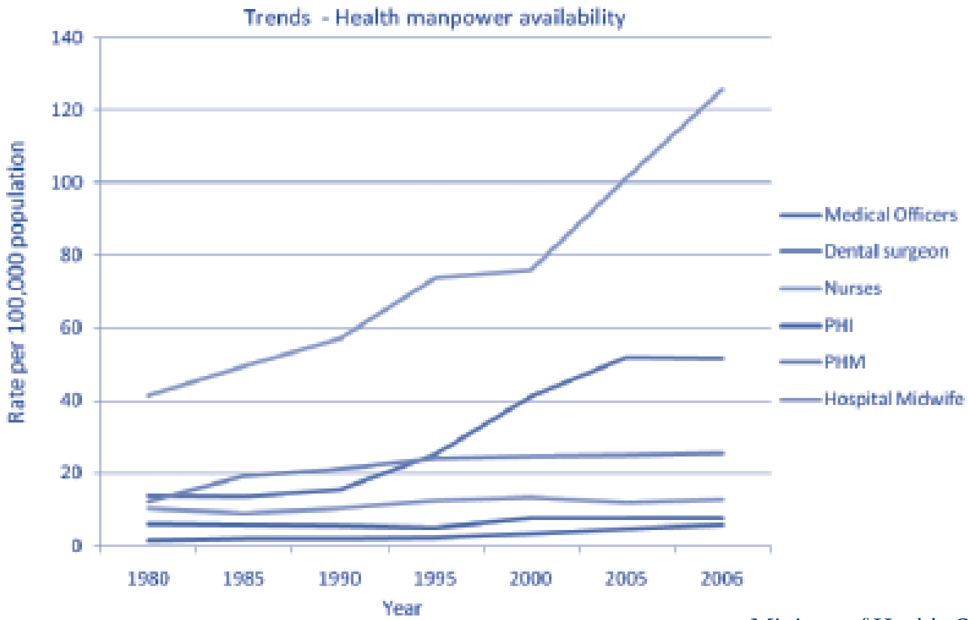
World Health Survey 2006[8]

In general the responsiveness is quite low for all domains and across all income quintile groups. It is slightly lower for the higher income quintiles perhaps due to the higher expectations of this group.

3.2 ICT application, in public health education and practice

Trained staff available and accessible has been a cornerstone in delivery of health care using the primary health care approach and to the progress achieved in Sri Lanka. Continuing professional development is almost non existent for many categories of PHC workers. It is not easy for the health system to release them for training. Thus an approach which provides them opportunity to undergo further competency development while still in the work place has many advantages and is very much in line with the principles of adult learning. The distance and open mode have been in use on a limited scale in Sri Lanka through the Open University which has undertaken training of nurses. It is also clear from Figure 6 that the trained staff fro PHC, such as public health midwives, inspectors, institutional midwives availability per 100,000 has not improved over the years. They are also among the lowest rates per population and needs to be improved. The scope for improving health through more trained staff can be utilized.

Figure 6: Health manpower available per 100,000 population



Ministry of Health, Sri Lanka 2008[4]

3.3 Could ICT be used?

A survey undertaken by the Department of Census and Statistics in 200 shows that only 8.2 percent f the households had a PC and 0.9 percent had a laptop computer. The proportion of households 5 -69 years who have used Internet in the preceding 12 months was 45.2 percent. The recently established centres at village levels and at administrative division at divisional level known as 'Vidatha' and by some NGOs as well as the private 'internet cafes' probably have been used. Slightly more males (%) of males than females (%), with higher levels of English literacy (57.3%) were more frequent users.

The availability of IT facilities for primary health care staff is not uniform and here too access is rather limited. Although IT is a useful effective and attractive form of adult learning with many advantages it is also necessary to make arrangements for them to develop the necessary skills. Supervision mentoring and tutor support too need to be ensured. As we look towards reforms to improve health systems in the current era of information technology, and of knowledge economy it is a worthwhile approach that we as member countries have to consider and develop.

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Scope for networking and collaboration among member institutes By Dr. Orapin Singhadej, Secretary-General of the Network for W.H.O. Collaborating Centers and Centers of Expertise in Thailand

Definition of networks

Networks are related organizations, which form or involve systems of interconnected actors or nodes and the links between them.

Common goals of networks

These actors interact and share resources in order to achieve common goals. These goals are knowledge sharing, innovation, knowledge development, inter-organizational capacity, joint problem solving, attribution of value, and sustained synergy.

Network structure

The structure of the network is more important than its characteristics and a network's structure can change over time.

Health service networks

Health service delivery networks are groups of three or more autonomous organizations working together across structural, temporal and geographical boundaries to implement a shared population health service or health services.

Our geographical boundaries can be extended because public health problems have no boundaries.Other types of networks are policy networks, public management networks and research networks.

Impact of networking

Knowledge Management (KM)

Knowledge Translation and Exchange (KTE)

Diffusion of innovations

Networks are particularly useful in the KTE of tacit knowledge.

Networks can foster innovation in the form of knowledge creation by developing new and more efficient services and by sharing their most effective and best practices within and between organizations and sectors.

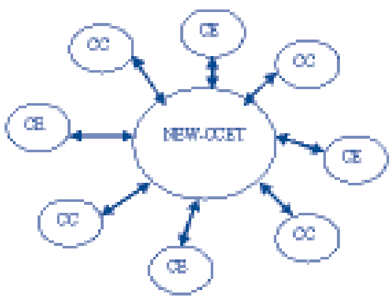
However, a network culture that is overly complex can lead to inter-organizational competition. Many countries have Master of Public Health international programs and this may lead to efforts being made to compete for enrolments by one program being presented as more attractive than another.

Types of networks

Different types of networks can be demonstrated by looking at NEW-CCET and SEAPHEIN.

NEW-CCET

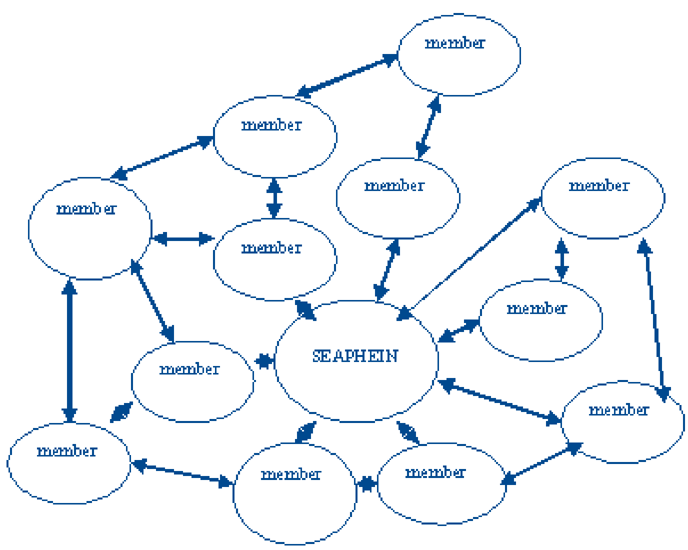
NEW-CCET represents organizations which although have largely unrelated areas of expertise, share the common goal of promoting the use of Thai expertise. The inception of NEW-CCET occurred concurrently with that of SEAPHEIN. NEW-CCET is also now five years old now and is comprised of 37 institutions and 33 national experts. Although, it was initially set up in the South East Asia region, I would like to expand it to include more countries such as India and Indonesia.



NEW-CCET acts as the central hub of a wheel to communicate and co-ordinate the similar interests of many organizations. The members of NEW-CCET center for communicable disease control, reproductive health and many other institutes that have their own specialist areas of interest. These members unite as members in this network to learn how to contact W.H.O. in both South East Asia and other regions and other potential developmental partners for support, training or for other kinds of development. I run the secretariat office of NEW-CCET. This is the Center of Education (CE) or public health education, health education, and also the specific Collaborating Center (CC) that runs HIV/AIDS, haemologic fever, as well as the clinical management of malaria and other diseases, as in Fig.1.

Fig. 1 Diagram showing NEW-CCET

Although SEAPHEIN is similarly comprised of organizations that share a common background and different areas of expertise, the individual entities within the network of SEAPHEIN also share a common goal. Networks also exist to assist individual entities in their specific pursuits. The SEAPHEIN secretariat's office acts as a central co-ordination hub and is located at the Faculty of Public Health, Mahidol University, Bangkok. SEAPHEIN members co-ordinate their activities by passing the central hub (Fig.2). This is the difference between NEW-CCET and SEAPHEIN.



SEAPHEIN has members from several countries in the South East Asia Region as well as in the Western Pacific Region. All members have their secretariat's office in Thailand. They can all interact for example, with India. Srilanka can interact with India, Indonesia or Japan. You may also interact with Lao and Kampuchea. SEAPHEIN members also have very common administrative structures. SEAPHEIN members are from several countries while all NEW-CCET members are Thai.

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Fig. 2 Diagram of SEAPHEIN

The nine principles for successful networking are as follows:

- Establish clear purposes and goals.
- Address the "hierarchy of needs".
- Establish a culture of trust instead of core values.
- Fulfill specific role functions.
- Maintain a flexible infrastructure.
- Establish supportive processes.
- Balance homogeneity and hetero-geneity among SEAPHEIN members.

Some countries, for example Indonesia, have institutes that have the same infrastructure and similar experiences. However, other countries such as Laos and Cambodia have only recently established their Public Health institutes.

- We have to balance sources of secure adequate resources. Having either a central secretariat's office or a member in WHO SEARO, makes an organization secure.
- You have to demonstrate your value to others before being recognized by others as being valuable.

Networking methods and tools

Back channel conversations are a secondary layer of private or shared conversations that allow more than one person to share their thoughts simultaneously. Since all member institutions now have representatives, any thoughts can be shared.

Brainstorming is a group method for generating ideas and sharing knowledge. You can brainstorm 'what should wedo next' for example, or 'what should be the next collaborate project among our institutions'?

Collaborative dialogue is evidence informed problem solving and decision making resulting from collective behavior. Everybody has to share in the dialogue.

Concept mapping is a visual tool to enhance the understanding of ideas.

Facilitation eases the process of guiding groups through areas of interest. One group may focus on further research while another may consider international training a higher priority. How do we go about it?

Technological tools

Voice Over Internet Protocol (VOIP) such as Skype, allows live international visual and audio communication at little to no cost.

Email distributions.
Discussion forums.
Social networking: Facebook, hi5, and twitter. These are all new technologies.
Advanced web tools such as wikis, blogs, webinars, and web casts, etc.

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Websites: Now everybody has their own website. In the NEW-CCET website, we have a collection or a database, which is the CV's of our experts. When anybody wants an expert in a specific field such as epidemiology, or H1N1 influenza for example, you can search, locate the expert, and then request a consultation with the expert of your choice. If you would ike to communicate between the websites of our institutes members, you can do this between SEAPHEIN and NEW-CCET. We can also share resources with other institutes that are very keen to study in some areas. How many other institutes are there in Thailand interested in the area of say, multi-center studies or multi-disciplinary research?

Other tools

Publications

Newsletters

Meetings

Teleconferencing is a cost effective tool for holding international discussions.

Evaluating networks:

The value and success of a network is often difficult to measure. The criteria for determining the effectiveness of a network varies both quantitatively and qualitatively. You have to revisit the objective of the network. Difficulties in evaluation are further complicated by the lack of consistency in the defining of networks as well as in the multiple practical applications of networks. Stakeholders and those who receive contributions from our network can be asked their opinions about how they are benefiting from the network.

Evaluation methods and tools:

Social network analyses:

Network maps that accurately reflect the network: Mapping can be drawn both geographically and objectively.

Identification of network leaders, content experts and mentors: Identify a network leader, a content expert, and a mentor. The network leadership is rotated among member organizations. Experts come at least once a year to NEW-CCET to share their expertise.

Identification of specific topics of common interest: A specific topic, such as health inequities for example, must firstly be identified and then the impact of SEAPHEIN or NEW-CCET on health inequities is evaluated.

Reasons members join, leave or become inactive: Involvement in NEW-CCET is often initiated by an institution, but sometimes involvement is initiated from an organization that will support the network. For example, six years ago, W.H.O. represented in Thailand by Dr. Samlee, started the idea that a collaboration between experts should precede work at the rural, national and regional levels. Consequently, both SEAPHEIN and NEW-CCET began to develop.

Member feedback:

Does the network effectively share information?

Are there opportunities for capacity development?

Are there opportunities for participation?

Identification of challenges and the strategies to address them:

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Evaluation methods and tools continued:

Member feedback: We can study and learn from member feedback. Our newsletter is a sufficient means of information sharing and also provides opportunities for capacity development. Many times the NEW-CCET and SEAPHEIN have provided programs for capacity strengthening for its member institutes. Although I receive comments from NEW-CCET members once a year, this does not provide sufficient participatory opportunities. However, the website can be used at any time to introduce ideas to everyone.

Sustainability: W.H.O. has stated that tools will be given to supporting organizations. However, such organizations may be expected to be self-sustaining after four or five years. As a member, you have to think about how to make your organization's membership sustainable. Dean Phitaya mentioned that there are six organizations would who have applied for membership to SEAPHEIN. We have to search for new members who are needed in the health service delivery management sector, namely in the fields of epidemiology, clinical management, as well as four in malaria, and also haemorlogic fever.

Growth of membership:

Value creation: You have to create your own value, as well as value for your money and your effort. Set specific and measurable indicators: You must have indicators for all planners, such as teaching and the planning of subjects.

Evaluation sources:

Membership list
Surveys
Anecdotes
Interviews
Focus groups
Website hits

Implications for network development:

Networks foster knowledge translation and exchange, and promote behavioral changes at individual and organizational levels. Occasional face-to-face interactions can provide further support and strengthen connections. It is important to determine the network's overall goals, membership and scope.

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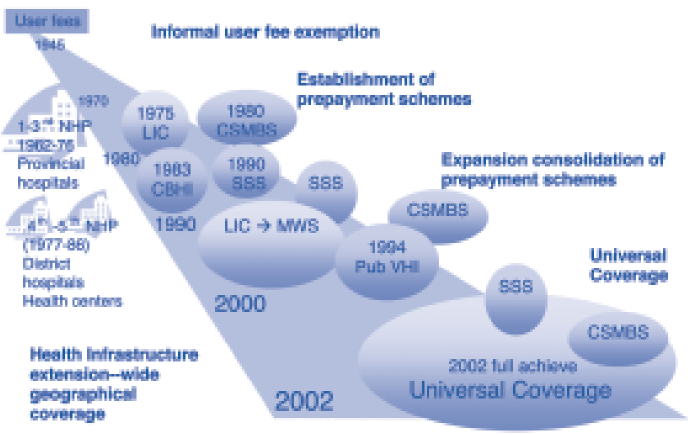
Health equity Achievements and challenges: lessons from Thailand
By Dr.Viroj Tangcharoensathien MD PhD.
International Health Policy Program

I will describe the 4-decade long march of health financing development and health system development, focusing on equity achievements before and after UC in 5 dimensions: equity in financial contributions, incidence of catastrophic health expenditure, impoverishment, and the distribution of public subsidies. This is advocated by the World Bank in the application of benefit incidence analysis.

I. Four decades of health development

Figure 1: Evolution of a chieving universal coverage in Thailand:



On the left panel is health system development since the first NHP – National Health Plan, the 5-year medium term plan, and plans 1, 2, and 3, which focus on Provincial Hospital development. In 1982, (no need to mention rural district hospitals or health centers), there were no evenly paved roads to access them.

This involves the period of time, before 1945 since the introduction of informal user fee exemptions. Then, the 1st to the 3rd National 5-year Health plans from 1962-76, development was emphasized on the most practical focus, the provincial hospital. From 1977 to 1986, during the 4th – 5th NHP, the government focused on the development of district hospitals and health centers. This was a profound policy and ideology focusing on the most disadvantaged populations in rural areas. These health centers play an important and vital role, that of PHC in rural areas. One health center covers a population of about 3000-5000 people.

The financial policy reforms started in 1975 where the Low Income Scheme (LIC) was launched by the Kukrit Pramoj government. This scheme was reformed and became the Medical Welfare Scheme (MWS) in the mid 1990's and covered not only the poor, but also the elderly, and socially disadvantaged groups.

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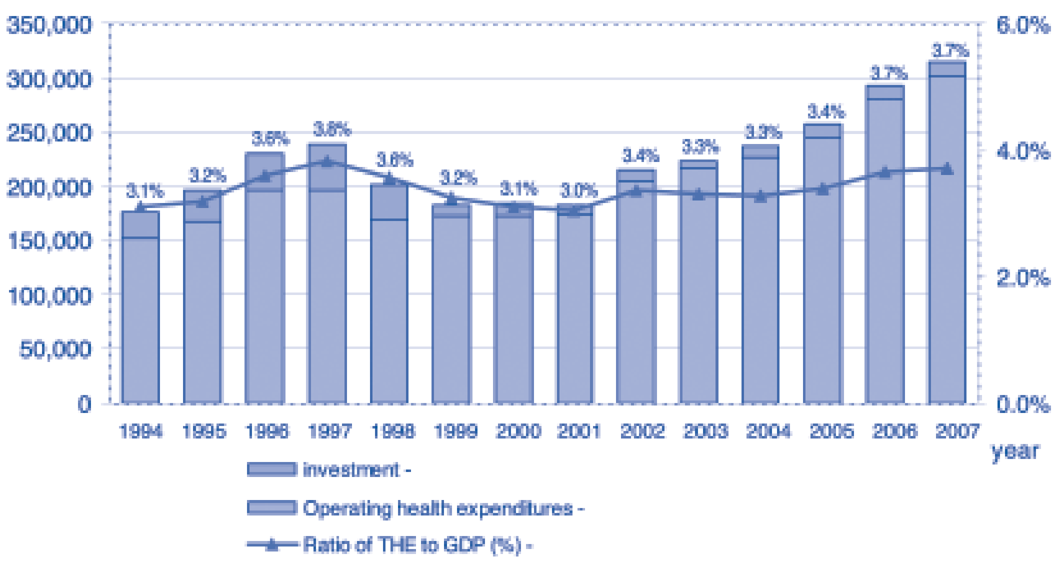
In the 1980's, the Civil Service Medical Benefit Scheme (CSMBS) was established and covered only government employees. Social health insurance or the Social Security Scheme (SSS) was launched in 1990. I was personally involved in the design of social health insurance, which led towards the capitation contract model introduced in 1991. It served as a stepping-stone for the design of the current universal coverage scheme. SSS covered 20+ aged workers, as well as smaller establishments of 10, 5, and more than a single employee in factories and companies.

In 1983, Community Based Health Insurance (CBHI) was established when Amorn Nontasut was the Health Minister, and before the declaration of Amata in 1987. My doctoral thesis was on the topic of CBHI. In 1994, when I was working at the Ministry of Public Health, CBHI was proposed to the cabinet. It was finally endorsed and became the public subsidized voluntary health insurance scheme. In this scheme, the government subsidized 500 baht and households paid another 500 baht in order to increase enrollments. This approach was started in 1975, when the government was strongly intent on covering the poor, but it wasn't until 2002 when the UC was achieved. This was the most important landmark of the millennium and not many countries have demonstrated the achievement of UC.

Table 1: National Health Account: selected indicators
1994 to 2007 (current prices) source: IHPP 2009

NHA indicators	1994	1997	2001	2002	2005	2006	2007
THE, Total Health Expenditure (Min Baht)	127,655	189,143	170,203	201,667	251,665	290,573	314,712
The as % GDP	3.5%	4.0%	3.3%	3.7%	3.6%	3.7%	3.7%
Public Financing Agencies (%)	45%	54%	56%	63%	64%	68%	73%
Private Financing Agencies (%)	55%	46%	44%	37%	36%	32%	27%
THE, Baht per capita	**2,160**	**3,110**	**2,732**	**3,211**	**4,032**	**4,625**	**4,992**
THE, USD per capita	**86**	**99**	**61**	**74**	**100**	**122**	**144**



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This Table and Figure represent the National Health Account (NHA) selected indicators. In 1994, we spent USD$ 86 per capita for health. This was dropped to USD$ 61 in 2001 due to the economic crisis. Total Health Expenditure (THE) per capita in 2007 was USD$ 144. Private financing agencies reduced their contributions by half, from 55% in 1994 to 27% in 2007. This was a most remarkable achievement. We know very well that when there is a higher proportion of out-of-pocket payments in total health spending, there is also a higher incidence of catastrophic health spending and a higher level of impoverishment from medical bills. Public financing agencies increased from 45% to 73% in 2007. THE was 3.5% of GDP at first, then increased to 4% in 1997, before dropping to 3.3%. This was then slightly increased and didn't stabilize until 2007.

The line curve in Figure 1 is the THE as a percentage of GDP. The operating health expenditure is represented by the blue columns, and the pink columns represent constant price investments.

Figure 3: Govt. and Non-Govt. Financing agent

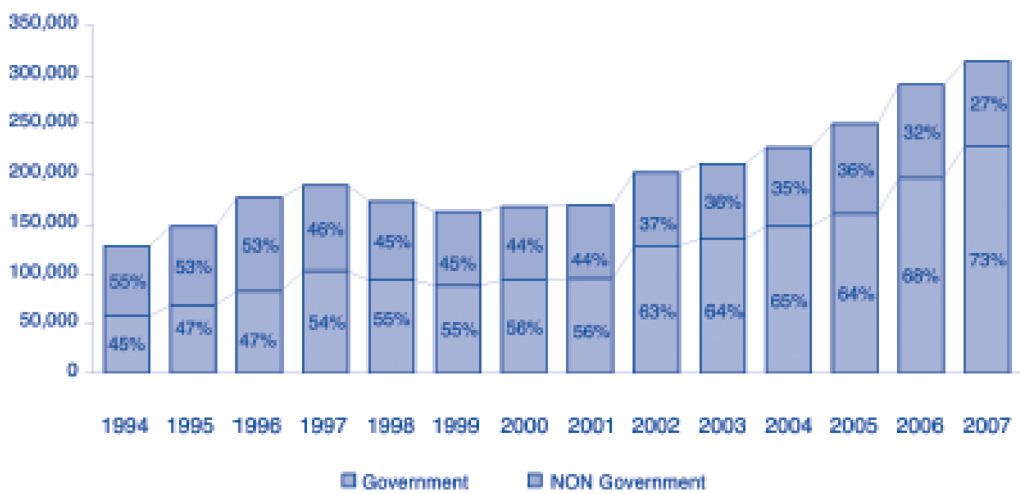
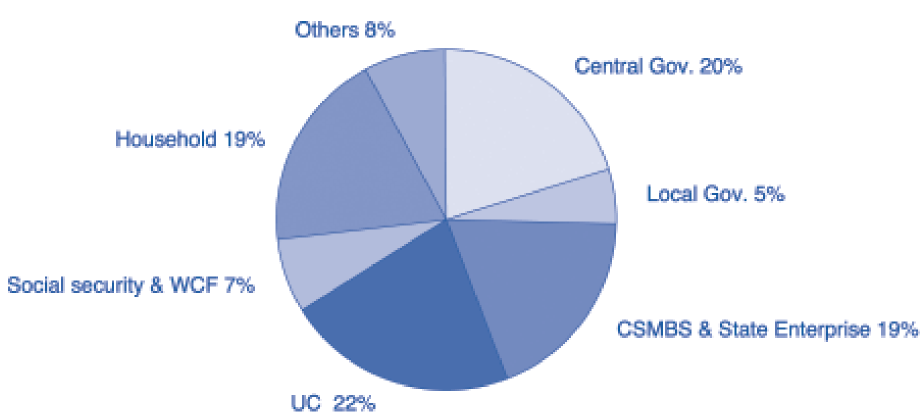


Figure 4: THE by financing agents, 2007 Source: IHPP 2009



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In Figure 3, government versus non-government proportions significantly increased. A sharp increase can be noticed from 2001 to 2002, 56% to 63% respectively, as a result of the universal coverage policy. The gradual increase up to 73% of THE in 2007 comes from government sources. This indicates a strong intention on policy and financial commitment by the Thai government to health sectors.

Figure 4 demonstrates THE by financing sources. Out-of-pocket payments by households were reduced to 19% and were much lower than the OECD average. Central government was at 20%, and the UC at 22%. We classify 22% as out. Actually, we have to combine central government, local government, CSMBS & State enterprise, UC, Social security & workers compensation funds as government sources.

There have been some concerns whether Thailand will be able to sustain its long-term financial capacity to continue supporting health sectors. We have conducted a 40-year projection using NHA, which proved that Thailand would be in good shape because of the provider payment design, which favors the close end method to which I will refer to later.

Figure5: Major flagship: UC Set foot 2002(1)

* By 2002
- UC introduced by terminating (a) low income scheme for the poor and vulnerable (b) voluntary public subsidized insurance scheme (health caard) and (c) include all the uninsured into UC umbrella
- Approximately 47 million (~75%) of total population who were neither beneficiaries of Social Health Insurance nor Civil Servant Medical Benefit Scheme are covered by UC
- UC : a tax financed national Health Insurance scheme

* Benefit Package of UC scheme
- Quite comprehensive and generous
- Comprising OP, hospitalization, health prom otion and disease prevention, most high cost health services, dental care, medicines na operations,
- Free at point of service with no copayment
- Medicines reference to national essential drug list, application of cost effectiveness well in place in drug selection into NEDL

Figure5: Major flagship: UC Set foot 2002(1)

* Health care financing strategies of the UC Policy:
- Removal of financial barrers to health services;
- Shift the main source of HCF from OOP to general tax;
- Change the provider payment methods from historical allocations to close-ended payments: capitation for OP, Global Budget + DRG for IP
- Promote the use of primary care, district health systems providers-district hospital + all health centres in the district is the main contractor provider, playing gatekeeper role,
- Proper referal to higher level of care is enforced.

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A Major flagship of 2002 stating that UC has set foot strongly was criticized by some World Bank colleagues who said it would fail or go bankrupt in 2 years. We survived that criticism by our World Bank colleagues. UC was introduced in 2002 by terminating (a) a low-income scheme for the poor and vulnerable (b) a voluntary public subsidized insurance scheme (health card) and (c) including all the uninsured of 2001 into the UC umbrella. By then, approximately 47 million, or 75% of the total population, who were neither beneficiaries of Social Health Insurance or the Civil servants Medical Benefit Scheme were covered by the UC scheme, which is a tax financed National Health Insurance scheme.

It comprises OP, hospitalization, health promotion and disease prevention, as well as the highest health cost services such as dental care, medicines and operations. It is provided free of charge at the point of service with no co-payment. Medicines refer to National Essential Drug List (NEDL). We have the capacity to conduct a cost-effective analysis enabling the prescription of the most cost effective drugs in the NEDL program.

Health care financing strategies of the UC policy:

We removed financial barriers to health services by shifting the main source of health care financing from out-of-pocket payment to general tax, and changed the provider payment methods from their historical allocations to close-ended payments: namely, capitation for OP, Global Budget and DRG for IP care. We also promoted the use of Primary Health Care, namely, district health systems providers whereby district hospitals and all district health centres become the main contractor providers. This is the first point of contact for UC members. This plays an effective gate-keeping role whereby proper referral to a higher level of care is enforced.

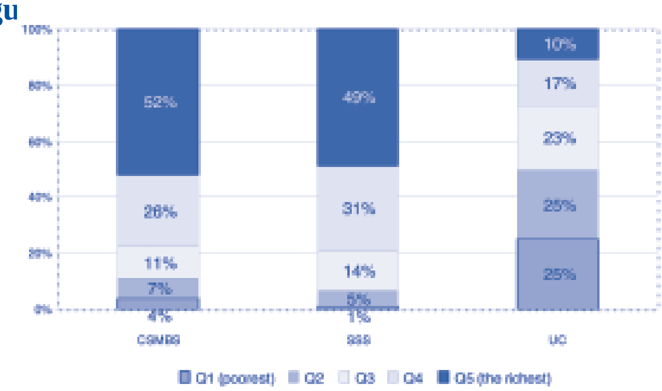
Figure 7: Characteristics of 3 public insurance schemes, after reaching UC, 2002

Insurance scheme	Population coverage 63m.		Financing source	Mode of provider payment	Access to service
Social Health Insurance	Private sector employees, excluding dependants	16%	Tri-partite contribution, equally shared by employer, employee and the government	Inclusive capitation for outpatient and inpatient services	Registered and private competing contractors
Civil Servant Medical Benefit Scheme	Government employees plus dependants (parents, spouse and up to 2 children age <20)	9%	General tax, non-contributory scheme	Fee for service, direct disbursement to mostly public providers	Free choice of providers, no registration required
Universal coverage	The rest of the population not covered by SHI and CSMBS	75%	General tax	Capitation for outpatients and global budget plus DRG for inpatients	Registered contractor provider, notably district health system

By 2002, there were 3 schemes, the Social Health Insurance (SHI), the Civil Servant Medical Benefit Scheme (CSMBS), and Universal Coverage (UC) for the coverage of 16%, 9%, and 75% of the population respectively. SHI applies a tri-partite contribution, while the CSMBS applies a general tax, which is a non-contributory scheme. This is a most inefficient system because the service provider's fee causes containment problems.

II. Equity achievements

Figu **004**



We used a number of national data sources for this analysis. This Figure shows the scheme beneficiaries by income quintiles. Fifty-two percent of the CSMBS members belong to the richest income quintile. Likewise, 49% of the SHI members belong to the richest income quintile, whereas only 10% of the UC members belong to this quintile. In contrast, 25% of the UC members belong to the poorest income quintile, and another 25% to the poor quintile. In other words, half of the 47 million people belong to the first and second income quintile, the poorest and the poor. This sends a strong message to the government that it must use the UC scheme, because half of the scheme covers the poor and the poorest income quintile, whereas only small percentages of these quintiles are covered by the CSMBS and the SSS.

Figure9: Kakwani indexs of different health care finance from 2000 to 2006
(Kawani = Conc. Index - Gini Coefficient)

	2000	2002	2004	2006
Out of Pocket	-0.1502	-0.0755	-0.0764	-0.0450
Direct tax	0.3913	0.4159	0.4424	0.3617
Indirect tax	-0.0964	-0.0691	-0.0435	-0.0831
Premium Insurance	-0.3623	-0.3906	-0.3233	na
Social heathi insurance Contribution	0.1650	0.1121	0.1046	na
Premium Insurance+SHI Contribution	na	na	na	-0.0491

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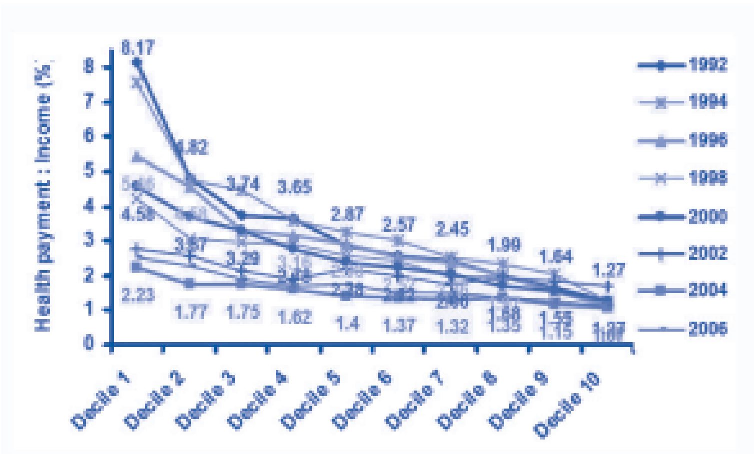
We applied the Kakwani Index (KI), which is the difference between the Concentration Index and the Gini co-efficient. This figure shows the KI for different sources of health finance. For example, in 2006, the KI for direct tax was 0.3617, which meant that the CI was much more skewed than the Gini co-efficient. The CI for direct tax contributions to health was more progressive than the Gini co-efficient, which is income distribution. The KI positive sign means equity or income re-distribution. That means when we use direct tax to finance health, it has some virtue of income re-distribution. It corrects the Gini co-efficient which is the income belonging to the richest quintile.

Figure 10: Household out-of-pocket payments (OOP) for health as % of household income, 1992-2006



Household out-of-pocket payments (OOP) for health, as a percentage of household income from 1992-2006, stretched across the income Decile 1 (the poorest) to Decile 10 (the richest). The gap between Decile 1 and 2 has gradually been reduced. This means that since 1992, the government financing reform of reducing the CBHI low income scheme and the SHI, has had a major impact on minimizing household out-of-pocket payments for health. This paper was published in the WHO Bulletin 2 years ago.

Figure 11: The incidence of catastrophic health payments from 2000 to 2007

	2000	2002	2004	2006	2007
Q1 (poorest)	4.0%	1.7%	1.6%	0.9%	1.9%
Q5 (richest)	5.6%	5.0%	4.3%	3.3%	2.8%
All quintiles	5.4%	3.3%	2.8%	2.0%	2.2%

Note: catastrophic health expenditure refers to household out-of-pocket payments for health exceed 10% of household consumption expenditure

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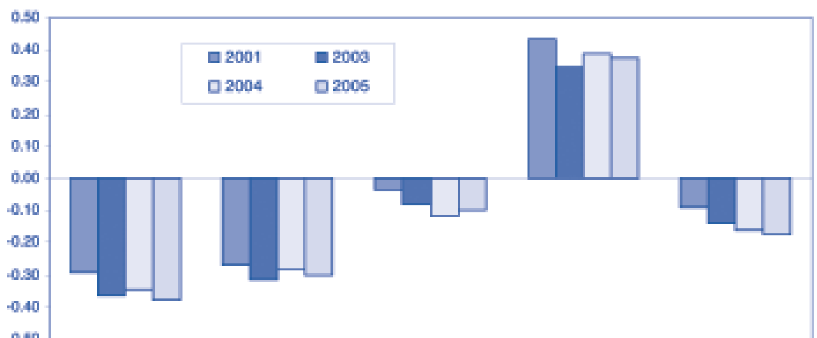
The incidence of catastrophic health payments was reduced after the launching of the UC in 2002 and continued through to 2007. In Figure 11, the data of all quintiles in the last row show that the incidence of catastrophic health payments was reduced from 5.4% to 2.2 % in 2007. This is a much lower percentage than the study in South Korea by Sun Man Kwon, which showed that national health insurance in Korea, is not doing well in terms of protecting its beneficiaries against catastrophic health expenditure. That means the benefits package is too shallow necessitating the substantial non-covered benefits being paid by the out-of-pocket members.

The catastrophic incidence among the poorest quintile is very low. Thailand was referred to in a WHO study published in the Lancet, as one of the best performing countries in terms of incidence of catastrophic health spending. Further, this study was published prior to a further reduction due to the inclusion of the recent coverage of dialysis into the benefits package. We all know that dialysis is the main source of catastrophic payment because patients have to pay almost 1,500 Baht, twice a week, and every year, either until the patients go bankrupt, die, or until there is no more resources from their social network available. Consequently, the IHPP had to provide evidence to have dialysis included in the benefits package of the UC scheme, which was at that time, two years ago, under the Prime Ministership of Surayuth Chulanont.

Figure 12: Equity in utilization: Concentration Index of OP service by type of health facilities: 2001 to 2005



	Health centre	Community hosp	Provincial hosp	Private hosp	Overall
2001	-0.294	-0.271	-0.037	0.431	-0.090
2003	-0.365	-0.315	-0.080	0.348	-0.139
2004	-0.345	-0.285	-0.119	0.389	-0.163
2005	-0.380	-0.300	-0.100	0.372	-0.177

Note : CI range from -1 to +1. Minus 1 (plus 1) means in favour of the poor (rich), or the poor (rich) disproportionately use more services than the rich (poor)

The graph in Figure 12 depicts equity in utilization in terms of the CI index for OP services. A minus CI favors the poor and a plus CI favors the rich. In Health Centers, the CI prior to the UC in 2001 favored the poor because it was mainly the poor who used Health Centers, Community Hospitals and District Hospitals. To a lesser degree, there was also service utilization in Provincial Hospitals. The lack of use of Private hospitals is understandable because patients have to pay. The overall CI is a minus, i.e. OP services were pro-poor in 2001, 2003, and 2005.

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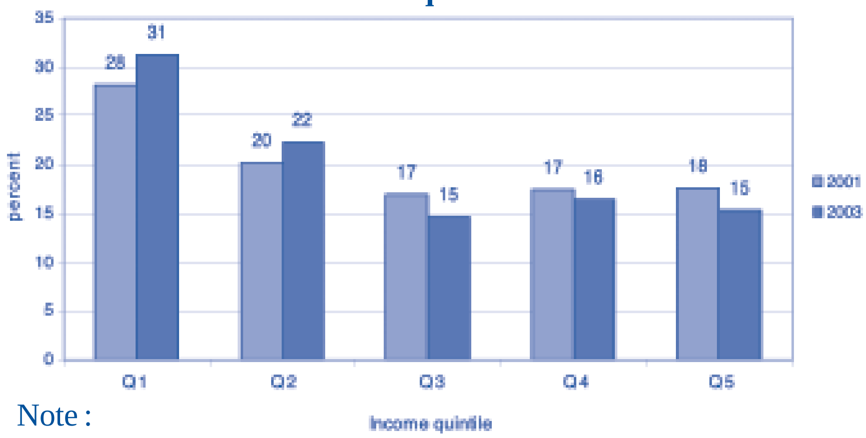
The data for admissions again demonstrates a pro-poor CI at Community Hospitals, District Hospitals, and Provincial and Regional Hospitals. It demonstrates a pro-rich CI in Private Hospitals as indicated by the plus sign. The overall CI's in 2001, 2003, and 2005 show a pro-poor dynamic, i.e. the poor used admissions more than the rich.

Figure 13: Equity in budget subsidies: BIA, 2001 and 2003
A comparison of percent distribution of net goverment health subsidies among different income quntiles in 2001 and 2003



Note :

- Overall net government health subsidies in 2001 were qpproximately 58,733 million Baht, and in 2003 were 80,678 million Baht (in 2001-value)
- The concentration index of government health subsidies in 2001 was -0.044 and in 2003 was -0.123

In equity in budget subsidies, we applied a Budget Incidence Analysis (BIA) when comparing 2001 to 2003. When we analyzed government subsidies on health gained by different income quintiles, we found that in 2003, the first and second quintiles gained a higher percentage of net government health subsidies, compared to the situation before the UC. In contrast, the proportion of the net government health subsidies in the third to the fifth quintiles, decreased after implementation of universal coverage. The concentration indexes of the net government health subsidies in 2003 had a higher negative value than the subsidy of 2001, which meant the poor gained more government health subsidies in 2003 than in the pre UC period. If the equity subsidies were perfect, all quintiles from Q1-Q5, would receive equitable 20% shares of government subsidies.

III. UC and MCH equity achievements

We applied the UNICEF Mixed Indicators Cluster Survey (MICS) round 3, which was conducted by the NSO in December 2005-February 2006. The samples included a total of 40,511 nationally representative sampled households.

Nine groups of MCH indicators including:-

1. Outcome indicators
    - teenage pregnancies
    - low birth weight
    - child malnourishment
    - child illnesses

2. Service coverage indicators
    - Family planning
    - Pre-natal care
    - Delivery
    - Child treatments
    - Child immunization

Each indicator was compared between

- The richest and poorest wealth quintiles/deciles
- Urban and rural domiciles
- Mothers/care givers education

A Concentration Index was produced to reflect inequities

- Range of CI from – 1 to +1
- -1 refers to concentration among the poor,
- +1 refers to concentration among the rich

There were 3 areas of inequities produced from this analysis: economic disparity, geographical disparity, and educational disparity.

Table 2: MCH outcome and service coverage Economic disparity

	MCH Health Outcome	**MCH Service coverge**
Economic dispartу	Child underweight (CI -0.2192, p<0.01) Stunting (CI -0.1767, p<0.01) Teenage pregnancy (CI -0.1073, p<0.01) U5 Suspected pneumoria (CI -0.0896, p<0.05) U5 Diarrhoea (CI -0.0531, p<0.1)	Distribution of the key interventions for MCH across economic strata is more uniform than health outcomes.

TableTeenage pregnancy **3: MCH outcome and service Geographical disparty**

MCH Health Outcome	**MCH Service coverge**	
Geographical disparty	* Except for low birth weight, all undesirable MCH indicators prevailed more among rural than urban residents: - child underweight, rural area - low birth weight, urban	* Small rural-urban gap. * > 97% had prenatal care, delivered in health facilities; * > 90% was immunization coverage for all antigens except hepatitis B

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The results in Table 2 show all negative CI's with a significant p-value indicating that these MCH health outcome indicators were prevalent among the poor. That means the poor are more prevalent on all 5 indicators than the rich. The MCH service coverage however, does not seem to be a problem. The distribution of the key interventions for MCH across economic strata is more uniform than the health outcomes. That means health outcomes are determined by factors outside the health sector.

The geographical disparity was between urban and rural areas. Except for the low birth weight health outcome, all the undesirable MCH indicators were more prevalent among rural residents than urban residents. For example, child underweight ness was more prevalent in rural areas, whereas low birth weight was more prevalent in urban areas. In regards to the MCH service coverage, there was a small rural-urban gap whereby 97% had pre-natal care delivered in health facilities, and more than 90% were immunized and coverage was provided for all antigens except hepatitis B. The differences between urban and rural areas in regards to MCH service coverage are low on the MCH list of health outcomes.

Table 4: MCH outcome and service coverage Educational disparity

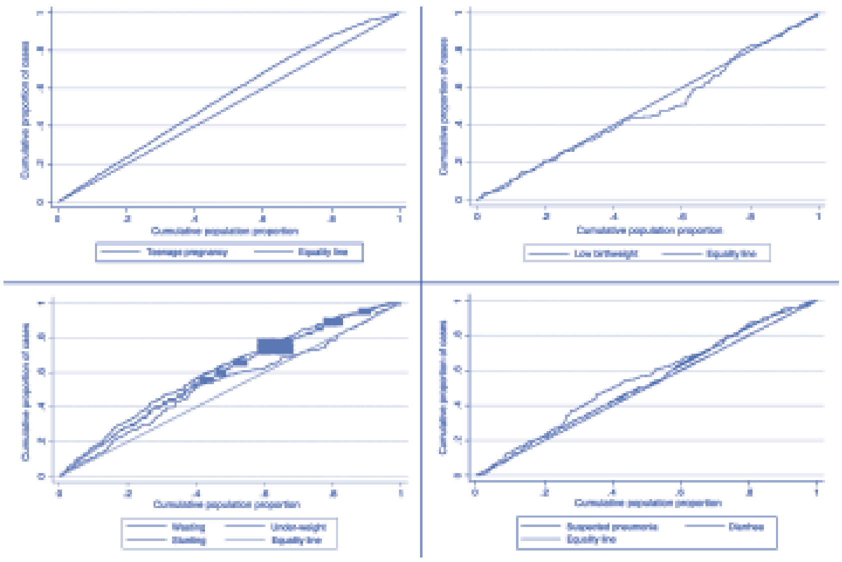
	MCH Health Outcome	MCH Service coverge
Educational disparity	Underweight, stunted, and wasting children were 71%, 61% and 48% respectively more likely to have mothers or care givers with no formal education	Surprisingly, children born to mothers who had been educated beyond the secondary level were 5-9% less likely than the uneducated to receive all types of vaccination (except MMR vaccine) befcre the first year of age.

Under weight : weight <2SD of the mean for same age, weight for age, weight for age
Stunting : height <2SD for the means for same age, height for age
Wasting : weight <2SD for the means for the same height, weight for height

In the area of educational disparity, it was found that underweight, stunted, and wasting children were 71%, 61%, and 48% respectively, more likely to have mothers or care givers who were not formally educated. This data is important to the Ministry of Education. Poorly educated and uneducated mothers contribute greatly to the ill health of their children.

In regards to MCH service coverage, surprisingly, children born to mothers who had been educated beyond the Secondary level were 5-9% less likely than uneducated mothers to receive all types of vaccinations (except the MMR vaccine) before their child/children reached the age of 1.

Figure 14: Concentration curves of MCH health outcomes



When the concentration curves lie above the perfect equity line of a 45-degree angle, this indicates that the distribution is more prevalent among the poor. The curve in the upper left graph shows that teenage pregnancies are more prevalent among the poor. In the lower left graph, stunting, wasting, and underweight ness, all lie above the perfect equity line, i.e. concentrated more among the poor.

The curves of the upper right graph show that low birth weight was emphasized in the third and fourth quintile, which are the middle class and rich groups. In the lower right graph, suspect pneumonia and diarrhea are more concentrated among the poor.

IV. Discussion

The health financing reform strategies of the UC policy accelerated equity in health care finance as well as in ambulatory and hospitalization health care use. These strategies demonstrated better financial risk protection, in terms of catastrophic health spending and distribution of government subsidies, especially to the poor.

Direct tax payment is the most progressive source of finance in Thailand. Out-of-pocket payments for health tended to be less regressive after the implementation of the UC policy. Health care use of Public Health facilities and the distribution of government subsidies were designed for the benefit of the poor before the UC, as well as the provision of better services after UC implementation. Health services at Primary and Secondary care levels were more pro-poor than those of the tertiary care and private sectors.

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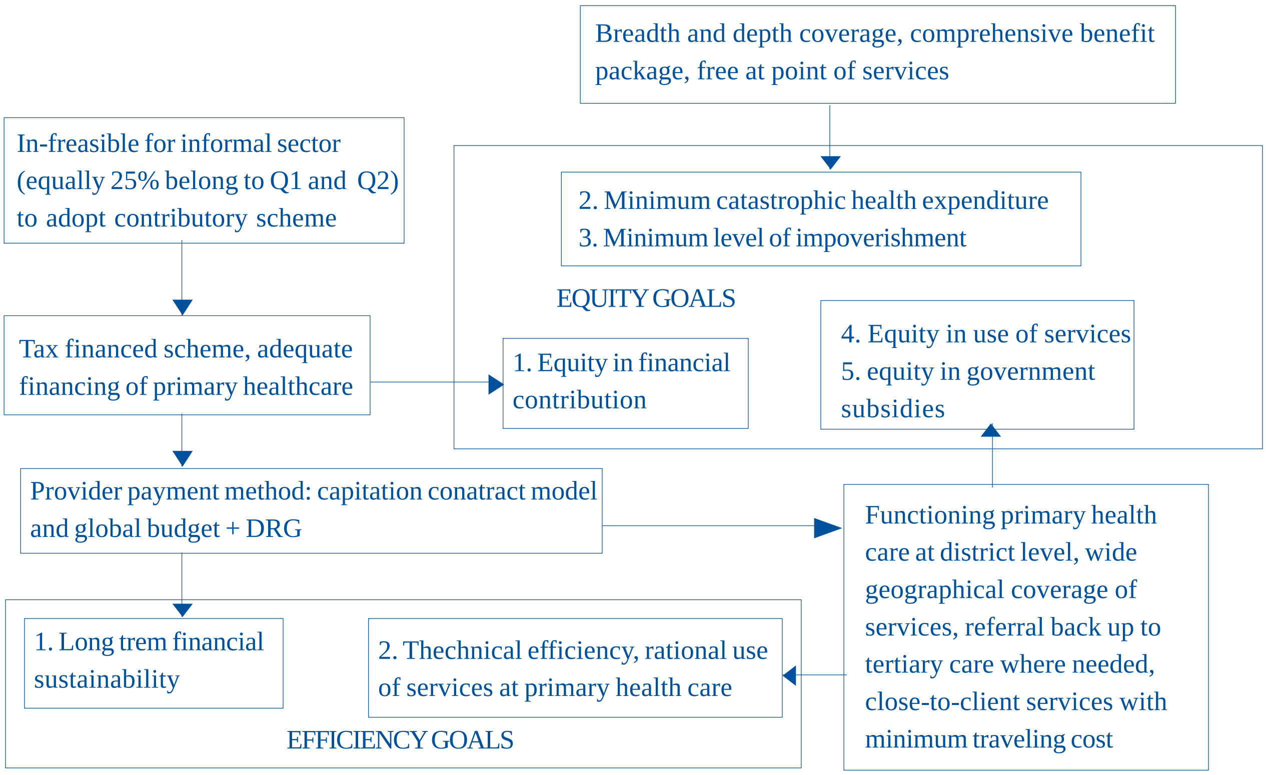
Figure 15: How equity and efficiency were achieved?



The scheme in Figure 15 represents our 10 years experience of consistent reforming, monitoring, and evaluating and produced the evidence to guide the subsequent policy. There are 5 Equity Goals shaded in yellow in the center box:

1. Equity in financial contribution
2. Minimum catastrophic health expenditure
3. Minimum level of impoverishment
4. Equity in use of services
5. Equity in government subsidies

These equity goals have 3 contributing factors. Firstly, coverage of breadth and depth, which has comprehensive benefits, package and is free at service points. Secondly, a tax financed scheme, which adequately finances Primary Health Care, and thirdly, a functioning Primary Health Care system at District levels, which covers a wide geographical area and provides an extensive coverage of services.

The provider payment method of capitation contract model and global budget, plus DRG contributes to achieve efficacy goals. It has been proven that for the next 40 years, given Thailand's modest GDP growth, health financing will still be in good shape. It will also be much lower than 5% of GDP for the next 15-20 years. We continue to achieve technical efficiency because we foster the use of Primary Care as a gatekeeper. In essence then, this sends a positive message to you as a Public Health Education Institution.

V. Conclusion

Strong political support is the key to achieving the goals of the UC. The Indian government also demonstrates strong political support towards its health sector as evidenced by the fact that India's Federal government has invested more in health in the last 2-3 years. The Vietnamese government has also been setting the goal of achieving UC by 2014. In this regard, the IHPP has a strong link with the Health Strategy Policy Institute (HSPI) in Vietnam to work side by side.

A health systems capacity and resilience to rapid nation-wide scaling-up programs are the most crucial and enabling factors in achieving the goals. Another important enabling factor is the nationwide extension of PHC coverage, which includes the provision of mandatory rural health services by recently graduating medical doctors and other health workers.

Lessons from predecessors include the following factors:

- SHI capitation contract modelling
- CSMBS "no go" fee for services due to cost escalation and inefficiencies
- Voluntary Health Card Scheme – adverse selection and financial non-viability

I would like to end by commenting on inequities in maternal and child health outcomes, which were largely due to socio-economic factors, particularly maternal education. This has put us in a difficult position because educational reform is far behind.

It is particularly difficult for the health sector alone to solve MCH outcome inequities.

- Intersectoral actions require the following:
    - Poverty reduction efforts
    - Education to keep female adolescents in Secondary and Tertiary education systems
    - Social and economic policies towards income redistribution.

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ICT in Public Health Education and Training
By Ms. Cecil Benavidez, InWent Health Asia

'Reducing health inequities through the strengthening of health systems and ICT applications in public health education and practice in PHC'.

A lightly edited transcription of the keynote speech by Ms. Cecil Benavidez on the topic of 'ICT in Public Health education and training', at the Dusit Hall of the Dusit Thani Hotel, Monday, October 19, 2009.

Good morning to each and every one of you. So, let's shift a bit to the most appropriate, or, I think, the most apt topic of this generation, which is the technological generation or the ICT generation. This is a little twist from serious health issues to navigating through an e-platform or e-portal.

Before I do so, let me firstly introduce my organization. INVENT is an an acronym for a very long German name, 'Internationala naita bilta biklung biembaha'. We translated it to English simplifying the name to 'capacity building international', but we are indeed known as INVENT. INVENT, is a very new organization which has recently emerged from very old organizations, mainly the 'Deutsche in vikloom', or DSE or the CDG, or the 'Carl Deusburgher de Gentart', which has been giving training programs for more than thirty years. Our core business is really the training profession. INVENT is one of the four German development agencies. The other one is G percent, which is engaged in providing technical assistance, the KFW, which provides financial co-operation, and INVENT which is the capacity-building arm.

INVENT does not speak for its other partners. We operate with an annual business quotium of about 137 million Euros, and the program has an average of 57,000 participants annually, 25% of which are Asians. Training programs are practically free when you are qualified. Later on, I'll show you how to avail yourself to training programs. I think SEAPHEIN could benefit a lot from us in a sense, because I think we are practically into the training profession. Training courses by INVENT are non-degree courses. The course durations range from five days to over a year and we also adhere to an annual goal just like SEAPHEIN. So far, in this particular meeting, I would like to emphasize that the health related development courses are related to the MPG goals, which reduces child mortality, improves health, and combats HIV AIDS, malaria, and other diseases. What is the main focus of our division, specifically for the health sector, or our vision or focus on public health specifically?

Our main fields of activity are as follows:

Health in international contexts. We support health policies and maintain a managerial capacity in the health sector. As I have mentioned earlier, our health policies and prevention strategies differ from country to country. Specifically, what we have done so far in the Philippines, in health financing and health equity, is that we have to tried to support and promote universal coverage. We tried to intervene by providing training programs for the local government unit heads.

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We have also assisted with health system financing, and TRIPS, which is something to do with the promotion of products, partners and registration. We have also assisted in the area of human rights through the provision of the 'health as a human right' training program, which we operate with W.H.O. As regards HIV AIDS and reproductive health, we provide on-line training programs, which create awareness of HIV prevention, diagnosis, treatment, care, and mainstreaming. For management courses health, we have been running a district health management course, which is a one-month comprehensive management course designed for this region. We finished our 22nd series of this course last May. This is a course designed specifically for Kampuchean, Vietnamese, Indonesian, and Filipino health managers. INVENT gives training provision specifically for managers and experts in the region. Another management course in health we have is the 'hospital management' course, which is also a BLANDID course. This is a face-to-face and on-line course, as is the 'management of medicine course'. What do we mean by a BLANDID course? BLANDID courses are regional courses, which are composed of both face-to-face seminars, and on-line courses. INVENT is proud of the fact that since 2002, we have provided e-learning courses to the region. We are also known for our training programs and the use of modern training dynamics. I'll now show you an example of an e-learning course.

What do we mean by an e-learning course? Running your seminars using a computer is not an e-course. An e-learning course is the delivery of a training course through the internet. Later on, I'll be showing you the advantages and disadvantages of an e-learning course, but you can see that an e-learning course is the trend of the times.

This is the technological generation and whether we like it or not, we have to cope with this. So later on, we will navigate together through the e-learning map for INVENT. I'll be showing you, on-line, what the e-learning platform looks like. Somehow, I hope that this will give you an idea of how to carry out an e-course, maybe through SEAPHEIN or your organization. I know it's a bit complicated at first but once it's installed, it will be of benefit to you. Since the year 2000, we have trained 95,000 participants, including one to one training, through our e-learning platform.

This is an example of one BLANDID course in South East Asia, which is an international approach to acquiring financing and social health insurance. This is specifically for the Kampucheans, Indonesians, Filipinos and the Japanese. One of the recipients of this course is Dr. Gufron. Dr.Gufron was one of the participants of our course last year. Later on, he will tell you how he has benefited from this. The progress ensured the continuance of the HIVE course. The health financing mechanism of this course provides schemes that have to be based on a thorough understanding of the basic principles of health financing. Later on, I will give this to your secretariat. We have a CD version of this course, which I will be sharing with you. The beauty of this course is that it is a regional course, and contains the best practices from different countries. I have stated the need for these practices to be appropriately applied and not simply copied. The participants learnt from each other's practical learning and experiences. That is the beauty of our regional courses, and on-line or not, will really reach a lot of participants and those aligned with participants.

Also, this course provides country specific conditions and capacities and these have to be taken into account to ensure sustainability in their specific contexts. The on-line learning platform of INVENT is what we call INVENT local campus 21. I will show you.

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So let's go to the local campus platform of events. This is how it looks. The website address is www.g621.de. When you get into the double campus 21 it looks like this. I will navigate you through this. If you speak German then you press Deutsche. Since we are all English speakers, we will follow the English. This is a public campus 21 open event and it was installed in the year 2000. g621 is a service for e-learning and development. It has specific features. I don't have to go into details about the g621 platform, but this can be locally adapted. I know in the Philippines we have an e-skills map, which came about from the e-learning development and implementation course that we had, and the Filipino participants just usually used notes as their base program.

I told you earlier that g621 had more than 57,000 registered participants, so even if there are other multiple users of this platform, the program can still be delivered. There are currently over 8,000 active participants using this because of the current e-learning courses that we provide. There are more than 900 workspaces including on-line work groups and on-line course environments.

How do you become a participant? Later on, I'll tell you. You need to enroll in any of our on-line courses. If you can qualify, this is for free, but only for specific target groups for specific courses. E-learning and INVENT was confronted with a lot of challenges on set and it's infrastructure initially cost a lot of money, but later on as we went along, it became very cost effective because it reached so many participants in their workplaces. You can see here the different activities of events. All the training programs are on this site and include their specific course announcements, info, deadlines, and on-line registrations. You can see here that there are a lot of on-line coures.

It's not only the health sector that we're serving. We are also serving different thematic areas. So, we'll just focus now on the on-line courses on health because this is so relevant. For demo purposes, we can log into g621 using a guest surname and a guest password which you can change. The on-line course that we have here is about HIV AIDS. This is specifically only for China. As you can see, we are currently also running a course on 'health and human rights', in partnership with W.H.O. Also, there is the 'management of medicine in international health', which is directed by doctors, pharmacists, managers and administrators. The upcoming course, which will start on November 23, is the 'International approaches to health financing and health insurance' course. Another course is specifically about the effects of the trades' agreement, which includes pharmaceutical medicines. We'll go back now to health financing so I can show you a demo version. This is what the portal looks like. It includes an introduction to what the course is all about, including an e-learning course module overview, which states the learning objectives. It also shows the dates covered, and a school type syllabus guide. Downloadable documents are also uploaded by the tutors and participants alike. So how it is facilitated? We have PIN words. PIN words are where you put in messages.

When there is no message, what you want to say or what you want to comment on, can be guided by the date like a calendar. There are discussion groups and chat rooms where you can exchange ideas on certain topics. In chat rooms for example, there is normally a speaker or nine participants. In every message that you post, your picture appears. Normally it is done at a specific time common to the maturity of the participants, and discussions will be carried out with the help of an on-line tutor and an on-line expert. Exercises are also posted here. You can also give your feedback. There is feedback for virtually every topic that has been discussed, so that you will be able to find out the satisfaction level of the participants. We also have pool or whatever you want to pool over. For example, you can poster your documents or certain references. There are also question boards and web mail. Learning happens even if you're off-line because tutors also watch over web mails and you can also get in touch with your tutor for certain questions. So that's what the portal looks like. In a way then, e-learning BLANDID courses of events starts with a face to face which normally happens at the start of the course and then the participants get one or two days to get adept with the technology. You'll be taught how to use the portal at the introduction to the course, and then you'll go on with the on-line courses, modular courses (for the next six months in the case of the HIF course), and then it ends up with a face to face at which time you have to assess whatever you have learned, and submit to the group output. There are group outputs and there are individual outputs on our on-line courses.

As I have said, there are other thematic areas in case you're interested, such as in the case of education. We also provide e-learning courses for each of these courses, which are run annually in the region and in other parts of the world.

The e-learning courses that helps financing and social health insurance is a BLANDID course, and everything is carried out through an e-platform or through the internet. So this is how it is in format. It starts with an intro and then tutored on-line courses, a face-to-face workshop, and on-line coaching, and of course includes professional development. This is something at INVENT we have really invested in. We have a lot on-line which we are still supporting in the form of capacity building.

So this is a picture of our participants you can see. Older students also participate. INVENT is then propagating life long learning. As long as you have an open mind and an open heart there is no limit to learning. You can see here our oldest participant. I don't want to go into details. You can navigate this on your own through the internet and see for yourself what an HIF course is all about. We don't just deal with the hosting of topics. As well as having an interactive module, we also use other didactics such as discussion, just like the notes we are able to drive, which are more innovative in a way, because they provide practical learning as we progress. We are very much aware that our participants are adult learners. We came up with different characters to ensure that we have an international look. That's why our slogans have different cultural looks. It's still an on-line course and so we will rush through this. We also provide linkages. For example, if you want to learn more about social health insurance, we provide you with basic linkage information where you can learn further.

Next, these are still pictures of the different activities that we have in the face-to-face component of the courses. Normally in the face to faces, such as a 'one way learning', we always engage our participants with small group discussions, recitations, SKITS and different other training methodologies.

This is another course of ours. It's a BLANDID course as well called 'management in health'. This is an international leadership training hospital management course for Africa and Asia. It runs for two years transferred or for one-year in Germany. Most of INVENT's training programs really start to add human resources. Another course which has been steadfastly running in Asia for the last six years is the 'health management - improving the the central lives health services' course, or otherwise called HSDM. Actually, it's a comprehensive one-month health management course and it deals with the four major essential topics a health leader should have, namely human resources, epidemiology, health administration, and financial management. This is quite

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a popular project of ours and we have brought this to the district health management for the 22nd time since 2004. In the early part of the delivery of this course, only India and Pakistan were involved, but in recent years, we have concentrated on South East Asia, involving participants again from Vietnam, Cambodia, Indonesia and the Philippines.

Another program that is running in the Philippines is the school health program called 'food for health'. This is run in co-operation with CHETESET. This involved the promotion of tooth brushing and hand washing for primary level students. This program achieved really tremendous results and had involved the local health government units in health prevention intervention strategies. We also involved the local health unit and the Department of Education in the promotion of hand washing, warming, and tooth brushing for primary aged school children because we know that dental care is currently a major problem in Asia. Another course that we have is the HIV AIDS course, which is a BLANDID learning course dealing with this topic, but is only for China. Another course that is running right now in partnership with W.H.O. is on human rights. Another course, which is also running is a combination course. Yet another BLANDID course is the 'hospital management' course, which entails various topics. Another course, which Dr.Hasbullah recently attended, was the course on social security training, which started in Manila. You can tap INVENT courses through our website, wwinvent.org and locate, on-line, whichever course or course are most appropriate for you and your staff. Once you get accepted, it's free. So it's something maybe SEAPHEIN can tap into later on. Simply get in touch with us, so we can devise the most appropriate means of delivering a program to you.

So, why an internet based training course? Of course, we know that e-learning is not a substitute for basic teaching and training in health, but it's the new and innovative option. As we also know, the present generation is a technological generation, and this is something with which we have to cope. This is really very appropriate for adult learning and working groups for a continuous education in capacity building. It's also suitable for multi-sectoral and multi-national corporations because it provides us with a borderless world.

The traditional ways of education you know very well through attendance at conferences and lectures as well reading articles and textbooks. However, learning is now multi-media based, so you can upload your references, flash presentations, and can have everything that is multi-media based. You can upload in a portal and this contemporary way of learning is also highly interactive and participatory.

And of course, what is important, if you are really busy people, is that it is close to the workplace and permanently available for courses running for six months. Everything is permanently available on the portal and orientated towards communication, team building and it is, of course, research orientated. E-learning and health capacity building is an option because unlike other methods, it's a wider scale facilitative multi-sector collaboration. It also provides a venue for the sharing of experiences and practical learning in the fields of medicine and economic science among specialists, while simultaneously allowing problem-orientated activities and constructive learning on the internet in a rapidly changing field of knowledge.

Before I give you the disadvantages of e-learning, here are the advantages of e-learning. Contents and methodologies can easily be adapted to the needs of the participants. It has flexibilities, in other words. Participants need to be on-line at the same time in order to co-operate. There is also email, or if you have established a ready connection with your other co-participants, that is another means of communication.

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Training and dialogue are to a huge extent independent. As I said, it's a borderless world. You needn't leave your workplace to avail yourself to e-learning and easy co-operation with other participants from other continents. Training can be offered whenever the need is felt by participants because each e-tutor is readily available to help at anytime.

Another set of advantages, earlier discussed, is that training can be done during work hours, and can be more specifically adapted to the problems and challenges of the work context. Documentation and repetition are very easy. Modules of training can be easily used in other courses and computer technology allows the use of many media pictures, graphics, movies, flash and others.

The disadvantages of e-learning:

E-learning is dominated by technology. Consequently, some participants as we know, have difficulties dealing with this, especially those who are not computer literate, but as I have said, BLANDID courses or e-learning courses always begin with an orientation of the technology, so don't be afraid of it. There was always a first time for others who have done these courses. New participants need training with the IT system first before they can begin their training. If you're not familiar with internet based things, then now is your time for learning. Although there are a few good teaching products, most courses need to be re-developed, it's true. Before ordinary courses can be transformed into e-learning courses, they have to be processed and adjusted to an e-learning module. As quality tutors are scarce and need to be trained, so we have run annual training courses for e-tutors over the past four years.

Also, there are constraints on the media, because there are times when some participants cannot go on-line and group chat rooms. There are also cases for some participants not having access to computers or to the internet, because they may have work commitments for example, but somehow we also give way to that, so participants are able to complete their e-learning courses.

For further information on INDEX courses, you may visit our global campus 21 site. The address is wwg621.de, or for further information or inquiries, you can get in touch with us through my email, which is inventhealthasia@our.com. For further collaborations, don't hesitate to get in touch with us.

We are at one in our purpose to deliver capacity building training programs to our colleagues in the health sector. We really really promote health improvement programs, specifically on public health, so, we want to be of help to you and your organization. So this is all for now. Hopefully, we have given you an insight on how e-learning courses function. Thank you very much.

The 5th annual SEAPHEIN Meeting : Group Work

Group work : Scope for networking and collaboration among SEAPHEIN member institutes

Objectives:

1. To share knowledge and practices gained through collaborations.
2. To identify common areas of research among member institutes.
3. To identify training needs in terms of developing multi-sector connections, including the mobilization of technical and financial resources from public and private sectors, and the marketing of member institutes abilities.
4. To form a committee, including the installing of a chairperson and committee members, to oversee the exploring of opportunities and, if possible, conduct training sessions on the needs identified above.
5. To form a committee including the installing of a chairperson and committee members, to be responsible for developing a collaborative research proposal over the course of the coming year.

To share knowledge and practices on collaborative activities

- Knowledge and practices should be regularly shared.
- Country/institution's reports should include details about their national inter-institutional and/or international collaborations.
- There should be a standardized guideline for reports.
- Each member should upload knowledge and practices gained through collaborative activities on their website.
- The SEAPHEIN website should be updated.
- Suggestions should be made to arrange the first SEAPHEIN International conference so knowledge can be shared.
- There are two meetings to attend. The APACPH meeting in Bali in late November 2010, and the Asia Pacific Epidemiology Conference in Srilanka in May 2010.
- Developing a standardized double degree in Master of Public Health among member countries.

To identify common areas of research among member institutes

- Forming groups working on specific projects such as a Management group and an epidemiology group, etc.
- Conducting educational research in Public Health in Myanmar and Indonesia, and in various fields of member countries including education, science, and public health and technology.
- Evaluation and research needs to be done on the viability of continuing education in order to improve practice capacity.
- Getting support from W.H.O. or other sources for the publication of research findings.
- After the international conference, collating a list of experts in each field.
- Encouraging more participants to present their research papers at SEAPHEIN international conferences.
- Establishing grants and funding resources for, and profiles of, experts from each country and field.

The 5th annual SEAPHEIN Meeting : Group Work

- Establish a SEAPHEIN Research and Training center to identify training needs in terms of developing multi-sector networks, and the mobilizing of technical and financial resources.
- To identify the training needs of member countries using data from research, health information programs and health care reform.

To determine how to do quality research, publications and health care financing (Thailand), economic and Public Health policy, Public Health ethics, and the evaluation of Public Health competency.

- Improving capacity building in advocacy.
- Public Health practitioners should be trained more effectively and practically at different levels, such as practice levels and administrative levels as well as in leadership qualities and working in public sector and private sector partnership programs.
- A two-week training course for SEAPHEIN members.
- Suggested training in the fields of health economics, Public Health policy, health financing and Public Health ethics.
- Public Health Curricula and competency levels should be evaluated in each member country.
- A committee needs to formed to ensure that MPH degree courses in each SEAPHEIN member country are standardized.
- Training and conferences can be made by a sub-committee such as Bhutan, Srilanka or Indonesia.
- A collaborative research proposal could be made at the executive board meeting.
- A guideline, country, and institution profiles should be updated on the website.
- Capacity building should be undertaken by junior staff of every member country.
- A SEAPHEIN Council needs to be established to ensure quality assurance.

The 5th annual SEAPHEIN Meeting : Group Work

Group work : Approaches and strategies to address health inequities

The objectives of the group work are as follows:

1. The roles and performances of academic institutions include the addressing of health inequities.
2. Based on the academic backgrounds of Public Health institutions, determining how to strengthen health system reforms and increase health equity in Asia.
3. Determining strategies to strengthen health systems in order to reduce or eliminate health inequities.
4. Developing strategic plans among Public Health institutions to address health inequities in Asia.

1. The roles and performances of academic institutions to address health inequities are as follows:

- Teaching courses in several topics such as: equity in health policy and management, health economics, and health anthropology etc.
- Arrange short courses using ICT and IT in this region.
- Provide regular courses for undergraduate, postgraduate (PG) and CME students.

The equity concept should be introduced in other disciplines through:

- The sharing and exchange of expertise and other research resources including inter-institutional and community-based research among SEAPHEIN members.
- Developing health equity indicators, and identifying the key indicators in order to define and measure equity.

The role of academic institutions in addressing health inequities are as follows:
To consult, advise, and advocate other organizations.
To capacity build among SEAPHEIN members.
To network collaboratively.
To serve the community by applying a pilot project to address health inequities.
To disseminate knowledge through publication of the SEAPHEIN journal, including using the SEAPHEIN committee as the editorial board of a special issue on the subject of health inequities.

2. Based on the academic background of Public Health institutions, the challenge is to strengthen health system reforms in order to increase health equity in the Asian region.

The strategies used to facilitate this goal are summarized below:

- Curriculum review and modification.
- Translating and transforming research findings into policy through dissemination at workshops and by advocating meetings using existing networks.
- Listening to stakeholders including the community.
- Making community field laboratory arrangements such as field research studies, community diagnoses, community treatments, community development, community health solutions, community empowerment and community participation in PHC.
- The exchanging of health professionals among member institutes.

The 5th annual SEAPHEIN Meeting : Group Work

3. The challenges of strategies to strengthen health systems in order to reduce or eliminate health inequities

The challenges of strategies to strengthen health systems are as follows:

- How to involve all the decision makers at all administrative levels.
- Translating and transforming research findings from the academic domain to policy and actual practice.
- The allocation of resources into health sectors.
- If Public Health professionals were to become politicians, they could facilitate the strengthening of their respective country's health systems.
- Ministry of Health personnel should include members of the Public Health profession and not just clinical doctors.
- Health system responsiveness.
- The development of a management strategy to counter resistance to change and reform needs to be prepared.
- The summary of strategies to address and/or overcome health inequities are:
- A borderless/ trans-boundary, cross-cultural collaboration to global health.
- Community empowerment and education.
- Public Health science and problems should be transformed into policy, which includes a budget policy.
- Building political commitment for equity in health care reform.
- Utilizing the mass media to create the demand for health equity.
- A customer focus approach in the delivery of quality care health services to the community.
- Establishing a health system performance measurement, especially in regards to fair finance.
- Strengthening and allocating highly competent manpower at different levels of health services focusing on Primary Health Care (PHC).

4. Strategic plans among Public Health institutions to address health inequities in Asia

Optimizing the efficiency and effectiveness of existing networks through the establishment of different task forces.

- The use of ICT.
- SEAPHEIN Journal.
- The documenting and sharing of the best practices in health equity.

Annual meetings of the task forces using e meetings, e seminars and e conference facilities.

- Finding other sources of financial support for SEAPHEIN activities.

The 5th annual SEAPHEIN Meeting : Group Work

Group work: ICT applications in Public Health Education and Practice in PHC

Objectives of the group work are as follows:

1. To utilize ICT in the improvement of access to health services in rural areas.
2. To utilize ICT in Public Health education campaigns in critical areas.
3. To utilize ICT in the transference of diagnostic information to specialized centers.
4. To utilize ICT in reducing the cost of transporting patients to urban areas

1. To utilize ICT in the improvement of access to health services in rural areas.

- Advertise mobile telephone numbers to help serve patients who are waiting.
- Register patients and link this information with higher centers.
- Provider's perspective: Health workers have to prioritize. Place patient's details on a data base to facilitate services.
- In order to improve access to centers, workers could be offered palm tops.
- Establish short courses in the use of teleconference and satellite transmission facilities in every country.
- Interactive student/tutor e-learning courses based on actual case studies.
- Establish a tele-medicine connection between health care providers and patients so necessary consultations can be offered.
- Prepare and update daily, a database of diseases and other health related events. This database could be used as a referent to monitor health status and can also be used for research purposes. An information system could be devised for all patients, including pregnant women and children and socioeconomic profiles could be prepared for every family.
- Hospitals could retain detailed patient records for dissemination for assessment by journals.
- Health workers could be trained by specialists in the telemedicine approach on an on-call basis.
- E platform
- Utilize the extensive e-learning program TUSK, which is free to access.
- Utilize MOODLE software to provide training in the use of online case discussions.
- Include the application of ICT including satellite programs in rural areas in post-graduate curricula. Students should be pro ICT.
- Each country should request from their government investment for infrastructure for health systems in rural areas.
- Use internet shops to participate in e-learning courses outside the area of health rather than buying infrastructure.
- There is a comparatively limited amount of ICT resources in the public sector compared to the private sector.
- ICT resourced higher centers can avoid delays in services, which may result in lower mortality rates.
- Use the existing World Bank e-learning package.
- Although power-point presentations are available, e learning is more appropriate.
- Upload updated information to the interactive SEAPHEIN website.
- SEAPHEIN members should have access to an e-learning journal.

The 5th annual SEAPHEIN Meeting : Group Work

- Have a SEAPHEIN e-learning package and organize a CAME using ICT.
- Utilize ICT to both assess health services in rural areas and estimate how ICT can contribute to the expansion of health services.
- ICT has a significant role in postgraduate education, on the job training, and in emergency medical services.
- Fulfill our role in influencing policy and programming by bringing into practice the results of SEAPHEIN member's operational research.
- Transmission fees are very expensive in rural areas.
- Mobile phones can be utilized to strengthen health information systems.
- Local government units are equipped with computers and mobile chips, which facilitate e-learning programs.

ICT in health services

- Use ICT for the diagnosis, treatment and management of outbreaks.
- Universal coverage is a more important priority than ICT. SEAPHEIN needs to achieve the MUG goals by integrating, through intersect oral collaboration, ICT in its pre-services and services be cause the health sector alone cannot address this need.
- Good ICT needs to exist within SEAPHEIN. It would cost approximately USD $2,000 to set up a website for SEAPHEIN plus annual maintenance costs of about USD $200.
- ICT connections between health centers and universities need to be established so that all Thai universities can access journals. THAIPHEIN for example, has such a connection.
- Primary health care centers could utilize ICT to report outbreaks of diseases in the districts while the districts independently utilize ICT to manage outbreaks of diseases.
- Implement video tele-conferencing to train students in rural centers because good lecturers are based in cities only.
- Design a SEAPHEIN website being aware of the beneficiaries of the site. The beneficiaries can be students, teachers, trainers, health workers in the public health sector and large communities.
- Get the commitment to invest in ICT and co-ordinate inter-sector projects.
- Establish a SEAPHEIN e-library from which public health data can be shared amongst members and other countries.
- Lower the language barrier by identifying resources that can be translated into a local language. Important books can also be translated to English.

The 5th annual SEAPHEIN Meeting : Group Work

Conclusion

Establish an interactive but uncomplicated SEAPHEIN website linked to websites of other institutes in order to make SEAPHEIN a vibrant organization.

Invest in IT staff for website development at the SEAPHEIN office.

Utilize an e-journal and e-learning packages, which identify common areas of interest to achieve the MDGs.

- **CPHE.**
- SEAPHEIN should conduct an ICT needs assessment in order to develop a document to improve the utilization of ICT.
  1. Utilize the current health care and training component networks and links.
  2. Utilize ICT in public health education campaigns in critical areas.
  3. Utilize ICT in the transference of diagnostic information to specialized centers.
  4. Utilize ICT to reduce the costs of transporting patients to urban areas.

Summary from SEAPHEIN executive board meeting
18 October 2009

Executive board members present:

Dr. Phitaya Charupoonphol, President

Dr. Hasbullah Thabrany, President-elect

Dr. K.R. John (representing Prof. Dr. Sulochana Abraham, focal point India)

Dr. Siswanto Wilopo (representing Dr. Ali Gufron Mukti, focal point Indonesia)

Prof. Dr. Myo Oo, focal point Myanmar

Prof. Dr. Chencho Dorjee, focal point Bhutan

Prof. Dr. Chhea Chhorvann (representing Prof. Dr. Vonthanak Saphonn, focal point Cambodia)

Assoc. Prof. Dr. Somboune Phomtavong, focal point Laos

Prof. Dr. Yoshihisa Watanabe (representing Prof. Dr. Takaaki Kinoue)

Prof. Dr. Somjits Supannatas, focal point Thailand

Prof. Dr. Nilambar Jha, focal point Nepal

Prof. Dr. Rohini De A. Seneviratne (representing Prof. Dr. Dulitha Fernando, focal point Sri Lanka)

Observers:

Dr. Jigmi Singay, WHO-SEARO

Dr. Prawit Taytiwat, Dean, Faculty of Public Health, Naresuan University

Assoc. Prof. Dr. Prayoon Fongsatitkul, Head, Department of Sanitary Engineering, Faculty of Public Health, Mahidol University

Assoc. Prof. Naowarut Chareonca

Assoc. Prof. Pornpimol Kongtip, Deputy Dean of International Relations, Faculty of Public Health, Mahidol University

Ms. Nancy Noma, SEAPHEIN coordinator

The first item on the meeting agenda was discussion of proposed charter revisions. The following items were discussed:

- Article 3.0 of the charter which discusses the composition and term of executive board members
- Location of the SEAPHEIN secretariat office – the charter states the secretariat office is permanently located at Mahidol University, there was extensive discussion as to whether the charter should be amended so that the secretariat office was housed at the institute where the SEAPHEIN President is located
- Need for full time staff member for secretariat office, various titles for this person were given manager, director, coordinator
- Lack of strategic planning
- Need to examine resources available for future activities
    - Financial
    - Human

Summary from SEAPHEN executive board meeting

- Increasing member payments of annual fees
- Role of national networks such as LankaPHEIN, ThaiPHEIN, IndoPHEIN
- Role of country focal points
- Proportional representation of countries was discussed but deemed not to be an issue at this time

As a result of discussion the following amendments to the charter were proposed:

1. Change title of Chair to President throughout the document (Starting in item 3.0).
2. Item 3.0 should be amended from "The Executive Committee is composed of a Chair, Treasurer, Member Secre tary and one representative for each country. The term of office shall be three years and is not renewable. Chair elected should be put in place. The Term of Office shall start and end at the closing of the Annual Meeting." to read as follows:
   "The Executive Committee is composed of a President, President-elect, Treasurer, Secretary, immediate past president and five elected focal points. The term of office shall be two years and is not renewable. Chair elected should be put in place. The Term of Office shall start and end at the closing of the Annual Meeting."
3. Focal points are defined as representatives of each country, selected by the country network or if only one institute exists within a country selected as a representative of that institute.
4. Add office of President-elect.
5. Item will be added stating a sub secretariat office will be located at the institute where the SEAPHEIN President is located.
6. Add an item noting that the Secretary shall be appointed by the President.
7. Add an item stating that fundraising activities will be pursued in order to assure income generation and maintain network sustainability.

Voting procedures for choosing officers were then discussed. The following guidelines were agreed upon:

1. For this election, those member institutes who have paid their 2009 membership dues will be allowed to vote.
2. Country focal points at the meeting are to meet with the delegates from their country and determine nominations for President-elect, Treasurer, and focal point.
3. For the office of President-elect, each country will propose a nominee. Those members of the general assembly representing institutes that have paid their dues will then vote from this list. The person with the greatest number of votes will become the new President-elect.
4. For the office of Treasurer, each country will propose a nominee. Those members of the general assembly representing institutes that have paid their dues will then vote from this list. The person with the greatest number of votes will become the new Treasurer.
5. For the executive board, each country will propose a focal point to serve on the executive board. Those members of the general assembly representing institutes that have paid their dues will then vote from this list. The five persons receiving the largest number of votes will become executive board members.

Summary from SEAPHEN executive board meeting

There was also discussion regarding proposed activities SEAPHEIN should undertake in the coming year. Following were some of the activities that were proposed:

a. Compiling a directory of all public health institutes in the region and their curriculums
b. Fundraising activities such as training workshops
c. Consultancy
d. Advocacy
e. Accreditation documentation
f. Text book
g. Compilation of a list of experts from SEAPHEIN member institutes

The following activities are to be undertaken in the coming year:

1. Compilation of a list of experts from SEAPHEIN member institutes The SEAPHEIN coordinator has already approached many members to ask them to submit a brief list of experts at their insti tutes prior to their departure from Bangkok. CVs and more detailed information will be pursued following the annual meeting. The focal points present at the meeting were asked to assist obtaining this information from the delegates from their country. A short list will be submitted to Dr. Jigmi Singay by 23 October 2009.
2. Fundraising enterprises

General Assembly

A General Assembly was held to address organizational issues. The program was initially chaired by the SEAPHEIN President, Dr. Phitaya Charupoonphol.

New members of SEAPHEIN

The following institutes were unanimously approved as new members by the General Assembly:

- Faculty of Public Health, Pathumthani University, Thailand
- University of Community Health, Magway, Myanmar
- School of Public Health, National Institute of Public Health, Cambodia
- Maldives College of Higher Education
- School of Medical Sciences, Katmandu University, Nepal
- Royal Institute of Health Sciences, Royal University of Bhutan
- This brings the total number of SEAPHEIN member institutes to 56 from 13 countries.

Charter amendments

The Executive Board proposed the following charter amendments:

1. Change the title of Chair to President throughout the document starting at item 3.0.
2. Item 3.0 should be amended to read as follows:
   "The Executive Committee is composed of a *President, President-elect, Treasurer, Secretary-General, immediate past president and five focal points who will be elected as representatives.* The terms of office shall be two years and are not renewable. *The elected President* should be put in place. The Term of Office shall start and end at the closing of an Annual Meeting."
3. Focal points are defined as representatives of each country, selected by that country's network, or, if only one institute exists within a country, selected as a representative of that institute.
4. Add office of President-elect.
5. An item will be added stating that a sub-secretariat office will be located at the institute where the SEAPHEIN President is located.
6. Add an item stating that the Secretary shall be appointed by the President.
7. Add an item stating that fundraising activities will be pursued in order to maintain sustainability and a decreased reliance on WHO-SEARO for support.
8. Change the title of Secretary to Secretary-General throughout the document.

After extensive discussion about the content in the proposed amendment of the charter, some issues have been raised and approved by the General Assembly. The following items have been approved by the General Assembly.

1. Change the title of Chair to President throughout the document starting at item 3.0.
2. Item 3.0 should be amended to read as follows:
   "The Executive Committee is composed of a President, President-elect, Treasurer, Secretary General and one representative from each country. The terms of office shall be two years and are not renewable. The President elected should be put in place. The Term of office shall start and end at the closing of an Annual Meeting."

3. Add office of President-elect.
4. An item will be added stating that a sub-secretariat office will be located at the institute where the SEAPHEIN President is located.
5. Add an item stating that the Secretary General shall be appointed by the President.
6. Change the title of Secretary to Secretary-General throughout the document.
7. Add an item stating that fundraising activities will be pursued in order to maintain sustainability and a decreased reliance on WHO-SEARO for support

Installment of Dr. Hasbullah Thabrany as President of SEAPHEIN

Dr. Hasbullah Thabrany was elected as President–elect for two years at the General Assembly of the 3rd Annual SEAPHEIN Meeting in Jaipur, India in 2007. He was installed as the President of SEAPHEIN at the closing of SEAPHEIN Meeting.

Election of officers

Dr. Hasbullah Thabrany, President of SEAPHEIN, chaired the session for the election of officers. Only full members may vote and each full member has only one vote. There were 30 voting Public Health institutions. Dr. Phitaya Charupoonphol was elected as President-elect and Dr. Ardini Raksanagara as Treasurer. Dr. Phitaya will resume as President of SEAPHEIN at the close of the 2011 annual SEAPHEIN meeting.

Priority areas of SEAPHEIN programs

Under the direction of the new SEAPHEIN President, Dr. Hasbullah Thabrany, the following priority areas for SEAPHEIN over the next two years are as follows:

Creating virtual discussion on topics relevant to strengthening the roles of SEAPHEIN members in order to achieve broad health improvements. Priority topics are health care reform, universal coverage, equity, health financing, health policy, a revitalization of Primary Health Care, and hospital management.

1. Establishing a directory of resources consisting of well-known faculty members who can serve as international consultants, temporary advisors to WHO–SEARO, and who can conduct joint training and courses.
2. Organizing collaborative courses, or, training two or more institutes from two or more countries, to be sold to the general public, governments, and the private sector, in order to raise funds for the organization as well as to contribute to the capacity building objective of SEAPHEIN.
3. To collaborate with other international organizations or funding agencies to strengthen training and education programs for member institutions.
4. Publish a SEAPHEIN journal and other materials to share knowledge among SEAPHEIN members and other stakeholders.

Photographs

International participants arrived at the Dusit Thani Hotel and were greeted by Dr. Pornpimol Kongtip, Deputy Dean of International affairs, before registering at the SEAPHEIN annual meeting.

After registering, Dr. Phitaya Charupoonphol, Dr. Samlee Pliangbangchang, Dr. Hasbullah Thabrany, and Dr. Somjit Supananatas met at the SEAPHEIN opening ceremony.

SEAPHEIN prime movers and colleagues President, Dr.Phitaya Charupoonphol and President–elect Dr. Hasbullah Thabrany greet each other at the SEAPHEIN meeting.

Participants in the opening ceremony: Prof. Dr. Somjit Supannatas, Assoc. Prof. Dr. Chalermchai Chaikittiporn, and Prof. Dr. Hasbullah Thabrany (above), and Mahidol staff address the key conference issues of collaboration and ICT usage.

Photographs

Group photo of 83 participants from 12 countries, primarily from South East Asia, who took part in the meeting.

Coffee talk: Dr. Phitaya Charupoonphol and Dr. Maureen E. Birmingham, WHO representative to Thailand discuss key regional issues.

Discussions and group work on the subjects of the application of ICT, and the importance of collaborative networking in the reduction of health inequities, 18-19 October 2009.

Photographs

Dinner party:

Time to relax and treat international guests to Thai hospitality, arts and cuisine. International guests were treated to a Thai traditional dance and spicy Thai foods.

General Assembly:

Voting for a new President-elect, Treasurer, and Secretary General.

Dr. Phitaya Charupoonphol officially congratulates new SEAPHEIN President, Dr. Hasbullah Thabrany from Indonesia.

Annex 1 Organizing Committee

Organizing Committee

Assoc. Prof. Dr. Phitaya Charupoonphol	Chairman
Assoc. Prof. Dr. Witaya Yoosook	Co-chairman
Asst. Prof. Sumalee Singhaniyom	Member
Assoc. Prof. Pipat Luksamijarulkul	Member
Assoc. Prof. Dr. Bhusita Intaraprasong	Member
Asst. Prof. Dr. Siranee Sreesai	Member
Assoc. Prof. Dr. Orawan Kaewboonchoo	Member
Assoc. Prof. Dr. Oranut Pacheun	Member
Assoc. Prof. Dr. Pakpimol Mahannop	Member
Assoc. Prof. Dr. Jareuyporn Suparp	Member
Assoc. Prof. Dr. Wonpen Kaewpan	Member
Asst. Prof. Pimsurang Taechaboonsermsak	Member
Asst. Prof. Supat Chaiyakul	Member
Asst. Prof. Dr.Bunyarit Panyapinyopol	Member
Lect. Supachai Pitikultang	Member
Assoc. Prof. Dr. Pornpimol Kongtip	Secretariat
Ms. Napaporn Moungsakul	Assistant Secretariat
Ms. Chenchira Natathong	Assistant Secretariat
Mr. Siam Boonkiew	Assistant Secretariat
Mr. Gavan Joseph Dunn	Assistant Secretariat
Ms. Nancy Noma	Assistant Secretariat

Administrative Committee

Assoc. Prof. Dr. Pornpimol Kongtip	Chairman
Assoc. Prof. Dr .Naowarut Charoenca	Member
Assoc. Prof. Dr. Nipapun Kungskulniti	Member
Asst. Prof. Dr. Kanittha Chamroonsawasdi	Member
Asst. Prof. Adisak Bhumiratana	Member
Lect. Tawee Saiwichai	Member
Lect. Dr. Itsanun Wiwatanaratanabutr	Member
Assoc. Prof. Dr. Duangrat Inthorn	Member
Assoc. Prof. Dr. Wonpen Kaewpan	Member
Asst. Prof. Supat Chaiyakul	Member
Asst. Prof. Pimsurang Taechaboonsermsak	Member
Lect. Supachai Pitikultang	Member
Asst. Prof. Chokchai Munsawaengsub	Member
Asst. Prof. Dr. Kwanjai Amnatsatsue	Member
Lect.Dr.Ann Jirapongsuwan	Member
Asst. Prof. Dr. Nithat Sirichotiratana	Member
Lect. Dr. Suthee U- sathaporn	Member
Lect. Dr. Aronrag Cooper Meeyai	Member
Ms. Napaporn Moungsakul	Member
Ms. Saijai Potisubsuk	Member
Ms. Siriwan Yenperng	Member
Ms. Prapaporn Youngvises	Member
Ms. Ketwaree Punyatham	Member
Mr. Kiattisak Inboon	Member

Annex 1 Organizing Committee

Mr. Prayot Sanjaithum	Member
Mr. Chatchai Yokyong	Member
Mr. Narin Pandee	Member
Ms.Weena Burawitkasetkron	Member
Ms. Sunanta Thaipop	Member
Mr. Sirote Yunchanon	Member
Mr. Chakkapan Charupoom	Member
Ms. Khanittha Pussa	Member
Ms. Chenchira Natathong	Secretariat
Mr. Siam Boonkiew	Assistant Secretariat

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Annex 4 SEAPHEIN membership payment schedule

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National Institute of Health and Family Welfare, India			✓				
National Institute of Preventive and Social Medicine, Bangladesh				✓			
National Institute of Public Health, Cambodia – 2009 Applicant	n/a	n/a	n/a	n/a	✓	✓	✓
Pathumthani University – 2009 Applicant	n/a	n/a	n/a	n/a	✓		
PGIMER School of Public Health, India			✓		✓		
Prince of Songkla University, Thailand							
Royal Institute of Health Sciences, Bhutan – 2009 Applicant						✓	
School of Public Health, National Institute of Public Health, Laos PDR							
Srinakarintharawirot University, Thailand		✓		✓			
Sukhothai Thammathirat Open University, Thailand							
Suranaree University of Technology, Thailand							
Thaksin University, Thailand		✓	✓				
Thammasat University, Thailand		✓		✓	✓		
Tokai University, Japan			✓	✓	✓		

Annex 4 SEAPHEIN membership payment schedule

INSTITUTION	2005	2006	2007	2008	2009	2010	2011
Tribhuvan University, Nepal				✓			
University of Airlangga, Indonesia	✓			✓	✓		
University of Colombo, Sri Lanka		✓	✓	✓	✓		
University of Community Health, Myanmar – 2009 Applicant	n/a	n/a	n/a	n/a	✓		
University of Indonesia, Indonesia	✓		✓	✓			
University of Kelaniya, Sri Lanka			✓	✓	✓		
University of Massachusetts Lowell, USA	n/a	n/a	n/a	✓			
University of Medicine 1, Myanmar		✓	✓	✓	✓		
University of Medicine 2, Myanmar		✓	✓				
University of Medicine, Magway, Myanmar					✓		
University of Medicine, Mandalay					✓		
University of Medicine and Pharmacy, Vietnam							
University of Padjadjaran, Indonesia	✓	✓	✓	✓	✓	✓	
University of Peradeniya, Sri Lanka							
University of Public Health, Myanmar	n/a	n/a	n/a	✓	✓		
Walailak University, Thailand					✓		
WHO Thailand Office			✓				

MEMO

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